

Annexure A to the notice to admit facts - the documents

WC/2024/227 · Shepherd v Workers' Compensation Regulator · the documents listed in the schedule to the notice, in the order there listed

Tab	Document	Date	Pages	At
1	Role description, Administration Officer, Switchboard Services (AO3), Logan Hospital	undated	4	3-6
1A	Email, Ms E Stibbard to the Appellant and the Switchboard team, "Hello & Update", produced under Tab 1 of the Stressor 1(a) particulars bundle served 11 August 2026	18 July 2023, 12:56 pm	1	7
2	Email chain, the Appellant to Ms C Taylor, Ms T Reese, Ms P Conaghan and Ms T Smith, "Increase of hours and Workplace issues", and the replies of Ms Taylor and Ms Reese in the same chain	7 to 8 August 2023	6	8-13
3	Email, Ms T Reese to the Appellant attaching HR Policy E12; and the emails of 4 and 8 September 2023	29 August to 8 September 2023	2	14-15
4	Document, the Appellant to Ms C Taylor, "Request to Increase Working Hours to Full Time Rotational Roster"; and the reply of Ms Taylor of 7 August 2023 at 1:43 pm and the email of Ms Reese to Ms Taylor of 7 August 2023 at 5:11 pm	31 August 2023	3	16-18
5	Email, Ms C Taylor to the Appellant, "Approved - Permanent Full Time FTE"	27 September 2023, 1:52 pm	2	19-20
6	Email, Ms C Taylor to Logan Switch and Switchboard staff, "Afterhours Oncall Process - Switchboard"	15 April 2024, 12:39 pm	2	21-22
7	Email chain, "Roster Concerns", as produced under the Regulator's tab of that name - Ms T Reese to the Appellant of 26 April 2024 at 1:52 pm; the Appellant to Ms Reese of 1 May 2024 at 1:18 pm; and Ms Reese to the Appellant of 8 May 2024 at 9:08 am	26 April to 8 May 2024	4	23-26
8	Emails, Ms T Reese to Mr M Pritchard "FW: Roster Concerns" with attachment "qh-gdl-401-3.3", and Ms T Reese to LBH_HR	10 and 20 May 2024	2	27-28
8A	Cover page of the attachment to that email - Fatigue risk management systems, Implementation guideline QH-GDL-401-3.3:2021	2021	1	29
8B	MASPER register email records - Ms Taylor to Vivian Kwok, MASPER Registrar, and Dr Wong, "RE: Switchboard issues 5 May, 7 May, and 8 May"; Ms Taylor to Logan Switch and Switchboard staff, "MASPER process"; and the underlying switchboard issue logs of 2 to 8 May 2024, produced under Tab 3 of the Stressor 1(a) particulars bundle served 11 August 2026	9 May 2024 (issues logged 2-8 May 2024)	4	30-33
9	Email, Ms C Taylor to Logan Switch and Switchboard staff, "Switchboard Manager - On call and Hours."	17 May 2024, 9:30 am	1	34
9A	Email chain, "Office Hours and Departmental Directives" - the Appellant to Ms C Taylor and Logan Switch copied to Switchboard staff, Ms T Reese and LBH_HR at 1:15 pm; the reply of Ms Reese at 6:23 pm; and the reply of the Appellant at 7:09 pm	15 May 2024	3	35-37
9B	Email, Ms T Reese to the Appellant, "RE: Office Hours and Departmental Directives"	21 May 2024, 2:53 pm	1	38
9C	Email, Ms C Taylor to Ms A McNamee, "FW: Office Hours and Departmental Directives"	17 May 2024, 1:20 pm	1	39
10	Email, Ms C Taylor to the Appellant, "Sick leave 14.05.24"; and, on the same page, the email of Ms Taylor to Ms T Reese of 15 May 2024 at 1:07 pm forwarding it	14 and 15 May 2024	1	40
11	Emails, Ms S Marriott to Logan Switch "Respiratory Nurse Educators"; the Appellant to Ms C Taylor; and the reply of Ms C Taylor	15 and 20 May 2024	2	41-42
12	Email thread, "Corey Shepherd 388372 Pay issues" - Payroll to the Line Manager copied to the Appellant; the Appellant to Payroll; Payroll to the Appellant	3 to 13 May 2024	2	43-44
13	Same thread continued - the Line Manager to the Appellant	21 May 2024, 12:33 pm	1	45

Tab	Document	Date	Pages	At
14	Email, Ms C Taylor to the Appellant copied to Ms T Reese, "Validation of Claims older than 3 months - Please sign"	28 May 2024, 8:36 am	1	46
15	myHR submissions report for the Appellant, produced by Metro South Health as Item 11	1 February to 31 May 2024	1	47
16	QH Leave Takings Report for the Appellant, produced by Metro South Health as Item 15	19 March 2024	1	48
17	Movement forms recording approved changes to working hours, approved by Mr S Hughes as delegate	27 February 2026	4	49-52
18	Movement form	17 April 2026	4	53-56
19	Movement form	9 June 2026	4	57-60
20	Letter, Metro South Hospital and Health Service to Commissioner Dwyer, reference K-LM26/729, signed by Ms N Cridland, Chief Executive	5 June 2026	5	61-65
21	Email, Ms L Forrest, Senior Consultant Human Resources, to the Appellant	7 July 2026	6	66-71
22	Consultation Paper - Proposed Rosters for Switchboard Services, Logan Hospital	November 2024	8	72-79
23	Consultation outcome - Proposed Rosters for Switchboard Services, Logan Hospital	December 2024	7	80-86
24	Email, Mr H Moran, Organiser, Together Queensland, to Ms C Jeffrey, Ms P Conaghan and the Appellant, "Switchboard Roster Feedback - For Delegates"	3 November 2025	2	87-88
25	Review Decision 69983, Workers' Compensation Regulator	24 October 2024	28	89-116
26	Notice to admit facts served by the Appellant on the Respondent, to which the response at Tab 27 was given	served before 18 February 2026	6	117-122
27	Respondent's response to that notice, signed by Ms R Matheson, Senior Appeals Officer	18 February 2026	4	123-126
28	myHR leave request history for Process Reference 15480560, produced by Metro South Health as Item 11 of the notice of non-party disclosure	20 February to 1 March 2024	1	127
29	Instrument of Human Resource Sub-Delegation, COVID-19 Pandemic Event - Paid Special Pandemic Leave, signed by Ms N Cridland, produced as Item 13	effective 5 December 2022	1	128

The documents follow in the order listed. No document has been annotated, highlighted or altered. Where only part of a document is reproduced, the balance is available on request.



Administration Officer, Switchboard Services (RD)

Job Ad Reference:			
Status:	Administration Officer, Switchboard Services		
Unit/Department:	Switchboard, Corporate Services		
Location:	Logan Hospital, Metro South Hospital and Health Service		
Contact:	Chloe Taylor (07) 2891 9190	Salary range:	
Classification:	A03	Closing date:	
Online applications:			
Email applications:	Chloe.taylor3@health.qld.gov.au		

****Applications from third parties will not be accepted****

Purpose of the role

To provide a highly effective and efficient communication service to the public and staff of the Logan Hospital within Metro South Hospital and Health Service.

The occupant of this position is required to work continuous shift work over the full 24-hour period, 7 days a week

Your key responsibilities

- This position reports to the Switchboard Manager, Corporate Services, Logan Hospital.
- Attend courteously to all telephone enquiries and clients' requests, displaying tact, diplomacy and interpersonal skills of a high standard when dealing with the public and members of staff. Maintain call queues to minimum at all times.
- Collate information and maintain Omnivista database and SharePoint to ensure information held within Switchboard Services is accurate and appropriate.
- Provide a service to support hospital staff in the allocation, coordination, and fault repair of all pager units for the hospital. This includes allocating pagers to hospital staff and submitting paging units for repair, maintaining database and registers with accurate and current information.
- Participate in the Emergency Response process by receiving emergency response notifications and distributing them to the appropriate response groups, dependent on the category of emergency, as per emergency code procedures, strictly adhering to protocols and timeframes.
- Ability to communicate at all levels and effectively establish and foster networks throughout the hospital to maintain accurate and current records for effective switchboard operations.
- Maintain discretion and exercise judgement where necessary to resolve problems within the scope of your role; in situations where precedence have not been set and procedures not defined.
- Interact in a sensitive, timely, effective, and appropriate manner with members of the general public and service providers, maintaining strict confidentiality at all times.
- The ability to multitask and operate under pressure, particularly where high volume call traffic is concerned.
- Contribute to a cohesive team environment by sharing information, supporting and assisting colleagues in a proactive manner.
- Ability to work effectively as an individual with limited supervision to meet deadlines and establish work priorities, and work as a member of a multi-disciplinary team in a changing environment.
- Ensure patients and other staff are treated fairly and with mutual respect and understanding, regardless of cultural, religious, ethnic and linguistic backgrounds, gender, identity, ages or abilities.

- Follow defined service quality standards, occupational health and safety policies and procedures relating to the work being undertaken to ensure high quality, safe services and workplaces.

Mandatory qualifications, professional registration and other requirements

- The position is a continuous shift working role. You must be able to work a roster which covers multiple shifts over a 24/7 period.
- Intermediate / advanced computer skills.
- Recent experience in the operation of a switchboard is desirable but not essential.
- **Vaccine Preventable Disease (VPD):** It is a condition of employment for this role for the employee to be, and remain, vaccinated against the following vaccine preventable diseases during their employment ([Health Employment Directive No. 01/16](#)):
 - measles, mumps, rubella (MMR)
 - varicella (chicken pox)
 - pertussis (whooping cough)
 - hepatitis B
- It is a condition of employment for this role for the employee to be, and remain, vaccinated against COVID-19 ([Health Employment Directive No. 12/21](#) and [Queensland Health Human Resources Policy B70](#)).

Are you the right person for the job?

How we do things is as important as what we do, therefore within the context of the responsibilities described under 'Your key responsibilities,' the ideal applicant will be someone who can demonstrate the following:

Communicate Effectively	▪ Strong communication and negotiation skills to understand customers' needs. Demonstrate respect, courtesy and fairness when interacting with patients, clients and colleagues.
Team Focus	▪ Value diversity of thought and include everyone within the team. Actively support a culture of diversity, trust, support and inclusion.
Technical Skills	▪ Intermediate / advanced computer skills.
Change & Quality Focus	▪ Be open to new ideas and approaches and be adaptable in the face of change. Support and participate in change initiatives. Follow procedures and processes understanding implications within a health care setting.
Work Ethics	▪ Be honest, ethical and professional. Work within boundaries of role.

How to apply

Please provide the following information to the panel to assess your suitability:

- A short written response (maximum 1-2 pages, dot points acceptable) on how your experience, abilities, knowledge and personal qualities would enable you to achieve the key responsibilities and meet the skill requirements of the role.
- Your current CV or resume, including two referees. You must seek approval prior to nominating a person as a referee.
- Late applications cannot be submitted online. For a late application to be considered, please arrange approval and submission via the contact person.
- Applications will remain current for 12 months or for the duration of the vacancy. Future vacancies of a temporary, full time and part time nature may also be filled through this recruitment process.
- Only those persons eligible to work in Australia may be employed by Queensland Health. Prospective employees are required to provide proof of identity and documentary evidence of their [right to work in Australia](#).

We are Metro South Health

We are the major public healthcare provider for Brisbane's south side, Logan, Redlands and the Scenic Rim. We operate five major hospitals and a range of community, specialty and state-wide healthcare services. We are renowned for teaching and research excellence.

We are truly dedicated people who care about our community. We come to work to make a difference. We save lives, changes lives and make the world a better place. We overcome the many challenges that come our way by going above and beyond and achieve the best possible outcomes by working together. We are bold, innovative, collaborative, inspiring.

We are part of and proud of the community we serve. We are *better together*.

Our values

Our values of Integrity, Compassion, Accountability, Respect, Engagement and Excellence, shape our culture. Our values guide our day-to-day decision making and are fundamental to what we care about as a health service, how we behave, how we interact with each other and provide care to the many patients who come through our doors every day.



INTEGRITY

We are authentic, truthful, and transparent, and strive for equity for all.



COMPASSION

We care for one another and ourselves with empathy, kindness and support.



ACCOUNTABILITY

We are accountable for our decisions, actions and behaviour.



RESPECT

We foster an environment of safety, civility, and inclusion.



ENGAGEMENT

We are one team working together to achieve our best outcomes.



EXCELLENCE

We empower each other and inspire innovation to deliver excellence.

Diversity and inclusion

We are the most culturally diverse area of Queensland with 28.5% of the community born overseas and 16% from non-English speaking countries. Our employees proudly reflect the community in which they work. At Metro South Health 20.5% of staff identify as coming from non-English speaking backgrounds.

Having a workforce that reflects and understands the needs and expectations of our community is important to delivering safe, kinder, and more inclusive care.

We recognise our strength comes from the diversity of our people and so we encourage people of all genders, ethnicities, ages, abilities, languages, sexual orientation and family responsibilities to apply.

We are committed to creating a diverse and inclusive workplace for our people and our community.

Flexible working arrangements

Metro South Health supports and encourages work-life balance for the mutual benefit of Metro South Health and its employees. Work-life balance is about a person's ability to manage their paid work commitments with their career goals, personal, community and cultural responsibilities. Through flexible working arrangements such as leave, flex time accrual or a reduction in working hours, there is an opportunity to match the individual's requirements with those of the workplace – delivering quality health services.

Additional information

For further information about this opportunity and the benefits and conditions of working at Metro South Health please refer to the **Information for Applicants** document attached with this Smart Jobs advertised vacancy.

So you have finished your Role Description, what's next? Check out the Recruitment Process Flow for next steps, [here](#)



Hello & Update

Ellen Stibbard <Ellen.Stibbard@health.qld.gov.au>

Tue 18/07/2023 12:56 PM

To: Anisha Ismail <Anisha.Ismail@health.qld.gov.au>; Carolyn Jeffrey <Carolyn.Jeffrey@health.qld.gov.au>; Cherrie Abbott <Cherrie.Abbott@health.qld.gov.au>; Chloe Taylor <Chloe.Taylor3@health.qld.gov.au>; Cory Harrison-Jones <Cory.Harrison-Jones@health.qld.gov.au>; Cory Shepherd <Cory.Shepherd@health.qld.gov.au>; Kaymarie Court <Kaymarie.Harvey@health.qld.gov.au>; Linda Hu <Linda.Hu@health.qld.gov.au>; Logan Switch <Logan.Switch@health.qld.gov.au>; LoganTelecomsManager <logantelecomsmanager@health.qld.gov.au>; Meegan Malsbury <Meegan.Malsbury@health.qld.gov.au>; Patricia Conaghan <Patricia.Conaghan@health.qld.gov.au>; Peter Fitzpatrick <Peter.Fitzpatrick@health.qld.gov.au>; Simon De Iacovo <Simon.Delacovo@health.qld.gov.au>; Sonya Willmann <Sonya.Willmann@health.qld.gov.au>; Stacey Sutton-Masson <Stacey.Sutton-Masson@health.qld.gov.au>; Taiha Zizek <Taiha.Zizek@health.qld.gov.au>

Hi Team,

Thought I would send out an email to say hi, and to give you all an update of what I am doing here.

My current role is a project role for the Switchboard team. It is a temporary position that predominantly focuses on fixing up the database that switchboard utilises. Yay! Its been a long time coming, and is very much needed. It is going to be a mammoth project that will take some time to complete.

While I am fixing up the database and all of its entries, I will be removing everyone's access to the database. If you need any entries amended, added or removed, please get the person wanting to update the entry to contact me directly, either via email or extension. I will then add this entry in. My working hours are every Tuesday and second Monday, 8:00 – 16:00 and my contact number is x6742. If something needs to be added during the other days of the week that I am not here, and it is urgent, please send it through to Chloe. If it is after hours (overnights, on the weekend or public holiday), it will have to wait until either Chloe or myself are back. If any of you find a number or an entry that needs to be amended, you can also contact me to do this. I have also removed the Contact & Number Changes book out of the room, as I would prefer, for now, to be sent all changes to my email.

I can also help people with most phone issues (diversions, name updates, programming of keys ect) and pager issues if they need help.

If you have any questions, please feel free to reach out, I will be working over in the back-up Switchboard office, if you wanted to come over and say hi. 😊

Thanks everyone,



Ellen Stibbard

Switchboard Telecommunications Coordinator

t. 07 2891 6742 | e. ellen.stibbard@health.qld.gov.au

Business Hours: Every Tuesday and second Monday 8:00 – 16:00

Switchboard Services | Digital Health Informatics

Level Two, Building One, Logan Hospital | **Metro South Health**

Cnr of Loganlea and Armstrong Roads, Meadowbrook, Qld, 4131

"Treat everyone with politeness and kindness, not because they are nice, but because you are."

Tammy Reese

Director HIMS | Health Information Management Services, Digital Health & Informatics

Logan Hospital, MSH | **Metro South Health**

Level 2, Building 1, Cnr Loganlea & Armstrong Rds, QLD, 4131

t. 07 3299 8934 | m. 0417986049

e. tammy.reese@health.qld.gov.au | metrosouth.health.qld.gov.au



Metro South Health acknowledges the traditional custodians of whose land and waters we provide health services and pays respect to Elders past, present and emerging.

From: Cory Shepherd <Cory.Shepherd@health.qld.gov.au>

Sent: Thursday, 10 August 2023 11:23 AM

To: Tammy Reese <Tammy.Reese@health.qld.gov.au>

Subject: Re: Increase of hours and Workplace issues

Hey Tammy,

I am confirming that we will have a meeting today at 13:15pm.

The overall purpose of the meeting is to improve the workplace and prevent ongoing conflicts.

I will go over in a bit more details in the meeting .

Thanks for your time.

From: Tammy Reese <Tammy.Reese@health.qld.gov.au>

Sent: Wednesday, August 9, 2023 9:54 am

To: Cory Shepherd <Cory.Shepherd@health.qld.gov.au>

Subject: Re: Increase of hours and Workplace issues

Thanks Cory for the reply. I am out of the office today but I will send you an appointment for tomorrow's meeting this afternoon. I have advised Chloe that you would like another replacement shift for Tuesday's shift and once she has reviewed the roster she is going you get back to you with another shift.

Kind regards
Tammy

Get [Outlook for iOS](#)

From: Cory Shepherd <Cory.Shepherd@health.qld.gov.au>

Sent: Tuesday, August 8, 2023 6:06:32 PM

To: Tammy Reese <Tammy.Reese@health.qld.gov.au>

Subject: Re: Increase of hours and Workplace issues

Hi Tammy,

I can confirm that I will be in tomorrow 0700-1500 9th August. I was also looking to pick up another shift if possible.

I also confirm us meeting at 1:15pm on Thursday.

Thank you.

Kind regards,

Cory Shepherd

Get [Outlook for iOS](#)

From: Tammy Reese <Tammy.Reese@health.qld.gov.au>

Sent: Tuesday, August 8, 2023 4:15:35 PM

To: Cory Shepherd <Cory.Shepherd@health.qld.gov.au>

Subject: RE: Increase of hours and Workplace issues

Good Afternoon Cory,

Thank you for that further information. I am sorry that you have these concerns but I am sure we can work through the various concerns you do have and try to find solutions where possible. How about we have a meeting Thursday at 1:15pm in my office? I will send you an appointment if that timing works for you. The various issues I believe you would like to discuss are as follows (and please feel free to add or change to the list below):

- Rostering concerns for yourself
- Distribution of roster timeframe concerns
- Concerns about Chloe
- Request for additional shifts.
- Etc. Any other items you might like to discuss.

With regards to an additional shift to replace the shift you did not work Monday due to a rostering error I believe Chloe offered you the shift tomorrow Wednesday 9th 0700-1500, thus can I ask if you are interested in picking up that shift? I believe you will then have 8 shifts this fortnight if you were to pick up that shift, thus can I ask were you looking for another shift on top of those 8 to replace the shift missed today also?

Kind Regards

Tammy

From: Cory Shepherd <Cory.Shepherd@health.qld.gov.au>

Sent: Monday, 7 August 2023 8:05 PM

To: Tammy Reese <Tammy.Reese@health.qld.gov.au>

Subject: Re: Increase of hours and Workplace issues

Hi Tammy,

Every attempt with Chloe has been made. To make this oversight is not acceptable anymore, Chloe would not roster a night shift on a day shift and the expectation is the same with days off. I have already discussed with other employees and know that Chloe would say an oversight. I suppose rostering me for 6 weeks on the 0700 start which is unheard of an oversight also.

Additionally, The date Corey has given leave from switchboard does not prevent Chloe from distributing the upcoming roster, it's an unreasonable time delay to break Q-Health contractual agreement with its employees.

I have never made a formal complaint; this Does not mean this department is without error. I have worked with and for Queensland Health since 2014 Security, Nursing, Scanning and Switchboard.

Issues that I have been dealing with include Chloe tearing out communications or concerns from the communications book, work has not been done with diligence, pagers have been distributed without updating the numbers, titles, or the paging system, the second stage IMT list of pagers completely disregarded and security with one pager the rely on when tests have been sent for months.

I have attended, informed, or actioned and resolved all issues. For going above board without expectations Chloe has come into office and either screaming at me for telling my colleagues to check numbers that have not been completed and signed for two months or demanding an explanation as to why I have not included her in a simple email to Ellen to check up on a pager. I clearly follow instructions outlined only days prior with all other employees not including Chloe in email regarding updates why would this happen?

I understand that when there is a problem or issue with the direct line manager the formal process is to reach out to the director of the department. If the screaming, yelling, coercion of the truth and starting rumours is ok with you it's not with me. I have received at-least 4-5 messages from Chloe which I have only taken as an insult and a coercion of the truth.

The last and final is the rumour Chloe has been telling my colleagues is I didn't show up for my shift last week because I wasn't requested in a way I would like.

I requested the shift, and it was withheld from me to see if Stacey responded, I then was not advised, and others received overtime is the answer.

This can be confirmed by Kay, and I should not need to explain myself to everyone again following the previous rumour made up.

I need some rest I spent all yesterday in a proposal for the department, liaising with those who seek to improve it.

I am rostered tomorrow morning but for the second fortnight I won't be without a rest day if I am not compensated for it.

I request that tomorrow's shift be removed from my roster in accordance with the current policy following a night shift. I should have an alternative day of work provided to me.

I do apologise and I do not want to have a negative impact, previously is was in support of Chloe wishing her the best but enough is enough, the manager should not be at the centre of the problem and the rumour starting.

Everything can be confirmed by my work colleagues, and I did mention to Chloe that I want to have a meeting with you, but the meeting I would like is structured towards a solution to the problems outlined.

Thank you

From: Tammy Reese <Tammy.Reese@health.qld.gov.au>

Sent: Monday, August 7, 2023 3:13 pm

To: Cory Shepherd <Cory.Shepherd@health.qld.gov.au>

Subject: RE: Increase of hours and Workplace issues

Good Afternoon Cory,

Thank you for your email.

Can we catch up with you are next on shift, so we can discuss further the various concerns that you have raised. I believe you on shift Thursday day shift and I am in the office also, thus please do not hesitate to drop in for a discussion when you can. I am free from 9 – 10:30 and then 12:00-4:30 Thursday.

As Chloe is your current line manager and as such you are required to continue to communicate with Chloe for work related issues, shift concerns, leave, etc. but your welcome to CC me into any emails your are sending to Chloe or raise any particular concerns with myself following trying to address these first with your line manager and as discussed above I would be happy to be able to have a chance to have a discussion with you further about the concerns you have raised.

I do realise in this instance there was a rostering error that was accidentally made by Chloe with regards to night shifts and days off and Chloe will endeavour not to make this mistake again and whilst I this should not happen in the future can I ask that you do let Chloe know of any issues you identify with regards to your rostered shifts as soon as possible, to ensure corrections can be made straight away.

With regards to your request for additional increase of hours that is something we will have to get further clarification on from HR and I will have to get back to you about this.

Kind Regards

Tammy

Tammy Reese

Director HIMIS | Health Information Management Services, Digital Health & Informatics

Logan Hospital, MSH | **Metro South Health**

Level 2, Building 1, Cnr Loganlea & Armstrong Rds, QLD, 4131

t. 07 3299 8934 | m. 0417986049

e. tammy.reese@health.qld.gov.au | metrosouth.health.qld.gov.au



Metro South Health acknowledges the traditional custodians of whose land and waters we provide health services and pays respect to Elders past, present and emerging.

From: Cory Shepherd <Cory.Shepherd@health.qld.gov.au>

Sent: Monday, 7 August 2023 12:21 PM

To: Chloe Taylor <Chloe.Taylor3@health.qld.gov.au>; Tammy Reese <Tammy.Reese@health.qld.gov.au>; Patricia Conaghan <Patricia.Conaghan@health.qld.gov.au>; Tracey Smith <Tracey.Smith8@health.qld.gov.au>

Subject: Increase of hours and Workplace issues

Importance: High

Hi Chloe,

I have not agreed to be rostered like this. I have had enough of the continuous gossiping and the discrimination against me.

The union has already passed the agreement for rest days after night shifts, and I have not agreed to be rostered any differently.

You may not issue rest days only on penalty shifts or the weekend and for the second fortnight in a row, I will not be working a Monday morning with less than a 24-hour break following a night shift.

I will be in contact with Tammy directly regarding a lot of the current workplace issues.

Conflicting interests
Escalation and gossiping
Initiating workplace rumours
Adverse workplace behaviours
Workplace bullying and Harassment.

You are not in the office Monday to Friday and disregard all agreements in the workplace. Please give me some space and stop with any further communication as I have had enough of it.

Clear breach of the EB11 agreement.

9.13 Display of rosters

9.13.1 All employees (excluding employees where schedule 3 applies) shall be provided with a roster prescribing their pattern of work. **The roster shall be published at least 14 days in advance of each roster cycle.** The roster will be made available electronically, or where not possible, displayed in a convenient place accessible by all employees.

To withhold a roster to facilitate the needs of any staff member will be directly discriminating against all other employees. If an employee needs to take leave, then the available shifts on display are to be allocated to staff members to policy, this roster is not to be withheld for a distribution of your choosing.

10.6 Workplace Behaviour

10.6.1 The employer recognises that workplace bullying is a serious workplace issue which is not acceptable and must be eliminated.

10.6.2 All employees have the right to be treated fairly and with dignity in an environment free from adverse behaviours such as intimidation, humiliation, harassment, victimisation, discrimination, and bullying.

10.6.3 The employer recognises that adverse behaviours such as these are serious workplace issues, which are not acceptable and must be eliminated from the workplace.

10.6.4 The Code of Conduct for the Queensland Public Service applies to all employees covered by this Agreement. If it is substantiated that an employee is found to have been involved in the above adverse behaviours, this may be a breach of the Code of Conduct and they may be subject to a disciplinary process.

10.6.5 The employer supports the accepted industrial principle that all employees have the right to raise concerns with their employer about issues of bullying or workplace behaviour without fear of victimisation. Unions may refer instances of alleged victimisation directly to the EB11IG for attention.

10.6.6 The employer is committed to protecting and improving the health and wellbeing of all employees and their immediate family by providing employee assistance.

Furthermore, to meet the demands and the additional hours of the department both Patricia Conaghan and I would

like to formally adhoc an additional 2 shifts per fortnight. This formal request and information can be confirmed by Patricia Conaghan via email Patricia.Conaghan@health.qld.gov.au

11.7 Additional Permanent Hours for Part-time Employees

11.7.1 Part-time employees, following approval, may work more than their substantive (contracted) hours on an ad-hoc or temporary basis. Where an employee works more than their substantive (contracted) hours on a regular basis over a 12 month period, the employee may request an amendment to their substantive permanent part-time hours to reflect the increased hours worked. Such requests should not be unreasonably refused. 11.7.2 Any agreed permanent increase to an employee's substantive part-time hours is limited to a maximum of 64 hours per fortnight for part-time employees employed under the *Hospital and Health Service General Employees Award - State 2015*, or full-time.

5.2 Process to Address Absences within Operational Services

5.2.1 All absences (planned and unplanned) within Operational Services will be backfilled. The options to backfill may include but are not limited to:

(a) Offering additional ordinary hours to Part-Time employees,

- (b) Offering additional work to Casual employees,
- (c) The application of Relief Pool staff,
- (d) The use of Overtime,
- (e) Utilisation of temporary engagement (e.g. extended period of absence).

Thank you,

Cory Shepherd.

Renee Matheson

From: Tammy Reese <Tammy.Reese@health.qld.gov.au>
Sent: Friday, 8 September 2023 11:42 AM
To: Cory Shepherd
Subject: RE: Increase of hours and Workplace issues

Hi Cory,

Thanks very much for your response and for your draft roster. We are happy to look at any rosters but we do ask that they fit the staffing lines we will have (e.g. we will shortly have 7 full-time lines, one 0.8 FTE, 3 x 0.7, 2 x 0.6 FTE and one 0.5 FTE), , must be fair and equitable to all staff ensuring fair distribution of penalties and be compliant with all industrial requirements. Thus if you do want to still present a roster could you please just review your roster and modify as required to suit the FTE we will have for Switch and you may just need to add some additional shifts to ensure there are enough for everyone and you will see from my released draft roster yesterday what sorts of shifts these might be e.g. an additional 12:00 to 8pm shift and an additional 0600-1400, etc. and then resend through to myself or HR. I am currently on leave until 3.10.23 (Marilla Fraser will be backfilling in my role) but upon my return I would be happy to catch up with you if you would like to talk further about your draft roster.

I am glad to hear things seem to be going well with Chloe and I am also glad to hear you applied for the additional shifts through the recent EOI.

Kind Regards
Tammy

From: Cory Shepherd
Sent: Monday, 4 September 2023 7:21 AM
To: Tammy Reese
Subject: Re: Increase of hours and Workplace issues

Good morning, Tammy,

I apologize for the delay in getting back to you regarding the rotational roster. I have attached a copy of the spreadsheet for you to view. I include many versions of the roster that you can change or edit as you please.

I appreciate your time and the information you have provided me regarding the grievance steps. I endeavour to resolve the issues I have raised, and I have also spoken to Chloe. We are seemingly on the path to working in a beneficial way and hopefully will not need to go through this process..

Moreover, the concerns I raised were more directed towards finding a solution for a positive workplace and a team environment. The time the roster is released is not the biggest concern. The issues are meeting the agreements and the purpose of the delay being discriminatory actions.

Additionally, I have also sent Chloe my official letter of response last week to increase my working hours prior to the cut-off date. I have attached it to this email.

I confirm that I am able and willing to work any roster that is presented to me, including the current schedule with full 24-hour availability. I have previously been employed offshore for four weeks at a time, working 12-hour shifts. Two weeks of those shifts were day shifts, and the other two weeks were night shifts. I have no issues with shift work.

Thank you,
Cory

From: Tammy Reese <Tammy.Reese@health.qld.gov.au>

Date: Tuesday, 29 August 2023 at 6:57 pm

To: Cory Shepherd <Cory.Shepherd@health.qld.gov.au>

Subject: RE: Increase of hours and Workplace issues

Hi Cory,

I am just following up with regards to our conversation on the 10.8.23. As discussed at this meeting you raised a number of roster concerns and I am now hoping you have had a chance to discuss these concerns with Chloe and please do discuss know you can discuss any concerns roster issues with Chloe as your line manager but please do not hesitate to raise any future roster concerns with myself you have tried that option or if you need any assistance to raise these concerns. Please note I encourage you to look at the roster when released in the first few days after it is released and get back to Chloe asap with any concerns you may have with regards to your rostering.

I noted you had concerns with the roster timeframe being released and upon review of previous 3 months of release dates for rosters and the majority were released in a timely manner with most well in advance of 14 days as per 9.13.1 but I do acknowledge your concern regarding that roster beginning 21.8.23 was sent out 1 day late and whilst we do aim ensure to comply with this there may be the rare occasions when circumstances may prevent us from doing so. However, please note we will try to ensure wherever possible that in future we do always comply with 9.13.1 and give Switchboard staff as much notice as possible of upcoming rosters.

In relation to your request for additional shifts where you confirmed in our discussion that you were potentially interested in increased perm full-time hours and you will see that the EOI has been opened up again for staff to apply if they do wish to request additional permanent FTE, just in case staff like yourself missed applying the first time, thus please do feel free to apply if you do want to request additional permanent hours.

With regards to the future roster and the draft you showed me as we discussed I would be happy to review your roster, so please feel free to send me a copy.

As discussed whilst I do recommend you do try and raise any concerns you do have with your line manager being Chloe and then myself, or if you have and your not happy with the outcome, you do have the option of lodging a Grievance directly with HR. As discussed at our meeting I have attached the [HR Policy E12 Individual Employee Grievances](#), which contains information regarding the grievance process.

You may submit your grievance to LBH_HR@health.qld.gov.au. Specifically, we ask that you provide the following details:

- Information relating to the instances that have aggrieved you. Who was involved (witness details?), what occurred, when and where did it occur?
- What outcome you are seeking from the grievance process?
- What attempts you have made to resolve the grievance locally?

As also discussed, as a Metro South Health employee you are able to access the Employee Assistance Program (EAP). Please see below contact information:

- Employee assistance offers a confidential counselling service which is free of charge to all employees of Metro South Health for up to six sessions per calendar year. Access to this service is by self-referral. Please contact LifeWorks on 1800 604 640 or Benestar on 1300 360 364.

Lifeworks website: [Employee Engagement & Wellbeing | Employee Assistance | LifeWorks AU](#)

Benestar website: [Contact Us | Benestar](#)

Please do not hesitate to let me know if you have any questions at all or if there is anything else I can assist you with.

Kind Regards
Tammy

Cory Shepherd
Switch Board Operator
Metro South Queensland Health
Cory.shepherd@health.qld.gov.au

31 Aug 2023

Subject: Request to Increase Working Hours to Full Time Rotational Roster

Dear Chloe,

I am writing to express my interest in the increase of working hours and working Full Time hours within Switch Board Department in Qld Health, and to outline how my experience, abilities, knowledge, and personal qualities make me an ideal candidate for this role. Please find below a summary of my qualifications in relation to the key attributes and responsibilities, as well as the key skill requirements.

Experience and Abilities:

- I possess extensive experience in operating equipment and performing administrative duties. This background has equipped me with a solid foundation in effective communication and record-keeping. Additionally, I am highly skilled in various programs and databases, including the Cerner program, Alcatel Switchboard Console and Database, HBCIS Database, EDIS Emergency Department Web Database, Spok Messenger program, as well as numerous other systems and equipment. Furthermore, I possess exceptional problem-solving skills that allow me to respond effectively even in high-pressure situations utilizing alternative communication systems during emergency or security response situations.

Personal Qualities:

- Ethical decision-making, effective prioritization, and a responsible work ethic are some of the qualities that define my professional character. I am diligent, an effective listener capable of coordinating the operations of the Switch room, handling difficult calls, managing conflicts.

Teamwork:

- I strongly believe in the power of communication and collaboration. Forging a cohesive team environment is essential to me. Therefore, I actively share information with colleagues while offering proactive support and assistance whenever needed. I am committed to adhering to Queensland Health policies, procedures, standards, and relevant legislation.

Interdisciplinary Workplace:

- My approach towards interdisciplinary workplace settings is marked by understanding and respect for all stakeholders involved. Clear communication channels alongside setting expectations are vital components for effective conflict management.

Compassion:

- As a patient-focused professional providing service delivery with self-awareness and resilience. The needs of patients remain at the forefront throughout ambiguity or complexity that may arise.

High Performance:

- My action-oriented mindset and drive for results have consistently shown through constant review, understanding, and research. This commitment aligns with Metro North Health's vision and purpose of exceeding expectations for patients and stakeholders.

Integrity:

- I prioritize accountability and work diligently to ensure high ethical standards in decision-making and work output. Advocating the education for a positive and fair working environment is of utmost importance to me.

Considering my qualifications, I believe that I am well-equipped to undertake the responsibilities associated within the department. I have also expressed my willingness to take on additional night shifts as part of the roster and advocacy to become a union representative for the switchboard department. I look forward to contributing further to our team and services.

Yours sincerely,

Cory Shepherd

From: [Chloe Taylor](#)
To: [Cory Shepherd](#)
Cc: [Tammy Reese](#); [Tracey Smith](#)
Subject: RE: Increase of hours and Workplace issues
Date: Monday, 7 August 2023 1:43:04 PM
Attachments: [image001.gif](#)
[image002.gif](#)
[image003.gif](#)
[image004.gif](#)
[image005.gif](#)
[image006.png](#)

Hi Cory,

Thank you for your email, I acknowledge your concerns and I will be following up on your email. Tammy will be in contact to arrange a time to meet and discuss any issues you may like to raise.

My sincere apologies about your rostered Monday 7th 0700-1500 shift, I can confirm this was an oversight. I did release PP30 roster on Monday 19th July, may I please ask why you have not made myself or Tammy aware of this rostering error until now? Would you like me to roster you off tomorrow Tuesday 8th 0700-1500 to give you the required rest period and offer you Wednesday 9th 0700-1500 shift? Please let me know.

Kind regards,

Chloe

Chloe Taylor

A/Switchboard Manager | Switchboard Services

Logan Hospital | **Metro South Health**

Level 2, Building 1, Cnr Loganlea and Armstrong Roads QLD 4131

t. (07) 2891 9190

e. chloe.taylor3@health.qld.gov.au | metrosouth.health.qld.gov.au



Renee Matheson

From: Tammy Reese <Tammy.Reese@health.qld.gov.au>
Sent: Monday, 7 August 2023 5:11 PM
To: Chloe Taylor
Subject: Rostered shifts Cory S. past 8 months.xlsx
Attachments: Rostered shifts Cory S. past 8 months.xlsx

Hi Chole,

Please see the rostered shifts for Cory for the past 8 or so months. Could I please get you to check the actual rosters worked to see if the number of shifts he ended up picking up are added and the shifts he worked are correct. This just gives us a guide as to how he has been rostered the past 8 months and also info about whether he routinely picks up additional shifts, in case he wishes to make a claim for additional shifts.

With regards to recent email communication you have had with Cory can I ask if you can send me an email of your recent communication with Cory about contacting yourself about missed shifts, as I could not find a copy of this email.

Kind Regards
Tammy

From: [Chloe Taylor](#)
To: [Cory Shepherd](#)
Subject: Approved - Permanent Full Time FTE
Date: Wednesday, 27 September 2023 1:52:00 PM
Attachments: [image006.png](#)

Hi Cory,

Just giving you an update on your application for Permanent Fulltime hours, I am very pleased to advise you that this has been approved.

I am currently in the process of completing Roster PP10 and would like to start you on this roster as fulltime. You will find in the next coming days an EMR movement form for your approval in MyHr.

Please let me know if have any questions or concerns, and happy to commence Full-time hours from the 16th October 2023.

Kind regards,

Chloe

Chloe Taylor

A/Switchboard Manager | Switchboard Services

Logan Hospital | **Metro South Health**

Level 2, Building 1, Cnr Loganlea and Armstrong Roads QLD 4131

t. (07) 2891 9190

e. chloe.taylor3@health.qld.gov.au | metrosouth.health.qld.gov.au



Healthcare Heroes

Metro South Health acknowledges the traditional custodians of whose land and waters we provide health services and pays respect to Elders past, present and emerging.

Fwd: Afterhours Oncall Process - Switchboard

1 message(s) · print-style export (Graph)



[1/1] Fwd: Afterhours Oncall Process - Switchboard

From: Cory Shepherd <coryshepherd1@hotmail.com>
To: Amy Mo <Amy.Mo@workcoverqld.com.au>
Date: 2024-08-29 06:02:50
Subject: Fwd: Afterhours Oncall Process - Switchboard

On April 15, 2024 with no consultation after hours on call delegation was removed from switchboard as both Ellen and Chloe decided effective immediately it was a necessity that they were on call and was done without consulting those who were taking the call and actioning the decision.

From: Chloe Taylor <Chloe.Taylor3@health.qld.gov.au>
Sent: Monday, April 15, 2024 12:39 PM
To: Logan Switch <Logan.Switch@health.qld.gov.au>
Cc: Tammy Reese <Tammy.Reese@health.qld.gov.au>; pmfitzpatrick@optusnet.com.au <pmfitzpatrick@optusnet.com.au>; Deborah Neville <Deborah.Neville@health.qld.gov.au>; Anisha Ismail <Anisha.Ismail@health.qld.gov.au>; Carolyn Jeffrey <Carolyn.Jeffrey@health.qld.gov.au>; Cherrie Abbott <Cherrie.Abbott@health.qld.gov.au>; Cory Shepherd <Cory.Shepherd@health.qld.gov.au>; Ellen Stibbard <Ellen.Stibbard@health.qld.gov.au>; Jesse Kendrick <Jesse.Kendrick@health.qld.gov.au>; Jude Nelson <Jude.Nelson@health.qld.gov.au>; Kay Marie Court <KayMarie.Court@health.qld.gov.au>; Linda Hu <Linda.Hu@health.qld.gov.au>; Meegan Malsbury <Meegan.Malsbury@health.qld.gov.au>; Patricia Conaghan <Patricia.Conaghan@health.qld.gov.au>; Peter Fitzpatrick <Peter.Fitzpatrick@health.qld.gov.au>; Richard Parry <Richard.Parry@health.qld.gov.au>; Simon De Iacovo <Simon.DeIacovo@health.qld.gov.au>; Sonya Willmann <Sonya.Willmann@health.qld.gov.au>; Stacey Sutton-Masson <Stacey.Sutton-Masson@health.qld.gov.au>; Taiha Zizek <Taiha.Zizek@health.qld.gov.au>
Subject: Afterhours Oncall Process - Switchboard

Hi Team,

Please find attached updated rosters for PP23 & PP24. You will see that Ellen and myself have added Afterhours on call, the days that are highlighted in purple show who is on call after hours. If you urgently need to contact myself or Ellen after hours, first instance please call/text on mobile otherwise email if non urgent.

Process during office hours remains the same, please contact myself through switch/office or mobile unless otherwise advised.

This new process is effective from today.

Any questions please don't hesitate to contact me,

Kind regards,
Chloe

Chloe Taylor
A/Switchboard Manager | Switchboard Services
Logan Hospital | Metro South Health
Level 2, Building 1, Cnr Loganlea and Armstrong Roads QLD 4131
t. (07) 2891 9190
e. chloe.taylor3@health.qld.gov.au | metrosouth.health.qld.gov.au

Metro South Health acknowledges the traditional custodians of whose land and waters we provide health services and pays respect to Elders past, present and emerging.

[Message has attachments ? export with --attachments to save files]



Roster Concerns

WCRS

Workers' Compensation Regulatory Services
worksafe.qld.gov.au



Renee Matheson

From: Tammy Reese <Tammy.Reese@health.qld.gov.au>
Sent: Wednesday, 8 May 2024 9:08 AM
To: Cory Shepherd
Subject: RE: Roster Concerns

Hi Cory,

Thanks for raising these observations and concerns. I am following up with regards to these with HR for further advice and I will get back to you asap with a response.

Kind Regards
Tammy

From: Cory Shepherd
Sent: Wednesday, May 1, 2024 1:18 PM
To: Tammy Reese
Subject: Re: Roster Concerns

Dear Tammy,

I am writing to further discuss some concerns and observations regarding our operations and rostering process.

Firstly, I appreciate your recent update and the clarity it provided regarding consecutive shifts. I realized that I had been mistakenly referencing this section and it did not support my points towards SECTION 4. HOURS OF WORK 10.4.1 in the OPERATIONS MANUAL.

FOR THE TIME BEING ASSOCIATED WITH SUCH HIGHER DUTIES.

SECTION 4: HOURS OF WORK

10.4.1 Hours of Work

(1) Hours of duty and relevant breaks and off-duty periods shall be in accordance with the CASA mandated fatigue management requirements (CAO 48.1) or Fatigue Management System (FRMS), as adopted by QGAir and applicable at the time. These will be specified in the QGAir Operations Manual.

(2) When a pilot is required to fly in an off-duty period, and the pilot is willing and able to undertake such duty this time may be converted to duty time and time off shall subsequently be granted. Such time off must be taken within twelve months of the day on which the time was worked.

SECTION 5: LEAVE

This section includes a toolkit required by management to manage fatigue and implement appropriate protocols and adhere to the workplace health and safety act. I have attached an image for your reference. It is incumbent upon management not only to prevent an increase in fatigue but also to actively reduce it. This aligns with the best practices outlined and, in the studies, I uploaded to Microsoft Teams over six months ago.

Other shift-related factors that should also be evaluated include shift swaps and remote/phone-access on call shifts.

		Low (1 points)	Moderate (2 points)	High (3 points)	Points
1.	Rostered shift length	<10 hours/week	10 – 14 hours/week	>14 hours per week	
2.	Actual hours worked per week (including overtime)	<48 hours	48-55 hours	>55 hours	
3.	Time worked between breaks	<2 hours	<5 hours	≥5 hours	
4.	Night shift	None	<4 days/week	>4 days per week	
5.	Rostered days off	2 days per week	1 day per week	<1 day per week	
6.	Breaks between shifts	12 hours or more	10 hours or more	<10 hours	
7.	On-call/recall shifts	1 – 2 days per week	3 – 4 days per week	>4 day per week	
8.	Shift rotation	Forward/consistent	Forward/inconsistent	No consistency or direction	
					TOTAL RISK SCORE
0 – 8: LOW RISK		9 – 16: MODERATE RISK		> 16 POINTS: HIGH RISK	

Figure 7: Roster risk assessment matrix

For instance, an increase in night shifts correlates with heightened fatigue levels. Similarly, inconsistent backward rotations exacerbate this issue. I forwarded the toolkit to Chloe last week for review and action but have yet to receive feedback and feel she is not interested.

Furthermore, upon reviewing the rosters in light of the Industrial Relations Act, it is clear that a defined process must be adhered to until a mutually agreed-upon rostering system is in place, in accordance with our individual award (Industrial Relations Section 6: Rosters). I have attached the relevant section below for your reference.

I believe it is crucial to address the ongoing issue of roster control and request facilitation. The numerous errors during Chloe's tenure in management have unfortunately contributed to a decline in our department's performance.

6. Rosters

- (a) Rosters will be worked over a 9 day fortnight where rostered hours are a maximum of 9 hours per day.
- (b) Unless otherwise agreed, rosters will be implemented as follows for the life of this Agreement.

	S	M	T	W	T	F	S	S	M	T	W	T	F	S
Saturday Roster	O	O	X	X	X	X	X	O	O	RDO	X	X	X	X

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Sunday Roster	X	X	X	X	X	O	O	X	X	X	RDO	O	O
Monday to Friday	O	X	X	X	X	X	O	O	RDO	X	X	X	O

- Where "O" means rostered off and "X" means rostered on.
 - Where industrial parties agree, individual teams may, by mutual agreement, perform work in accordance with alternative rosters as provided for in the Award.
- Program management agree to provide notification to the Union and all affected staff of proposed changes to rosters (ie. changes between Saturday, Sunday and Monday/Friday rosters). Management will make all attempts to provide such notification at least one month prior to the earliest possible date of effect.
- Except as provided in clause 8, if work is required to be performed on an RDO, overtime rates shall apply.

I am confident that by addressing these issues, we can improve our operational efficiency and enhance the well-being of our team.

I appreciate your attention to these matters. I am eager to contribute to the development of a roster that ensures equitable distribution of shift assignments. I propose to undertake this task over a span of three months. During this period, I can provide detailed insights and reflect wage percentages accurately. For instance, an employee with a .7 designation could anticipate receiving 70% of a full-time employee's wage, and so forth.

Best regards,

Cory

From: Tammy Reese <Tammy.Reese@health.qld.gov.au>

Date: Friday, 26 April 2024 at 1:52 PM

To: Cory Shepherd <Cory.Shepherd@health.qld.gov.au>

Subject: Roster Concerns

Good Afternoon Cory,

In follow up to our meeting on the Tuesday 16th April where you raised concerns about an issue you had with roster out with the previous fortnight's roster line for yourself ending with nights then 3 days off and then the new draft roster with you returning to nights again. I discussed that Chloe was aware of the concern you had with regards to the nights and then nights again on the new roster for the fortnight and Chloe was working to fix this error and would get in touch with you about what alternative shifts she could offer. You also raised a few recent issues you had with how you had been rostered by your line manager and you stated that due to section 15.2(c) Shift work arrangements of the *Award* that you wanted to only be rostered in future as per this section:

Hospital and Health Service General Employees (Queensland Health) Award – State 2015

15.2 Shift work arrangements

- (a) Shift work may be introduced to meet operational requirements. Such shift work shall be worked in accordance with a roster mutually agreed between the employer and the union, in consultation with the employees directly affected. Any discussions concerning the introduction of shift work or roster variations shall be conducted in accordance with the provisions of clause 6.2 of this Award.
- (b) A roster setting out the employee's days of duty and starting and finishing times on such days shall either be displayed in a convenient place or made available electronically to employees at least one calendar week in advance of the roster cycle, provided that notification by 1300 on Monday will be sufficient notification for the week commencing the following Monday.
- (c) A shift worker shall not perform more than two consecutive shifts (e.g. day shift/afternoon shift; afternoon shift/night shift).
- (d) Changes within a roster shall be by agreement between the employer and the employee concerned but, failing agreement, 24 hours' notice of any change in the roster must be given by the employer or double time is to be paid for the employee's next shift.

I think your interpretation of this section is that you only wished to be rostered not more than two consecutive shift types in a row, e.g., Day, Day, or afternoon, afternoon, etc., per fortnight. I discussed that my interpretation of this section was different, and I thought it meant no more than 2 shifts in one day for a shift worker but that I may not necessarily be correct in my interpretation thus we would need to seek HR advice on this. I asked Chloe to follow upon what should be the interpretation of this section with HR and as per the email advice received that I believe Chloe has now sent to you regarding this it states "...the meaning of consecutive shifts is in relation to shifts on the same day, they cannot work 2 shifts in one day. It is not in relation to consecutive days." Thus, based on this interpretation I do believe Chloe's rostering is in compliance with this particular section of the award. I would however be happy to discuss this or any other issues further with if you like just to let me know and we can schedule a time next week or when your next available. If you have immediate issues with the offered roster line for yourself, please discuss this with your line manager in the first instance but your welcome to escalate any concerns to myself. As per the HR email your welcome to discuss any award provisions further with HR as to their interpretation.

Kind Regards

Tammy

Tammy Reese

Director HIMs | Health Information Management Services,

Digital Health and Informatics,

Logan Hospital | **Metro South Health**

Level 2, Building 1, Cnr Loganlea & Armstrong Rds, QLD 4131

t. 07 32998934 | m. 0417 986 049

e. tammy.reese@health.qld.gov.au | metrosouth.health.qld.gov.au



Renee Matheson

From: Tammy Reese <Tammy.Reese@health.qld.gov.au>
Sent: Monday, 20 May 2024 4:07 PM
To: LBH_HR
Subject: FW: Roster Concerns
Attachments: qh-gdl-401-3.3 (3).pdf

Good Afternoon HR,

I am just following up on this query I raised a little while ago in relation to a email I had received from a staff member about our rostering practices. Thus if someone might be able to give me a call regarding these staff concerns, that would be great. Happy to chat via Teams if that is easier any time.

Kind Regards
Tammy

From: Tammy Reese
Sent: Friday, May 10, 2024 2:08 PM
To: Mackenzie Pritchard
Subject: FW: Roster Concerns

Good Afternoon Mackenzie,

Welcome to your new role and I think you are our new HR consultant.

I was wondering if you might be able to assist me with some wording of how best to respond to the email below, I received from one of our Switchboard staff Cory Shepherd. Chloe is our Switchboard Manager, but staff do occasionally send queries direct to myself like in this instance. Anyway, I believe Chloe may have already sought some advice from yourself in how best to answer some of the emails complaints/concerns from Cory.

I am not 100% sure what Cory is hoping will be outcome from his email, but I think he is trying to raise that he has concerns over how his manager is rostering for the Switchboard team and how it is impacting on staff fatigue, or more specifically his fatigue. The first extract I am a little confused about as I note this is from the Civil Aviation Safety Authority – Safety Management Systems for aviation: a practical guide and I am not sure how applicable this is to rostering of our Switchboard Service; however I think this document is referenced in the QH Fatigue Risk Management Systems – Implementation Guidelines. Anyway, as per Cory's second extract detailing the Roster Risk assessment Matrix, Chloe and I have run thought this and we say at best there would be a rating of 11 which is moderate and thus have tried to implement what rostering measures we can include and certainly we with the new draft roster we are hoping to implement shortly for Switchboard minimising fatigue for staff has been something we have tried to do throughout the new roster. I acknowledge there has been a few rostering errors made by Chloe with regards to Cory's line in past rosters, but this were just accidental mistakes and with the new roster to be implemented shortly should solve the majority of rostering errors. Also, the current Business Case for Change for implementing a new roster for Switchboard is the process we do need to follow as per the Industrial Relations Act to modify a department's roster and that it is in accordance with the relevant award.

With regards to the 3rd attachment in the document I don't understand what this is of or where these provisions have come from, thus I think I need to go back to Cory and ask the origins of this roster, as it does not follow our shifts at all or rostering patterns. I do believe Cory is asking if he can contribute to developing a new roster for equitable distribution of Shifts but I believe this work has already been conducted through our Business Case for Change and Roster review for Switchboard and that was all the staff consultation about draft rosters already and staff already had the opportunity to provide suggested rosters for Switchboard and Cory did actually provide a few as part of that feedback process but unfortunately the two he provided were not fair and equitable to part-time staff in terms of shift penalties (proportionate on their FTE) thus Cory's roster will not be considered, however Cory

is not yet aware of this as we cannot advise what is the roster we are planning on going with before we send out the implementation plan for this business case for change. But essentially the roster consultation process has been going on for 2 years now and we are almost at the end but that the draft roster which is being proposed does ensure there is as fair as possible equitable distribution of penalties based on FTE.

Sorry for the long email but happy to accept any advice on how best to respond to Cory's email below. Please call me or Teams me if you have any queries at all in regard to this.

Kind Regards
Tammy

Tammy Reese

Director HIMIS | Health Information Management Services,
Digital Health and Informatics,
Logan Hospital | **Metro South Health**
Level 2, Building 1, Cnr Loganlea & Armstrong Rds, QLD 4131
t. 07 32998934 | m. 0417 986 049
e. tammy.reese@health.qld.gov.au | metrosouth.health.qld.gov.au



From: Cory Shepherd <Cory.Shepherd@health.qld.gov.au>

Sent: Wednesday, May 1, 2024 1:18 PM

To: Tammy Reese <Tammy.Reese@health.qld.gov.au>

Subject: Re: Roster Concerns

Dear Tammy,

I am writing to further discuss some concerns and observations regarding our operations and rostering process.

Firstly, I appreciate your recent update and the clarity it provided regarding consecutive shifts. I realized that I had been mistakenly referencing this section and it did not support my points towards SECTION 4. HOURS OF WORK 10.4.1 in the OPERATIONS MANUAL.

for the time being associated with such higher duties.	
SECTION 4: HOURS OF WORK	
10.4.1 Hours of Work	
(1)	Hours of duty and relevant breaks and off-duty periods shall be in accordance with the CASA mandated fatigue management requirements (CAO 48.1) or Fatigue Management System (FRMS), as adopted by QGAir and applicable at the time. These will be specified in the QGAir Operations Manual.
(2)	When a pilot is required to fly in an off-duty period, and the pilot is willing and able to undertake such duty this time may be converted to duty time and time off shall subsequently be granted. Such time off must be taken within twelve months of the day on which the time was worked.
SECTION 5: LEAVE	

This section includes a toolkit required by management to manage fatigue and implement appropriate protocols and adhere to the workplace health and safety act. I have attached an image for your reference. It is incumbent upon management not only to prevent an increase in fatigue but also to actively reduce it. This aligns with the best practices outlined and, in the studies, I uploaded to Microsoft Teams over six months ago.



Queensland Health

Fatigue risk management systems

Implementation guideline QH-GDL-401-3.3:2021



Queensland
Government



Fwd: MASPER process

From Patricia Conaghan <Patricia.Conaghan@health.qld.gov.au>

Date Wed 15/05/2024 1:20 PM

To Cory Shepherd <Cory.Shepherd@health.qld.gov.au>

Get [Outlook for iOS](#)

From: Chloe Taylor <Chloe.Taylor3@health.qld.gov.au>

Sent: Thursday, May 9, 2024 10:15:59 AM

To: Logan Switch <Logan.Switch@health.qld.gov.au>

Cc: pmfitzpatrick@optusnet.com.au <pmfitzpatrick@optusnet.com.au>; Anisha Ismail

<Anisha.Ismail@health.qld.gov.au>; Carolyn Jeffrey <Carolyn.Jeffrey@health.qld.gov.au>; Cherrie Abbott

<Cherrie.Abbott@health.qld.gov.au>; Cory Shepherd <Cory.Shepherd@health.qld.gov.au>; Deborah

Neville <Deborah.Neville@health.qld.gov.au>; Ellen Stibbard <Ellen.Stibbard@health.qld.gov.au>; Jesse

Kendrick <Jesse.Kendrick@health.qld.gov.au>; Jude Nelson <Jude.Nelson@health.qld.gov.au>; Kay Marie

Court <KayMarie.Court@health.qld.gov.au>; Linda Hu <Linda.Hu@health.qld.gov.au>; Meegan Malsbury

<Meegan.Malsbury@health.qld.gov.au>; Patricia Conaghan <Patricia.Conaghan@health.qld.gov.au>; Peter

Fitzpatrick <Peter.Fitzpatrick@health.qld.gov.au>; Richard Parry <Richard.Parry@health.qld.gov.au>; Simon

De Iacovo <Simon.Delacovo@health.qld.gov.au>; Sonya Willmann <Sonya.Willmann@health.qld.gov.au>;

Stacey Sutton-Masson <Stacey.Sutton-Masson@health.qld.gov.au>

Subject: MASPER process

Hi Everyone,

I have included below email trail for your reference. There have been many ongoing issues raised by the MASPER and the medical department about calls being transferred to the wrong medical teams.

To try and eliminate this can I please ask that all calls that switch transfer to the MASPER phone #5223 are being introduced.

This way we can determine whether they are coming from switchboard or other departments. We will review in the next month and if I receive any more reports from the MASPER reg, I will be requesting that the switch team keep a log for all calls that we receive from the MASPER or regarding MASPER for our records.

Thanks so much team for your support with this,

Kind regards,
Chloe

Chloe Taylor

A/Switchboard Manager | Switchboard Services

Logan Hospital | [Metro South Health](#)

Level 2, Building 1, Cnr Loganlea and Armstrong Roads QLD 4131

t. (07) 2891 9190

e. chloe.taylor3@health.qld.gov.au | metrosouth.health.qld.gov.au



Metro South Health acknowledges the traditional custodians of whose land and waters we provide health services and pays respect to Elders past, present and emerging.

From: Chloe Taylor

Sent: Thursday, May 9, 2024 9:20 AM

To: Vivian Kwok <Vivian.Kwok@health.qld.gov.au>; Pan Jane Wong <PanJane.Wong@health.qld.gov.au>

Subject: RE: Switchboard issues 5 May, 7 May, and 8 May

Good morning Vivian,

Thank you so much for taking the time to document and address these issues which are occurring.

To try and eliminate these issues that are being raised, I have now asked the team to introduce all calls that are transferred to the MASPER phone. This will help whether these calls that are going to MASPER phone are coming from the switchboard. Please keep in mind that all departments have the ability to transfer calls to dect/mobiles its not only switch. The call might have originated from switch but can be then transferred again from another department.

I would just like to clarify that it was directed by Deputy Director of Emergency Department & the ED Admin Manager in October 2023 that all calls that require to speak to a clinician is to be transferred through to #8004 for ED admins management, they will then transfer the call to the appropriate clinician. Might be worth clarifying with ED to ensure they are up to date with MASPER process also.

[@Pan Jane Wong](#) Can I please confirm your business hours so we can have this for our reference, it is not provided on the rosters. I have noticed your dect phone number #5997 is currently switched off.

I appreciate your feedback and ensuring that we try eliminating the issues that are coming from switchboard.

Kind regards,
Chloe

Chloe Taylor

Switchboard Manager | Switchboard Services

Logan Hospital | [Metro South Health](#)

Level 2, Building 1, Cnr Loganlea and Armstrong Roads QLD 4131

t. (07) 2891 9190

e. chloe.taylor3@health.qld.gov.au | metrosouth.health.qld.gov.au



Metro South Health acknowledges the traditional custodians of whose land and waters we provide health services and pays respect to Elders past, present and emerging.

From: Vivian Kwok <Vivian.Kwok@health.qld.gov.au>
Sent: Wednesday, May 8, 2024 5:28 PM
To: Chloe Taylor <Chloe.Taylor3@health.qld.gov.au>
Cc: Pan Jane Wong <PanJane.Wong@health.qld.gov.au>
Subject: Re: Switchboard issues 5 May, 7 May, and 8 May

Hello,

Here are the switchboard issues 5th May to 8th of May

5/05/2024 at 14:45

Switch put through call to x9320 - was meant to call ward med reg at the time (which was x5223)

7/05/2024 at 0745

Switch put through a call to CMR (Dr Pan Wong) outside work hours with no introduction.

8/05/2024

At 8:53am switch put through GP from Kuraby asking for ED consultant on-call; was incorrectly put through to MASPER

At 9:59am switch put through PAH gastro reg asking for ED consultant on-call; was incorrectly put through to MASPER

Kind regards,

Vivian

From: Vivian Kwok
Sent: Friday, 3 May 2024 3:06 PM
To: Chloe Taylor <Chloe.Taylor3@health.qld.gov.au>
Cc: Pan Jane Wong <PanJane.Wong@health.qld.gov.au>
Subject: Switchboard issues 2nd and 3rd May 2024

Hello,

I've been requested to email you with the list of switchboard issues from yesterday and today

2/5/2024

At 14:47, switch put ED through to x9320 when ED were looking to refer to MASPER (should have been put through to x5223)

3/5/2024

At 10am: switchboard put through PAH phone call to x2147 for medical referral (should have been put through to x5223)

At 10:22am: switchboard put phone call through to x5290 for someone who was asking for day consult registrar; should have put person through to x8653 as per roster today (had to be redirected by MASPER)

At 11:43am, switch put person through to x5223 - when they were asking for PALL CARE CNC

per roster) - had to be redirected by MASPER

At 14:46 - MET call team called x5290 asking where MET call was located "VHUB" - switchboard could not tell them where VHUB was. Had to be redirected by MASPER

Kind regards,

Vivian
MASPER Registrar

From: [Chloe Taylor](#)
To: [Logan Switch](#)
Cc: [Tammy Reese](#); [Ellen Stibbard](#); [Anisha Ismail](#); [Carolyn Jeffrey](#); [Cherrie Abbott](#); [Cory Shepherd](#); [Deborah Neville](#); [Jesse Kendrick](#); [Jude Nelson](#); [Kay Marie Court](#); [Linda Hu](#); [Meegan Malsbury](#); [Patricia Conaghan](#); [Peter Fitzpatrick](#); [Richard Parry](#); [Simon De Iacovo](#); [Sonya Willmann](#); [Stacey Sutton-Masson](#)
Subject: Switchboard Manager - On call and Hours.
Date: Friday, 17 May 2024 9:30:14 AM
Attachments: [image001.gif](#)
[image002.gif](#)
[image003.gif](#)
[image004.gif](#)
[image005.gif](#)
[image006.png](#)

Happy Friday Team,

Just letting you know I will not be in the office until around 11am on Monday 20.05.2024 due to my daughter's kindergarten Interview. I will be contactable on my mobile if anything urgent, otherwise Tammy and Ellen will be in the office.

As per Ellen's email, Ellen is on call until Sunday 19.05.24 and I will be on call from Monday 20.05.24 – Sunday 26.05.24.

As I will be on call from Monday, regardless of my start/finish times next week you are to please contact me through the day/afterhours either via switch, office or mobile.

My office hours can vary due to having to take my girls to school in the morning, on the days that I do have school drop off I always let switch know that I will be in later between 0800-830am.

Otherwise my hours are from 06:30-14:30.

Moving forward so communication is clear for the team, I will be sending an email to switch to advise of any change to my office hours for the week.

Thanks team for being so supportive, have a lovely weekend

Kind regards,

Chloe

Chloe Taylor

Switchboard Manager | Switchboard Services

Logan Hospital | **Metro South Health**

Level 2, Building 1, Cnr Loganlea and Armstrong Roads QLD 4131

t. (07) 2891 9190

e. chloe.taylor3@health.qld.gov.au | metrosouth.health.qld.gov.au



Metro South Health acknowledges the traditional custodians of whose land and waters we provide health services and pays respect to Elders past, present and emerging.



From: Cory Shepherd

Sent: Wednesday, 15 May 2024 1:15 PM

To: Chloe Taylor ; Logan Switch

Cc: Anisha Ismail ; Carolyn Jeffrey ; Cherrie Abbott ; Deborah Neville ; Ellen Stibbard ; Jesse Kendrick ; Jude Nelson ; Kay Marie Court ; Linda Hu ; Meegan Malsbury ; Patricia Conaghan ; Peter Fitzpatrick ; Richard Parry ; Simon De Iacovo ; Sonya Willmann ; Stacey Sutton-Masson ; Tammy Reese ; LBH_HR

Subject: Office Hours and Departmental Directives

Hi Chloe,

I've informed the switchboard of my leave today due to the uncertainty around your current office hours.

As a formal request, could you please share your office hours so the entire department can be aware of your regular schedule? There has been some noted inconsistency in your arrival and departure times, and having a clear schedule would be beneficial for everyone.

Additionally, please ensure that any directives and changes within the department is made in consultation with the team to ensure that any concerns can be appropriately addressed.

Kind regards,

Cory

Renee Matheson

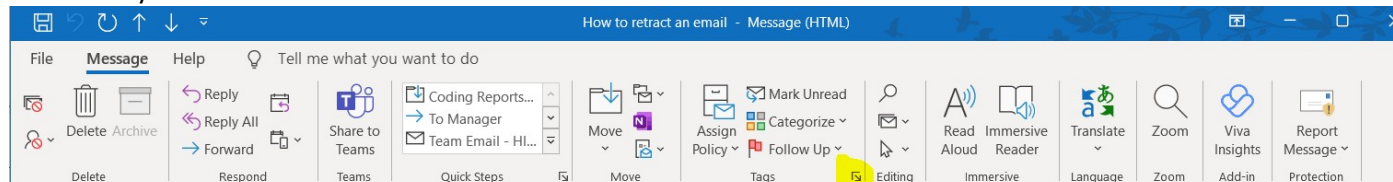
From: Tammy Reese <Tammy.Reese@health.qld.gov.au>
Sent: Wednesday, 15 May 2024 6:23 PM
To: Cory Shepherd
Cc: Chloe Taylor
Subject: RE: Office Hours and Departmental Directives

Importance: High

Good Afternoon Cory,

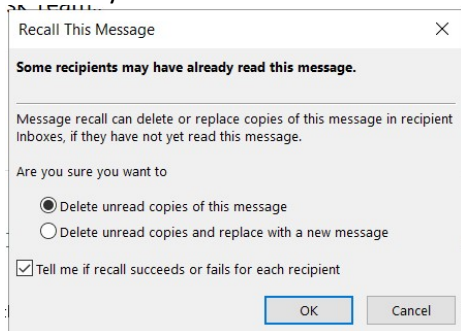
Whilst I understand you are trying to make a number of suggestions and give feedback about your line manager in your email that you sent and copied to all department staff, this did not demonstrate our iCARE2 value of Respect and did not comply with our Code of conduct 'showing respect towards all persons...'. There is a correct way to give feedback and to offer suggestions such as these and that would be by communicating with your manager and/or myself, to give your manager (or myself) the opportunity to consider these suggestions and discuss with yourself but not by sending such an email to all department staff in the first instance as well. Thus, could I please ask you to retract this email. Steps included below to assist if required as to how to do this.

- Go to your Sent items and select the email.



How to retract an email

- Then open that email, then along the top of the email at the Move Icon right click on the arrow downwards (as highlighted in yellow) to select 'Recall This Message', then select delete unread copies of this message, then click okay.



Chloe and I will follow-up with you next week about issues you have raised in your email.

Kind Regards
Tammy

Tammy Reese

Director HIMs | Health Information Management Services,
Digital Health and Informatics,
Logan Hospital | **Metro South Health**
Level 2, Building 1, Cnr Loganlea & Armstrong Rds, QLD 4131
t. 07 32998934 | m. 0417 986 049
e. tammy.reese@health.qld.gov.au | metrosouth.health.qld.gov.au

Renee Matheson

From: Cory Shepherd <Cory.Shepherd@health.qld.gov.au>
Sent: Wednesday, 15 May 2024 7:09 PM
To: Tammy Reese
Subject: Re: Office Hours and Departmental Directives
Attachments: SEllen Buisness hours witchboard After Hours On Call Manager PP24 13:5:24 - 26:5:24 - Inbox • cory.shepherd@health.qld.gov.au.pdf

Hi Tammy,

Thank you for your email, although I believe it reflects a misuse of authority.

I want to emphasize that one of our core iCARE values is integrity. Requesting clarity on business hours is a reasonable question, especially when no one in the department can provide a definitive answer in response to directives to contact Chloe during office hours. Including the entire department in my email was a conscious decision, as I believe that not sharing Chloe's business hours contradicts our values of integrity and transparency. Sharing this information openly would demonstrate our commitment to these principles.

To address a matter that I believe needs clarification and transparency regarding our departmental operations. Specifically, I am seeking clarity on Chloe's official office hours to ensure that the entire team is on the same page and can efficiently direct inquiries.

Over recent times, there has been some confusion due to the lack of clear communication about Chloe's business hours despite repeated notifications requesting to reach her only during those times. To uphold our iCARE2 values, particularly around transparency and integrity, I think it is essential for these hours to be explicitly stated and shared with all team members.

The principles of integrity compel us to be transparent in our dealings, and this includes sharing crucial information such as availability and office hours. I understand that you may be reluctant to share this information but withholding it could be interpreted as a breach of these values.

Additionally, there have been several instances where clinicians and staff from other departments have sought updates on pager statuses and other requests, often left unresolved due to ambiguity around when Chloe is available. Ellen, who works with us, openly shares her business hours with the department, which I believe is a demonstration of our integrity values. I have attached an email from Ellen for review, she gives clear times of when she is in the office and available.

On the broader spectrum of our iCARE2 values:

- ****Integrity****: We should ensure complete transparency in sharing information.
- ****Compassion****: We should show kindness and support, particularly in addressing payroll concerns and ensuring managerial responsiveness.
- ****Accountability****: Clear office hours help ensure that Chloe can be accountable and accessible during defined periods.
- ****Engagement****: Proper consultation and communication about changes and directives ensure that everyone is informed and able to contribute meaningfully.
- ****Respect****: By clearly communicating such basic yet critical information, we show respect for each other's time and responsibilities.

Renee Matheson

From: Tammy Reese <Tammy.Reese@health.qld.gov.au>
Sent: Tuesday, 21 May 2024 2:53 PM
To: Cory Shepherd
Subject: RE: Office Hours and Departmental Directives

Hi Cory,

Thanks for your response. I have asked you to retract an email and gave reasons for why I recommended this action. With regards to your concerns about having more clarity as to what are Chole's business hours I will follow up on the issues raised. One last question I note in your email below for Engagement you state "Proper consultation and communication about changes and directives ensure that everyone is informed and able to contribute meaningfully", can I ask was there one or more particular changes and/or directives where you had concerns about consultation and communication?

Kind Regards
Tammy

Tammy Reese

Director HIMIS | Health Information Management Services,
Digital Health and Informatics,
Logan Hospital | **Metro South Health**
Level 2, Building 1, Cnr Loganlea & Armstrong Rds, QLD 4131
t. 07 32998934 | m. 0417 986 049
e. tammy.reese@health.qld.gov.au | metrosouth.health.qld.gov.au



From: Cory Shepherd
Sent: Wednesday, 15 May 2024 7:09 PM
To: Tammy Reese
Subject: Re: Office Hours and Departmental Directives

Hi Tammy,

Thank you for your email, although I believe it reflects a misuse of authority.

I want to emphasize that one of our core iCARE values is integrity. Requesting clarity on business hours is a reasonable question, especially when no one in the department can provide a definitive answer in response to directives to contact Chloe during office hours. Including the entire department in my email was a conscious decision, as I believe that not sharing Chloe's business hours contradicts our values of integrity and transparency. Sharing this information openly would demonstrate our commitment to these principles.

To address a matter that I believe needs clarification and transparency regarding our departmental operations. Specifically, I am seeking clarity on Chloe's official office hours to ensure that the entire team is on the same page and can efficiently direct inquiries.

From: [Chloe Taylor](#)
To: [Chloe Taylor](#)
Subject: Fw: Office Hours and Departmental Directives
Date: Thursday, 10 July 2025 1:30:08 PM
Attachments: [image001.png](#)
[image002.png](#)
[image003.gif](#)
[image004.gif](#)
[image005.gif](#)
[image006.gif](#)
[image007.gif](#)
[image008.gif](#)
[image009.png](#)
[Cory Shepherd - emergent leave 15.05.24.msg](#)

From: Chloe Taylor
Sent: Friday, 17 May 2024 1:20 PM
To: Adriana McNamee
Subject: FW: Office Hours and Departmental Directives

Good afternoon Adriana, as requested.

I would just like to give you a little background from the email that was sent by Cory.

Monday 13.05.24 – Cory called the switchboard 15 mins before his shift start time 1500-2300 to advise he was in the car on the way to work but he felt unwell and needed coverage for his shift. The operator told cory they would transfer the call to me, but he declined. The operator who took the call came into my office to advise the call from cory and mentioned she couldn't get a clear understanding of what he was asking due to him driving. I called Cory on his mobile, he asked why I was calling and I advised I was just checking to see if he was coming in for his shift or if I needed to cover the shift as the message was unclear to the operator. He told me he was on his way to this hospital but unwell. I told him I am more than happy to cover his shift and if he was unwell, it would be preferred for him not to come into work. While I was on the phone, I politely reminded him of the call process and when staff call in for any leave to please contact me either via switch, office or my mobile.

Tuesday 14.05.24 – Cory called the switchboard around 12:00pm and advised the operator that he will not be in for his 1500-2300 shift that day. He verbally told the operator that he did not want to speak to me. I sent an email (attached) to advise Cory again of the call process and a query about his leave. I also sent a follow up text, advised by the last HR rep to also follow up with a text to Cory as there has been no response in the past to my emails and Cory advising he did not receive them.

Wednesday 15.05.24 – Cory called the switchboard around 13:00 and advised the operator that he will not be in for his 1500-2300 shift that day. He verbally told the operator that he did not want to speak to me and asked the operator to relay the message that he would like to take Annual leave for his shifts. 15 minutes later at 13:15 I received the email below around the uncertainty around my hours.

I am open with communication with all my staff and if for any reason my hours change, I notify them whether its via phone or email. There would never be a time unless I am on emergent leave that I wouldn't be in the office around the times Cory had called to advise his leave.

I would just like to add I have had mutiple staff inform me that in the past not just this week, when Cory calls in for emergent leave he verbally tells the staff he does not want to speak to me. I have had been advised from staff verbally telling me that he has told them he is out to get me and for me to watch/be careful of Cory. These are just a few of the comments I have received of late from switchboard staff. It is not unknown within the switchboard of Corys behaviour and malice towards me which is highly inappopropriate and its starting to affect my ability to perform best in my role. I am currently experencing high anxiety interacting with this staff member and am fearful as to what his next actions maybe.

Thank you for your assistance with this,

Kind regards,

Chloe

Chloe Taylor

Switchboard Manager | Switchboard Services

Logan Hospital | **Metro South Health**

Level 2, Building 1, Cnr Loganlea and Armstrong Roads QLD 4131

t. (07) 2891 9190

e. chloe.taylor3@health.qld.gov.au | metrosouth.health.qld.gov.au



Renee Matheson

From: Chloe Taylor <Chloe.Taylor3@health.qld.gov.au>
Sent: Wednesday, 15 May 2024 1:07 PM
To: Tammy Reese
Subject: Cory Shepherd - emergent leave 15.05.24

Hi Tammy,

Just letting you know that Cory Shepherd called in for emergent leave for his shift today 1500-2300. He called the switch and refused to speak to me. This was after the reminder I gave him on Monday 13th May and yesterday's email/text about the call-in process. He is obviously receiving my emails/texts because he requested to the switch staff today that his leave is to be taken as Annual Leave and that the message will need to be passed on to myself.

Kind regards,
Chloe

From: Chloe Taylor
Sent: Tuesday, May 14, 2024 12:08 PM
To: Cory Shepherd
Cc: Tammy Reese
Subject: Sick leave 14.05.24

Hi Cory,

Thanks for letting the team know that you are unable to make your shift today. Just another friendly reminder that in business hours you are to follow the correct process and speak to me directly if its regarding emergent leave, you can contact me either through switch or my office/mobile. This process still seems to be unclear, and I am more than happy to discuss if you have any questions around this.

Please let me know what leave you would like to take as you are low in sick leave with only 1.68hrs available. Would you prefer to take annual leave in lieu of sick leave?

Kind regards,
Chloe

Chloe Taylor

Switchboard Manager | Switchboard Services

Logan Hospital | **Metro South Health**

Level 2, Building 1, Cnr Loganlea and Armstrong Roads QLD 4131

t. (07) 2891 9190

e. chloe.taylor3@health.qld.gov.au | metrosouth.health.qld.gov.au



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From: [Chloe Taylor](#)
To: [Chloe Taylor](#)
Subject: Fw: Respiratory Nurse Educators
Date: Tuesday, 1 July 2025 10:24:33 AM
Attachments: [image004.gif](#)
[image005.gif](#)
[image006.gif](#)
[image007.gif](#)
[image008.gif](#)
[image010.png](#)
[image011.png](#)
[image012.gif](#)
[image013.png](#)
[image014.png](#)

From: Chloe Taylor
Sent: Monday, 20 May 2024 4:30 PM
To: Cory Shepherd
Subject: RE: Respiratory Nurse Educators

Hi Cory,

Thank you for bringing this to my attention however this task was being actioned. I had discussed with Richard this morning about the update of outpatients respiratory/medical.

While I appreciate you taking initiative and looking into procedures with OPD, this was already being actioned by Richard and I believe you were aware of that.

If any other processes require updating that have been brought to your attention, I am more than happy for you to follow up for my approval. Taking note of your recommendation, we can also put the updated procedures out to the team for consultation before implementing.

Kind regards,

Chloe

Chloe Taylor

Switchboard Manager | Switchboard Services

Logan Hospital | **Metro South Health**

Level 2, Building 1, Cnr Loganlea and Armstrong Roads QLD 4131

t. (07) 2891 9190

e. chloe.taylor3@health.qld.gov.au | metrosouth.health.qld.gov.au



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From: Logan Switch
Sent: Monday, May 20, 2024 2:05 PM
To: Chloe Taylor
Cc: Brett Windeatt ; Sue Marriott
Subject: FW: Respiratory Nurse Educators
Importance: High

Hi Chloe,

Please review the email attached confirming the clinics runout from Medical Outpatients. Understandably, switchboard staff may not be aware of the clinics due to modifications to the Document: Outpatients Department –Clinic contact Details. on the 22nd of February 2024. I recommend a modification and review of the document.

Kind regards,

Cory

Switchboard Services

Telecommunications | Health Information Management Services, Digital Health & Informatics

Logan Hospital | **Metro South Health**

Level 2, Building 1, Cnr Armstrong & Loganlea Roads, Meadowbrook, Qld 4131

t. 07 3299 8899

e. logan.switch@health.qld.gov.au | metrosouth.health.qld.gov.au



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From: Sue Marriott <Sue.Marriott@health.qld.gov.au>
Sent: Monday, May 20, 2024 11:03 AM
To: Logan Switch <Logan.Switch@health.qld.gov.au>
Cc: Brett Windeatt <Brett.Windeatt@health.qld.gov.au>
Subject: FW: Respiratory Nurse Educators

Importance: High

Hello team,

Just a courtesy reminder, we continue to get calls put through to us for Respiratory Medical Outpatients. Could you please ensure your phone directory reflects Respiratory Nurse Educators – Integrated Respiratory Service does not have doctors attached to the service and we can not help patients or other clinical staff with OPD issues..

Thanks again,

Kind regards,

Sue Marriott

Administration Officer | Integrated Respiratory Service

Logan Hospital | **Metro South Health**

10 Armstrong Road. Meadowbrook 4131

t. 32998768

e. sue.marriott@health.qld.gov.au | metrosouth.health.qld.gov.au

Working hours: Monday - Friday 7.00 - 3.30

[Find out more about our values](#)



From: Sue Marriott
Sent: Wednesday, May 15, 2024 11:47 AM
To: Logan Switch <Logan.Switch@health.qld.gov.au>
Cc: Brett Windeatt <Brett.Windeatt@health.qld.gov.au>
Subject: Respiratory Nurse Educators
Importance: High

Hello team,

Could you please amend your number registry/directory to show #8768 belongs to the Integrated Respiratory Service, housing Respiratory Nurses and Respiratory Physiotherapist and Pulmonary Rehabilitation.

We are not Respiratory Medical OPD and we do not have any doctors working out of this area, nor are we able to reschedule, cancel etc any respiratory medical appointments.

PayrollMetroSouth

From: PayrollMetroSouth
Sent: Monday, 13 May 2024 8:21 AM
To: Cory Shepherd
Subject: RE: Corey Shepherd 388372 Pay issues

Hi Corey,

I cannot see that any of the issues below have been corrected. Please speak to your Line Manager to have them corrected with an AVAC submitted through My HR as this will also be affecting your RDO balances as well as your pay.

Kind regards

Elaine

Elaine Grant

Client Service Officer, Metro South Payroll Team

Payroll Transactional Services, Corporate Enterprise Solutions, Corporate Services | Queensland Health

P [07 3608 5985](tel:0736085985)
E elaine.grant@health.qld.gov.au
W health.qld.gov.au
A [Level 1, 11-13 University Drive, Meadowbrook QLD 4131](#)

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Queensland Health acknowledges the Traditional Custodians of the land across Queensland, and pays respect to First Nations Elders past, present and future.

From: Cory Shepherd <Cory.Shepherd@health.qld.gov.au>
Sent: Friday, May 10, 2024 2:21 PM
To: PayrollMetroSouth <PayrollMetroSouth@health.qld.gov.au>; Elaine Grant <Elaine.Grant@health.qld.gov.au>
Subject: Re: Corey Shepherd 388372 Pay issues

Hi Elaine,

I am writing to follow up on a matter concerning Chloe.

I have previously expressed some concerns regarding her rostering and the challenges she's been facing with payment processing. I was wondering if you have received any response or updates from her regarding these issues.

In case Chloe has already responded or managed to process the missing payments, I would appreciate it if you could keep me informed.

Thank you for your attention to this matter.

Best Regards,
Cory

From: PayrollMetroSouth <PayrollMetroSouth@health.qld.gov.au>
Sent: Friday, May 3, 2024 9:39 AM
To: Chloe Taylor <Chloe.Taylor3@health.qld.gov.au>
Cc: Cory Shepherd <Cory.Shepherd@health.qld.gov.au>
Subject: Corey Shepherd 388372 Pay issues

Hi Chloe,

Corey has wages top up on fortnight 05.02.24 for 0.95mins as he is under his contracted hours so it is topping up the missing wages from his RDO balance.

This will be due to having a 7hr shift on 09.02.24 instead of an 8hr shift. AVAC PRN 15397775 . If a shift is shortened when RDO accruing the remaining shift not worked needs to be booked as leave without pay to stop the top up. The same has happened for the next fortnight 19.02.24 due to 28.02.24 shift AVAC PRN 15605601 has this shift as 7hrs topping up 0.95 for missing hour.

Fortnight 18.03.24 is the opposite and has too many ordinary shifts for the fortnight resulting in wage reduction of 7.55hrs as one of the shifts needs to be overtime.

Fortnight 01.04.24 is the same, 0.95hrs over contracted 76hrs needs to be overtime. I think this should be planned OT on 01.04.24 as that is a 9hr shift.

Please submit an AVAC to correct these shifts for each fortnight so Cory is paid corrected and his RDO balance will then be amended.

Kind regards
Elaine

Elaine Grant

Client Service Officer, Metro South Payroll Team

Payroll Transactional Services, Corporate Enterprise Solutions, Corporate Services | Queensland Health

P [07 3608 5985](tel:0736085985)
E elaine.grant@health.qld.gov.au
W health.qld.gov.au
A [Level 1, 11-13 University Drive, Meadowbrook QLD 4131](#)

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Queensland Health acknowledges the Traditional Custodians of the land across Queensland, and pays respect to First Nations Elders past, present and future.

Renee Matheson

From: Chloe Taylor <Chloe.Taylor3@health.qld.gov.au>
Sent: Tuesday, 21 May 2024 12:33 PM
To: Cory Shepherd
Subject: RE: Corey Shepherd 388372 Pay issues

Hi Cory,

Just letting you know that I am still I am waiting payroll confirmation about a few of these payroll issues and as soon as I do get that confirmation, I will submit an AVAC for next pay run. I will let you know PRN once it has been submitted.

Thanks Chloe

Chloe Taylor

Switchboard Manager | Switchboard Services

Logan Hospital | **Metro South Health**

Level 2, Building 1, Cnr Loganlea and Armstrong Roads QLD 4131

t. (07) 2891 9190

e. chloe.taylor3@health.qld.gov.au | metrosouth.health.qld.gov.au



Metro South Health acknowledges the traditional custodians of whose land and waters we provide health services and pays respect to Elders past, present and emerging.

From: PayrollMetroSouth
Sent: Friday, May 3, 2024 9:40 AM
To: Chloe Taylor
Cc: Cory Shepherd
Subject: Corey Shepherd 388372 Pay issues

Hi Chloe,

Corey has wages top up on fortnight 05.02.24 for 0.95mins as he is under his contracted hours so it is toping up the missing wages from his RDO balance.

This will be due to having a 7hr shift on 09.02.24 instead of an 8hr shift. AVAC PRN 15397775 . If a shift is shortened when RDO accruing the remaining shift not worked needs to be booked as leave without pay to stop the top up.

The same has happened for the next fortnight 19.02.24 due to 28.02.24 shift AVAC PRN 15605601 has this shift as 7hrs topping up 0.95 for missing hour.

Fortnight 18.03.24 is the opposite and has too many ordinary shifts for the fortnight resulting in wage reduction of 7.55hrs as one of the shifts needs to be overtime.

Fortnight 01.04.24 is the same, 0.95hrs over contracted 76hrs needs to be overtime. I think this should be planned OT on 01.04.24 as that is a 9hr shift.

From: [Chloe Taylor](#)
To: [Chloe Taylor](#)
Subject: Fw: Validation of Claims older than 3 months - Please sign
Date: Thursday, 10 July 2025 1:27:15 PM
Attachments: [image001.gif](#)
[image002.gif](#)
[image003.gif](#)
[image004.gif](#)
[image005.gif](#)
[image006.png](#)
[SKM_C300i24052808020.pdf](#)
[SKM_C300i24052808010.pdf](#)
[RE Corey Shepherd 388372 Pay issues.msg](#)

From: Chloe Taylor
Sent: Wednesday, 4 September 2024 9:45 AM
To: Mack Pritchard
Subject: FW: Validation of Claims older than 3 months - Please sign

Hi Mack,
Please see email and attached for reference.
Kind regards,
Chloe

From: Chloe Taylor
Sent: Tuesday, May 28, 2024 8:36 AM
To: Cory Shepherd
Cc: Tammy Reese
Subject: Validation of Claims older than 3 months - Please sign
Importance: High

Good morning Cory,
Please find attached Validation of claims older than 3 months, please sign and return to be as soon as possible so I can escalate for delegate approval. Once this is processed you will be paid correctly and RDO balance will be amended.
Please let me know if you have any questions,
Kind regards,
Chloe

Chloe Taylor
Switchboard Manager | Switchboard Services
Logan Hospital | **Metro South Health**
Level 2, Building 1, Cnr Loganlea and Armstrong Roads QLD 4131
t. (07) 2891 9190
e. chloe.taylor3@health.qld.gov.au | metrosouth.health.qld.gov.au



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Reference Number	Form Code Text	Person ID	Position	Object ID	Start Date	Date	Business Status	Processing Date	Case Key	Sem Object-Action	Form Category	Employee Name	Name	Name	Initiator Name	Payteam ID	foi Payteam ID	Text	Processor	Form Ty	Process
15325947	Attendance Variation and Allowance Claim (AVAC)	388372	32002276	72010875	6/02/2024	22/01/2024	Completed	8/02/2024	16090527EA951EEEB18F4EC08F46D526	Forms-Upload	AVACs	Shepherd, Cory	AO Switchboard	MSH CI LOGH Switchboard Svcs	Donovan-Taylor,Chloe	PS-11	Metro South		P	ZHR_UPLOAD_FORMS	
15480560	Smart Leave Request	388372	32002276	72010875	20/02/2024	20/02/2024	Completed	1/03/2024	16090527EA951EEEB3F1DAD735E25618	inboxdetail-display	Leave	Shepherd, Cory	AO Switchboard	MSH CI LOGH Switchboard Svcs	Shepherd,Cory	PS-11	Metro South		U	ZHR_F_LEAVE_APPLICATION	
15848692	Smart Leave Request	388372	32002276	72010875	27/03/2024	25/03/2024	Completed	27/03/2024	028C8CF6765D1EEEBAF8687DC8E4905E	inboxdetail-display	Leave	Shepherd, Cory	AO Switchboard	MSH CI LOGH Switchboard Svcs	Donovan-Taylor,Chloe	PS-11	Metro South		U	ZHR_F_LEAVE_APPLICATION	
15849573	Attendance Variation and Allowance Claim (AVAC)	388372	32002276	72010875	27/03/2024	18/03/2024	Completed	1/04/2024	16090527EA951EDEBAF8FE077E705248	Forms-Upload	AVACs	Shepherd, Cory	AO Switchboard	MSH CI LOGH Switchboard Svcs	Donovan-Taylor,Chloe	PS-11	Metro South		P	ZHR_UPLOAD_FORMS	
15969838	Attendance Variation and Allowance Claim (AVAC)	388372	32002276	72010875	9/04/2024	30/03/2024	Completed	11/04/2024	028C8CF6765D1EDEBDC332B8B7618D10	Forms-Upload	AVACs	Shepherd, Cory	AO Switchboard	MSH CI LOGH Switchboard Svcs	Donovan-Taylor,Chloe	PS-11	Metro South		P	ZHR_UPLOAD_FORMS	
16326886	Attendance Variation and Allowance Claim (AVAC)	388372	32002276	72010875	15/05/2024	7/05/2024	Completed	16/05/2024	16090527EA951EEF84CD1578FA599A18	Forms-Upload	AVACs	Shepherd, Cory	AO Switchboard	MSH CI LOGH Switchboard Svcs	Donovan-Taylor,Chloe	PS-11	Metro South		P	ZHR_UPLOAD_FORMS	
16450619	Attendance Variation and Allowance Claim (AVAC)	388372	32002276	72010875	28/05/2024	30/03/2024	Part Completed	30/05/2024	16090527EA951EEF878ECC45DE88E5C	Forms-Upload	AVACs	Shepherd, Cory	AO Switchboard	MSH CI LOGH Switchboard Svcs	Donovan-Taylor,Chloe	PS-11	Metro South		P	ZHR_UPLOAD_FORMS	

Search Results (7)



Process Number	Request Type	Employee	Position	Organisation	Submission Date	Effective Date	Status	Processing Date	
16328886	Attendance Variation and Allowance Claim (AVAC)	Shepherd, Cory (388372)	AO Switchboard (32002276)	MSH CI LOGH Switchboard Svcs (72010875)	15.05.2024	07.05.2024	Completed	16.05.2024	>
16450619	Attendance Variation and Allowance Claim (AVAC)	Shepherd, Cory (388372)	AO Switchboard (32002276)	MSH CI LOGH Switchboard Svcs (72010875)	28.05.2024	30.03.2024	Part Completed	30.05.2024	>
15969838	Attendance Variation and Allowance Claim (AVAC)	Shepherd, Cory (388372)	AO Switchboard (32002276)	MSH CI LOGH Switchboard Svcs (72010875)	09.04.2024	30.03.2024	Completed	11.04.2024	>
15848692	Smart Leave Request	Shepherd, Cory (388372)	AO Switchboard (32002276)	MSH CI LOGH Switchboard Svcs (72010875)	27.03.2024	25.03.2024	Completed	27.03.2024	>
15849573	Attendance Variation and Allowance Claim (AVAC)	Shepherd, Cory (388372)	AO Switchboard (32002276)	MSH CI LOGH Switchboard Svcs (72010875)	27.03.2024	18.03.2024	Completed	01.04.2024	>
15480560	Smart Leave Request	Shepherd, Cory (388372)	AO Switchboard (32002276)	MSH CI LOGH Switchboard Svcs (72010875)	20.02.2024	20.02.2024	Completed	01.03.2024	>
15325947	Attendance Variation and Allowance Claim (AVAC)	Shepherd, Cory (388372)	AO Switchboard (32002276)	MSH CI LOGH Switchboard Svcs (72010875)	06.02.2024	22.01.2024	Completed	08.02.2024	>

Leave Takings Report

Employee Team:
Include Sub Teams:
IWFM Only Teams:
Employee:
Report Selection Mode:
Date Selection:
Date From:
Date To:

No
No
00388372
General Mode
Manual
19/03/2024
19/03/2024

Leve Category
Leave Type:
Leave Status:
Sort By:
Requested Leave Hours:
Comments:
Show Paramters:

ALL
ALL
All
Employee Number
No
No
Yes

* Leave Taken (hours) only reflects hours during the leave period containing underlying scheduled shifts

Employee Team	Employee ID	Employee Name	Leave Category	Leave Type	Time Code	Leave Period Date	Leave Taken* (Hours)	Leave Status
72010875 LOGH CORP Switchboard Operations	00388372	Shepherd, Cory	Sick	Sick Leave	SCK	19/03/2024 - 19/03/2024	7.60	APPROVED

Movement Details

Movement Task: Change to Working Conditions (Temporary)
Effective Period: 01.03.2026 - 15.03.2026
Movement Reason: Change in Hours - Other
Position ID: 32002276
Position Title: AO Switchboard
Person ID: 388372
Person (Preferred) Name: Cory Shepherd
Personnel Assignment Number: 388372
Organisational Unit ID: 72010875
Organisational Unit Name: LOGH CORP Switchboard Operations

Employment Conditions

Award/CBA/Contract: Hospital and Health Service General Employees (Queensland Health) Award – State 2015
Classification: A03
Level: 04
Employment Basis: Part Time
Number Of Fortnightly Hours: 56.00
Concurrent/Aggregate Current Status: Not Applicable
Concurrent/Aggregate Proposed Status: ☐ Indicate here if the employee will be working in multiple positions at the same time resulting in Concurrent or Aggregate employment.
Working Arrangement: Standard Hours
Shift Arrangements: Continuous Shift Worker
Reason For Additional Leave: Working Public Holidays, Continuous Shift Work
Recreation Leave Accrual (weeks): 6

Work Schedule

Roster Type: Non-Cyclic

Other

Comments: Cory Shepherd to extend reduce hours contract - 1.03.26 - 15.03.26 possible extension

Manager Certification:



I certify that I have:

*(Where the employee is seeking release or extension of a previously approved movement from another work unit) successfully negotiated the terms of the agreement with the line manager of the employee's substantive position

*Informed this employee of any changes to their FBT Concession Eligibility status as a consequence of this variation to their employment contract

*Discussed with this employee the consequences of this change to their position, employment status, terms of employment and/or roster

*Informed the employee where this change applies to a temporary employee moving between temporary assignments, of any impact (i.e. the ending or likelihood of extension of their previous contract) as a consequence of accepting appointment to this proposed position and

*Undertaken all required checks and cited all relevant documentation including, where required, verification against the AHPRA Registers of Practitioners.

*Completed all necessary pre-employment checks with evidence attached and have submitted registration and Working with children checks to Payroll Services for processing.

*Where the employee is required to work in a position which requires evidence of vaccination or proof that they are not susceptible to specified vaccine preventable diseases (VPDs); that the relevant vaccination status in accordance to their assigned position has been sighted.

Note: Vaccination records must be retained in a manner that conforms with both the *Information Privacy Act 2009 (QLD)* and the *Public Records Act 2002 (QLD)*

Chloe Donovan-Taylor

19.02.2026

Employee:

I agree to the above changes to my employment hours/position. I hereby claim for the extra remuneration for hours worked in a higher duties capacity (where applicable). I also certify that I have been informed by my line manager/supervisor of the consequences of this change to my:

* FBT Concession Eligibility status that may result from this variation to my employment contract and

* Position, employment status, terms of employment and/or roster. I also acknowledge that as this appointment is of a temporary nature, the contract may be ended by my line manager with the appropriate notice in accordance with award provisions.

I understand that my current existing superannuation scheme and/or contribution arrangements will continue to apply to my new health employer within Queensland Health (where applicable), with any future changes to superannuation arrangements to be applicable to current and past employer entities within Queensland Health, unless otherwise specifically advised.

Temporary employees only:

Your appointment is for a temporary period. Continuation or extension of employment in this position cannot be guaranteed beyond the end date stated above. Please note that your employment may be terminated prior to the end date of your fixed term appointment, by either party, in accordance with the termination clause in the above mentioned Award or other industrial instruments relevant to your employment.

Disclosure of personal information:

I understand that personal information in relation to my employment may be disclosed by my current health employer to another health or government agency, subject to applicable law, in connection with my movement/change to a role or employment with (or any application for or assessment of me having a role or employment with) that other health or government agency.

Cory Shepherd

26.02.2026

Reviewer Group:

I, the authorised Endorser for this appointment confirm that all impacts have been reviewed and confirmed.

Paul Ianna

27.02.2026

Delegate:



If applicable review the changes to the employee recurring allowances contained in this application.

I, the authorised delegate for appointments, approve:
*any increase above the position's established FTE as a consequence of this appointment, and
*the above appointment subject to the receipt of acceptable criminal history report (where necessary) in accordance with *Section 67 of the Public Service Act 2008 or Section 67 of the Hospital and Boards Act 2001* [whichever is applicable].

HES/SES higher duties only:
Will the employee be allocated a government owned motor vehicle for private use or home garaging during this period of relief?

Scott Hughes

27.02.2026

Movement Details

Movement Task: Change to Working Conditions (Temporary)
Effective Period: 16.03.2026 - 26.04.2026
Movement Reason: Change in Hours - Other
Position ID: 32002276
Position Title: AO Switchboard
Person ID: 388372
Person (Preferred) Name: Cory Shepherd
Personnel Assignment Number: 388372
Organisational Unit ID: 72010875
Organisational Unit Name: LOGH CORP Switchboard Operations

Employment Conditions

Award/CBA/Contract: Hospital and Health Service General Employees (Queensland Health) Award – State 2015
Classification: A03
Level: 04
Employment Basis: Part Time
Number Of Fortnightly Hours: 56.00
Concurrent/Aggregate Current Status: Not Applicable
Concurrent/Aggregate Proposed Status: ☐ Indicate here if the employee will be working in multiple positions at the same time resulting in Concurrent or Aggregate employment.
Working Arrangement: Standard Hours
Shift Arrangements: Continuous Shift Worker
Reason For Additional Leave: Working Public Holidays, Continuous Shift Work
Recreation Leave Accrual (weeks): 6

Work Schedule

Roster Type: Non-Cyclic

Other

Comments: Cory Shepherd reduced hours from 1FTE to 0.7fte per fortnight 16.03.2026 - 26.04.2026 with possible extension after review.

Manager Certification:



I certify that I have:

*(Where the employee is seeking release or extension of a previously approved movement from another work unit) successfully negotiated the terms of the agreement with the line manager of the employee's substantive position

*Informed this employee of any changes to their FBT Concession Eligibility status as a consequence of this variation to their employment contract

*Discussed with this employee the consequences of this change to their position, employment status, terms of employment and/or roster

*Informed the employee where this change applies to a temporary employee moving between temporary assignments, of any impact (i.e. the ending or likelihood of extension of their previous contract) as a consequence of accepting appointment to this proposed position and

*Undertaken all required checks and cited all relevant documentation including, where required, verification against the AHPRA Registers of Practitioners.

*Completed all necessary pre-employment checks with evidence attached and have submitted registration and Working with children checks to Payroll Services for processing.

*Where the employee is required to work in a position which requires evidence of vaccination or proof that they are not susceptible to specified vaccine preventable diseases (VPDs); that the relevant vaccination status in accordance to their assigned position has been sighted.

Note: Vaccination records must be retained in a manner that conforms with both the *Information Privacy Act 2009 (QLD)* and the *Public Records Act 2002 (QLD)*

Chloe Donovan-Taylor

09.04.2026

Employee:

I agree to the above changes to my employment hours/position. I hereby claim for the extra remuneration for hours worked in a higher duties capacity (where applicable). I also certify that I have been informed by my line manager/supervisor of the consequences of this change to my:

* FBT Concession Eligibility status that may result from this variation to my employment contract and

* Position, employment status, terms of employment and/or roster. I also acknowledge that as this appointment is of a temporary nature, the contract may be ended by my line manager with the appropriate notice in accordance with award provisions.

I understand that my current existing superannuation scheme and/or contribution arrangements will continue to apply to my new health employer within Queensland Health (where applicable), with any future changes to superannuation arrangements to be applicable to current and past employer entities within Queensland Health, unless otherwise specifically advised.

Temporary employees only:

Your appointment is for a temporary period. Continuation or extension of employment in this position cannot be guaranteed beyond the end date stated above. Please note that your employment may be terminated prior to the end date of your fixed term appointment, by either party, in accordance with the termination clause in the above mentioned Award or other industrial instruments relevant to your employment.

Disclosure of personal information:

I understand that personal information in relation to my employment may be disclosed by my current health employer to another health or government agency, subject to applicable law, in connection with my movement/change to a role or employment with (or any application for or assessment of me having a role or employment with) that other health or government agency.

Cory Shepherd

13.04.2026

Reviewer Group:

I, the authorised Endorser for this appointment confirm that all impacts have been reviewed and confirmed.

Paul Ianna

15.04.2026

Delegate:



If applicable review the changes to the employee recurring allowances contained in this application.

I, the authorised delegate for appointments, approve:
*any increase above the position's established FTE as a consequence of this appointment, and
*the above appointment subject to the receipt of acceptable criminal history report (where necessary) in accordance with *Section 67 of the Public Service Act 2008 or Section 67 of the Hospital and Boards Act 2001* [whichever is applicable].

HES/SES higher duties only:

Will the employee be allocated a government owned motor vehicle for private use or home garaging during this period of relief?

Scott Hughes

17.04.2026

Movement Details

Movement Task: Change to Working Conditions (Temporary)
Effective Period: 25.05.2026 - 28.06.2026
Movement Reason: Change in Hours - Other
Position ID: 32002276
Position Title: AO Switchboard
Person ID: 388372
Person (Preferred) Name: Cory Shepherd
Personnel Assignment Number: 388372
Organisational Unit ID: 72010875
Organisational Unit Name: LOGH CORP Switchboard Operations

Employment Conditions

Award/CBA/Contract: Hospital and Health Service General Employees (Queensland Health) Award – State 2015
Classification: AO3
Level: 04
Employment Basis: Part Time
Number Of Fortnightly Hours: 40.00
Concurrent/Aggregate Current Status: Not Applicable
Concurrent/Aggregate Proposed Status: ☐ Indicate here if the employee will be working in multiple positions at the same time resulting in Concurrent or Aggregate employment.
Working Arrangement: Standard Hours
Shift Arrangements: Continuous Shift Worker
Reason For Additional Leave: Working Public Holidays, Continuous Shift Work
Recreation Leave Accrual (weeks): 6

Work Schedule

Roster Type: Non-Cyclic

Other

Comments: Cory Shepherd request to reduce contracted hours from 1.0FTE - 0.5TE 40hrs per fortnight.
Temp request until 28.06.26 until further review and documentation to be provided to support request.

Manager Certification:



I certify that I have:

*(Where the employee is seeking release or extension of a previously approved movement from another work unit) successfully negotiated the terms of the agreement with the line manager of the employee's substantive position

*Informed this employee of any changes to their FBT Concession Eligibility status as a consequence of this variation to their employment contract

*Discussed with this employee the consequences of this change to their position, employment status, terms of employment and/or roster

*Informed the employee where this change applies to a temporary employee moving between temporary assignments, of any impact (i.e. the ending or likelihood of extension of their previous contract) as a consequence of accepting appointment to this proposed position and

*Undertaken all required checks and cited all relevant documentation including, where required, verification against the AHPRA Registers of Practitioners.

*Completed all necessary pre-employment checks with evidence attached and have submitted registration and Working with children checks to Payroll Services for processing.

*Where the employee is required to work in a position which requires evidence of vaccination or proof that they are not susceptible to specified vaccine preventable diseases (VPDs); that the relevant vaccination status in accordance to their assigned position has been sighted.

Note: Vaccination records must be retained in a manner that conforms with both the *Information Privacy Act 2009 (QLD)* and the *Public Records Act 2002 (QLD)*

Chloe Donovan-Taylor

27.05.2026

Employee:

I agree to the above changes to my employment hours/position. I hereby claim for the extra remuneration for hours worked in a higher duties capacity (where applicable). I also certify that I have been informed by my line manager/supervisor of the consequences of this change to my:

* FBT Concession Eligibility status that may result from this variation to my employment contract and

* Position, employment status, terms of employment and/or roster. I also acknowledge that as this appointment is of a temporary nature, the contract may be ended by my line manager with the appropriate notice in accordance with award provisions.

I understand that my current existing superannuation scheme and/or contribution arrangements will continue to apply to my new health employer within Queensland Health (where applicable), with any future changes to superannuation arrangements to be applicable to current and past employer entities within Queensland Health, unless otherwise specifically advised.

Temporary employees only:

Your appointment is for a temporary period. Continuation or extension of employment in this position cannot be guaranteed beyond the end date stated above. Please note that your employment may be terminated prior to the end date of your fixed term appointment, by either party, in accordance with the termination clause in the above mentioned Award or other industrial instruments relevant to your employment.

Disclosure of personal information:

I understand that personal information in relation to my employment may be disclosed by my current health employer to another health or government agency, subject to applicable law, in connection with my movement/change to a role or employment with (or any application for or assessment of me having a role or employment with) that other health or government agency.

Cory Shepherd**05.06.2026****Reviewer Group:**

I, the authorised Endorser for this appointment confirm that all impacts have been reviewed and confirmed.

Melissa Spink**08.06.2026**

Delegate:



If applicable review the changes to the employee recurring allowances contained in this application.

I, the authorised delegate for appointments, approve:
*any increase above the position's established FTE as a consequence of this appointment, and
*the above appointment subject to the receipt of acceptable criminal history report (where necessary) in accordance with *Section 67 of the Public Service Act 2008 or Section 67 of the Hospital and Boards Act 2001* [whichever is applicable].

HES/SES higher duties only:

Will the employee be allocated a government owned motor vehicle for private use or home garaging during this period of relief?

Scott Hughes

09.06.2026



Office of the Chief Executive

Enquiries to: Myla Ruttan
Principal Lawyer
Metro South Legal Services
Telephone: 07 3069 4755

Email: metrosouthlegal@health.qld.gov.au
Our Ref: K-LM26/729
Date: 5 June 2026

**Metro South
Health**

Commissioner Dwyer
Queensland Industrial Relations Commission
Level 21 66 Eagle Street Brisbane
Qld, 4000
By email: qirc.registry@qirc.qld.gov.au

Dear Commissioner Dwyer

WC/2024/227 – Cory Lea Shepherd v Workers' Compensation Regulator

I refer to the Form 29 – Notice of non-party disclosure (the **Notice**) regarding the above matter. Mr Shepherd is a current employee of Metro South Hospital and Health Service (**Metro South Health**) as Switchboard Operator at Logan Hospital.

The Notice compels Metro South Health to produce various documents that Mr Shepherd claims to require for his Workers' Compensation Appeal.

Metro South Health has complied with some of the Items requested in the Notice as outlined below. In accordance with Rule 64E(4) of the *Industrial Relations (Tribunals) Rules 2011*, our objections to production/compliance with the Notice are also outlined below.

At the outset, it should be noted that Metro South Health is not in receipt of the Applicant's Amended Statement of Facts and Contentions (Form 9A). Throughout the Notice, the Applicant has expressed various opinions and views about the relevance of the documents to the allegations and stressors the subject of the appeal. While Metro South Health's substantiative grounds of objection to compliance with the Notice are outlined below, our ability to determine whether the requested documents are 'relevant' to the proceeding (and therefore give rise to an objection under Rule 64E4(b)) has, to some extent been limited. We submit this ought to be taken into consideration by the Commissioner.

Items 1 and 2 - SPOK emergency paging export and Aggregate SPOK activation counts

Metro South Health cannot comply with the request for Items 1 and 2. The requested records do not exist. The system was upgraded in SPOK, and records predating that upgrade were not retained. However, a spreadsheet of recorded MET calls is available for the period 17-18 March 2024.

Item 3(a) - Complaints received - server side exchange filter

Metro South Health objects to the production of Item 3(a). The reason for this objection is the expense and inconvenience that Metro South Health will incur (Rule 64E(4)(a)).

Better together

All employee complaints relating to Logan Hospital Switchboard operational errors are made directly to the Line Manager of Switch Board and managed solely via email or verbally with the complainant.

Metro South Health's core function is to provide health services to the public. Compliance with a request of this date range would require approval from the Health Service Chief Executive to engage external resources and assistance from eHealth Queensland at the Department of Health to review and search individual staff member email accounts. This would have a significant administrative and operational burden. Particularly as the date range requested is so broad (between the period 1 December 2023 to 30 June 2024). In circumstances where the Matter in Issue as pleaded in the Notice relates to "consecutive shifts of 17 to 18 March 2024", Metro South Health submits a search of the requested range and breadth will take an unreasonable amount of time and resourcing and detract from our core functions of providing health care to the community.

Item 3(b) - Management discussion of complaints

Metro South Health relies on the same grounds for objection as for Item 3(a) above.

Item 3(c) - Consequential changes to operating procedures

Metro South Health cannot comply with the request for Item 3(c) because there have been no 'consequential' changes to operating procedures over the period requested.

Item 4 - FRMS risk assessment record

Metro South Health cannot comply with the Notice in relation to Item 4. This is because the requested documents do not exist. Mandatory Fatigue Risk Management System training only applies to health practitioners and clinical assistants. The Logan Hospital Switchboard staff are non-clinical staff, and therefore there is no mandatory requirement for them to complete Fatigue Risk Management System training.

Item 5 - FRMS training and competency records

Metro South Health cannot comply with the request in relation to Item 5. The implementation of fatigue risk management assessment at Switchboard Logan Hospital occurred after 30 June 2024 in connection with an organisational change related to the reporting lines for Switchboard.

Item 6 – FRMS delegation and governance record

Metro South Health encloses the attached Role Description for Manager of Switchboard Services.

Item 7 - FRMS register entries and switchboard roster expectations

Metro South Health cannot comply with the Notice in relation to Item 7. This is because the requested documents do not exist. Mandatory Fatigue Risk Management System training only applies to health practitioners and clinical assistants.

Item 8 – Management System Log in records

Metro South Health objects to the production of Item 8. The reason for this objection is the expense and inconvenience that Metro South Health will incur (Rule 64E(4)(a)).

Compliance with a request of this date range would require approval from the Health Service Chief Executive to engage external resources and assistance from eHealth Queensland at the Department of Health to review and search individual staff member email accounts. This would have a significant administrative and operational burden, particularly as the date range requested is so broad (between the period 1 December 2023 to 30 June 2024) and a search of the requested

range and breadth will take an unreasonable amount of time and resourcing and detract from our core functions of providing health care to the community. In circumstances where the Matter in Issue as pleaded in the Notice relates to “consecutive shifts of 17 to 18 March 2024”, Metro South Health submits this date range is a reasonable range to search.

Further, the reference to “*Management system login records - Active Directory / Entra ID / Citrix. Native electronic 2024 export of Windows Active Directory, Entra ID and Citrix session-login audit records*” is unreasonably broad (lacking particularity). Metro South Health uses various application platforms and it is not clear which system is being referred to.

Item 9 - Management building access

Metro South Health objects to the production of Item 9 on the basis that the request lacks relevance, particularity and could affect a person (Rule 64E(4)(b)(c) and (f)). The Switchboard office is collocated in an office area where there are other staff from various departments going through the office area. The switchboard office does not have its own access-swipe access.

The Applicant is a current employee and reports directly to the line manager. The request is unreasonably broad (lacks particularity) and production could have an effect on the line manager.

Item 10 - various

Metro South Health objects to the production of the documents requested in Item 10. The reason for this objection is the expense and inconvenience that Metro South Health will incur (Rule 64E(4)(a)). Compliance with a request of this date range would require approval from the Health Service Chief Executive to engage external resources and assistance from eHealth Queensland at the Department of Health to review and search individual staff member email accounts. This would have a significant administrative and operational burden. Particularly as the date range requested is so broad (between the period 1 December 2023 to 30 June 2024).

In circumstances where the Matter in Issue as pleaded in the Notice relates to “consecutive shifts of 17 to 18 March 2024”, Metro South Health submits a search of the requested range and breadth will take an unreasonable expenditure of time and resourcing, and detract from our core functions of providing health care to the community.

Item 11 - Appellant's MyHR audit trail - COVID Special Pandemic Leave (Process Reference 15480560)

Metro South Health encloses the following documents in compliance with the request for Item 11

- MyHR report leave submission for period 1 February 2024 to 31 May 2024;
- Leave form evidence of pandemic leave for period 20 to 27 February 2024;
- Leave form for period 20 to 27 February 2024;
- AVAC history for pandemic leave for the period 20 to 27 February 2024.

Item 12 – Comparator MyHR audit trails

In compliance with the request for Item 12, Metro South Health encloses a complete audit trail of the requested pandemic leave for the period requested for Logan Hospital Switchboard.

Metro South Health otherwise objects to the production of other documents as outlined in Item 12.

This request would require accessing the relevant staff members' supporting evidence of their Special Pandemic Leave requests during the period 1 January 2024 – 30 June 2024. Examples of supporting evidence could be a photograph of a positive COVID test, medical certificate, lab results confirming COVID diagnosis, or letter from their child's school in the event of school closure.

This request is unreasonable in circumstances where the effect of production would have on the other staff members (i.e. breach of their right to their privacy and reputation) and that the proof of COVID test contains confidential and sensitive personal health information of the relevant staff members.

Item 13 – Line Manager authority framework – COVID Special Pandemic Leave processing

In compliance with the request for Item 13, we enclose:

- HR Delegation COVID-19 Pandemic Event – Paid Special Pandemic Leave signed 23 November 2022;
- Pandemic Directive 01/20 – Employment Arrangements in the event of a Health Pandemic
- Implementation Guideline – Directive 01/20 dated 14 April 2020.

Item 14 - Text message and electronic communication corpus - Appellant and Line Manager concerning COVID Special Pandemic Leave

The requested information in Item 14 is not in control or possession of Metro South Health. The line manager was not issued with a Queensland Health mobile device, and as such the SMS messages exchanged between the Applicant and the Line manager relating to the pandemic leave request was made using the line manager's personal mobile device. The Queensland Health retention policy does not apply to employee's personal mobile.

The Switchboard line manager was issued with Queensland Health mobile device in or around the third quarter of 2024.

Item 15 - Leave ledger - 19 March 2024

In compliance with the request for Item 15, we enclose:

- Leave takings report for the Applicant 19 March 2024.

Item 16 - AVAC payroll export - Appellant (PRN 15397775 and PRN 15605601).

In compliance with the request for Item 16, we enclose:

- Daily Staffing variance form for the period 5 to 18 February 2024.
- Daily Staffing variance form for the period 19 February 2024 to 3 March 2024.

Item 17 – Comparator payroll export

Metro South Health objects to the production of Item 17 on the basis of Rule 64E(4)(e). Disclosing the requested information would infringe upon the human rights to privacy and reputation of the team members within the switchboard team, because:

- The staff members' identity can be easily re-identified even if redacted.
- The Logan Hospital Switchboard team is comprised of a small team.
- Given that the Applicant is a current employee within the same team, re-identification of the other team member would create a real risk of harm to their reputation.

Item 18 - Communication Book and 15 November 2023 Business Case minutes

Metro South Health objects to the production of Item 18 on the grounds of irrelevance (Rule 64E(4)(b)). The Notice indicates that the destruction of the record had already been pleaded at Form 9A, Part B, para 2 Stressor I(b) (Form 24, Para 14). In those circumstances, it is difficult to identify the relevance of the present request, given that the issue appears to have already been conceded.

Item 19 - Union delegate correspondence - server-side Exchange server

Metro South Health objects to the production of Item 19 on the basis of Rule 64E(4)(a) and (f).

Metro South Health employed approximately 20,006 MOHRI occupied headcount during the period of the dates of the Notice. The requested documents require searching through these staff's email messages received from Together Queensland. Compliance with a request of this date range and email recipients would require approval to engage external resources and assistance from eHealth Queensland at the Department of Health, resulting in a significant administrative and operational burden. We consider that the date range ought to be narrowed to 17-18 March 2024, limited to emails received from Together Queensland by the Switchboard line manager and their Director, or by members of the Logan Hospital Human Resources team who were employed by and remained with Metro South Health.

Further, any disclosure of correspondence involving Together Queensland would also require consultation with or issuing a production notice to them (as an affected party), if they were included in the email communication.

Item 20- Operational HR reaction to PID 24-ESU-1130

The information requested in item 20 does not exist, as the reference cited in the Notice being "PID24-ESU-1130" commenced in November 2024, which falls outside the specified search period of 13 May 2024 to 31 May 2024.

In the event you have any questions or concerns about this correspondence, I have arranged for a Principal Lawyer from the Metro South Health Legal team to be available to assist. Please contact metrosouthlegal@health.qld.gov.au

Yours sincerely



Digitally signed by
Noelle Cridland
Date: 2026.06.05
14:54:08 +10'00'

Ms Noelle Cridland

Chief Executive

Metro South Hospital and Health Service

Enc. Documents relating to items 6, 11, 12, 13, 15 and 16 of the Notice

Cory Shepherd_ECC further information

From: Lyndelle Forrest

Lyndelle.Forrest@health.qld.gov.au

To: coryshepherd1@hotmail.com

coryshepherd1@hotmail.com

Cc: LBH Injury Management

LBH_InjuryManagement@health.qld.gov.au

Sent: Tuesday 7 July at 10:54

Good morning Cory

I refer to the Employee Capability Checklist (**ECC**), completed by Dr Day Hong Ma of My Doctor Clinic on 3 July 2026 and returned to the Injury Management Team at 2:30pm that day. It is further noted that due to a section of this document being missing, you were asked to resubmit your ECC for consideration, you sent this on Friday at 3:06pm.

While you have been certified fit to return to work effective from 3 July 2026, this certification is subject to reasonable adjustments being made to your duties and hours of work. As the completed ECC was received late on Friday, 3 July 2026, the Health Service was unable to review Dr Hong Ma's medical recommendations at that time to determine whether the recommended restrictions and adjustments could be reasonably accommodated and whether it was safe for you to return to duty.

I can advise that your ECC has now been reviewed and carefully considered. However, before a final decision can be made regarding your return to the workplace, further information is required. Specifically, it is noted that your treating medical practitioner has advised that there are unresolved workplace matters involving line management. At this stage, the Health Service does not have sufficient information to understand the nature of these matters or assess whether they present any psychosocial hazards or risks that may affect your proposed return to work.

The Health Service remains committed to supporting your safe and sustainable return to work, with your health, safety and wellbeing remaining central to that process. Accordingly, further information is required to enable the Health Service to assess any identified risks, determine whether reasonable adjustments can be supported, and ensure that any return to work is planned and managed in accordance with its obligations under work health and safety legislation.

To assist us in expediting consideration of your reasonable adjustments, please provide further clarification and/or reports referenced below, within seven (7) calendar days.

ECC Recommendation	Further information required
<i>Diagnosis of Major Depressive Disorder with anxious distress, a work-related psychological injury with onset 18 June 2024, with symptoms exacerbated on exposure to identified workplace stressors (complaint handling; being held accountable and blamed for failures of others; unpredictable rostering.</i>	The Health Service is seeking to better understand the work-related psychological injury for which you are currently receiving treatment from a psychologist. It is also noted that Dr Day Hong Ma has referenced a report from your treating psychiatrist dated 13 February 2025. To date, the Health Service has not been provided with, nor had the opportunity to review, this report. To enable the Health Service to appropriately consider and address these matters, further information and/or a copy of this report is requested regarding the workplace issues referred to in the ECC and their impact on your capacity for work.

Complaint handling No complaint-handling duties, complaints received at the Switchboard to be logged and redirected, not actioned or resolved by Mr Shepherd.	The Health Service understand that the established process for management of complaints is for all patient concerns to be redirection to the Consumer Liaison Services (CLS). There is no expectation for Switchboard operators to manage or deal with these matters. Where a consumer raises concerns in relation to a Switchboard employee, these concerns are referred to the Manager, Switchboard or in her absence, CLS. To understand this concern better and adjustments required, please clarify what features of complaint handling are being referenced and you are expected to resolve within this recommendation.
Predictable Rostering practices, minimum 10-hour break between shifts	Rosters are generally published with four (4) weeks' notice, on four-week rotation. Occasionally, in extenuating circumstances these might be delayed. However, all rosters are released with a minimum of two weeks' notice. The Health Service is not aware of breaching

	this provision and has not received any Workload Concerns to review, can you please clarify the concerns relating to not providing the minimum break between rostered shifts? Noting that rostering currently provides more than 10-hour breaks between shifts, however, acknowledge you also signed an 8 hour agreement on 17 June 2020, which allows you to work with only an 8 hour break, however this is only applied where staff initiated shift swaps have occurred. Please clarify the specific stressors which may exacerbate symptoms.
Being held accountable and blamed for failures of others	It is unclear what matters you are being held accountable and blamed for based on the failures of others or what these concerns relate to. To-date the Health Service is not aware of any concerns being raised for appropriate management. Further clarity on this would assist in understanding this matter.

Until this information is available and has been appropriately assessed, the Health Service is not in a position to safely facilitate your return to normal duties.

Should you have any further questions or concerns, please do not hesitate to contact me via the details below.

Kind regards

Lyndelle Forrest
Senior Consultant
Human Resources
Logan and Beaudesert Health Service
[Metro South Health](#) | 

e. Lyndelle.Forrest@health.qld.gov.au | [Chat on Teams](#)
p. [07 3542 2028](tel:0735422028)
a. Mon - Fri 7:30am - 4:00pm



Metro South Health recognises and pays respect to the traditional custodians of the land and waters – the Yugambeh, Quandamooka, Jaggera, Ugarapul, Turrbal, and Mununjali peoples – and to Elders, past, present and emerging.

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maintaining and ensuring appropriate use of its
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Consultation Paper - Proposed Rosters for Switchboard Services, Logan Hospital

1. Purpose

The purpose of this document is to reinvigorate the consultation process for a proposed roster for Switchboard Services, Logan Hospital, Corporate Services, Logan and Beaudesert Health Service (LBHS), Metro South Health (MSH).

2. Background

On 22 July 2021, Metro South Health (MSH) management, affected staff and Together Queensland representatives commenced negotiating a new roster for Switchboard Services, Logan Hospital.

Over the course of these negotiations, valuable feedback from staff and Together Queensland has been considered and included in numerous draft rosters.

In August 2024, Switchboard Services, Logan Hospital was realigned from Health Information Management Services (HIMS), MSH to Corporate Services, LBHS, MSH.

3. Proposed change

Based on the comprehensive feedback to date and acknowledging the passage of time since roster negotiations commenced; LBHS management have developed a new proposed roster (**Attachment 1**) to reinvigorate the consultation process.

The proposed roster is intended to introduce a more equitable roster for Switchboard Services, Logan Hospital staff and includes:

- Redistribution of nights for greater equity based on FTE.
- Rotational roster for predictability and equity to all staff ensuring fair distribution of penalties.
- Extension of the current weekday (Mon-Fri) 0900-1400 five (5) hour shift to a 0700-1500 eight (8) hour shift.
- An additional eight (8) hour shift (1200-2000) on weekdays.
- An additional eight (8) hour shift (0700-1500) on Saturdays, to assist with call volume.
- An additional eight (8) hour shift (0600-1400) on certain days in the roster to provide for Switchboard administration, upskilling employees to provide them with a wider skill base, and more professional development opportunities.

The proposed roster will be trialled for a period of 6 months. At the conclusion of 6 months a review will be undertaken to evaluate the effectiveness.

To support effective operations of rostering and staffing for Switchboard Services, Logan Hospital; rostering principles and information forms part of **Attachment 2**.



4. Ballot and feedback

In accordance with Clause 6.2 of the [Hospital and Health Service General Employees \(Queensland Health\) Award – State 2015](#), agreement between MSH and the union, or MSH and the majority of employees affected must occur.

Agreement is defined as obtaining consent from the majority (50+1%) of affected employees, with employees invited to cast their vote and/or provide feedback via [clicking on this link](#) or scanning this QR Code:



5. Support for affected employees

LBHS is committed to the health, safety, and wellbeing of all our employees and acknowledges change may be a difficult time for affected employees. The following support mechanisms are offered to staff during this time:

- Employees may access the Employee Assistance Program by telephoning TELUS Health on 1800 604 640 and accessing information at <http://qheps.health.qld.gov.au/eap/>.
- Availability of unit management
- If an employee is a member of a Union, please contact your Union Organiser or Delegate who can also provide industrial relations and other guidance and support at this time.

6. Union Contacts

Contact/ Title	Name	Email	Phone Number
Together Queensland (TQ)	James Douglas	james.douglas@together.org.au health@together.org.au	

7. Proposal on endorsement (after consultation)

Should the proposed roster (**Attachment 1**) be agreed between MSH and the Together Queensland, or MSH and the majority (50+1%) of affected employees, it is proposed the roster be trialled for 6 months, after which an evaluation will be undertaken to review the effectiveness.

In the event agreement is not reached, LBHS commits to working with affected employees and Together Queensland to amicably resolve concerns with a view to reach agreement, prior to the proposed implementation date.

8. Dates

Date	Activity
12 November 2024	Consultation period opens
8 December 2024	Consultation period closes
16 December 2024	Feedback considered and addressed
20 January 2025	Proposed roster implemented

9. Approval

Name		Approval	
Approved to commence consultation:		Signature	
Scott Hughes Director Corporate Services Logan and Beaudesert Health Service		Date	12 / 11 / 2024

Name	Week Three							Week Four						
	Monday	Tuesday	Wednesday	Thursday	Friday	Saturday	Sunday	Monday	Tuesday	Wednesday	Thursday	Friday	Saturday	Sunday
	0700-0900	0900-1100	1100-1300	1300-1500	1500-1700	1700-1900	1900-2100	0700-0900	0900-1100	1100-1300	1300-1500	1500-1700	1700-1900	1900-2100
A Existing Perm Appointment														
B Existing Perm Appointment														
C Existing Perm Appointment														
D Existing Perm Appointment														
E Existing Perm Appointment														
F Existing Perm Appointment														
G Existing Perm Appointment														
Part Time 0.8 FTE, 64 hr														
A Existing Perm Appointment														
Part Time 0.7 FTE, 56 hr														
A Existing Perm Appointment														
B Existing Perm Appointment														
Part Time 0.7 FTE (New)														
A Perm Appointment (extra night)														
Part Time 0.5 FTE (New)														
A Perm Appointment (extra night)														
Part Time 0.6 FTE														
A Existing Perm Appointment														
Part Time 0.5 FTE														
A Existing Perm Appointment														
Total Hours														

Attachment 2: Switchboard Services, Logan Hospital Rostering Principles and Information

ROSTER PRINCIPLES

- **Shift Swaps:**

Switchboard staff are to provide three working days' written notice and AVAC is to be completed for shift swap requests by staff members and sent to Switchboard Manager for approval. All shift swaps for weekends must be made to management for consideration by the Wednesday prior and management agree to review within 48 hours with written response to requestor. If shift swap requested is not approved, then staff are required to work their original rostered shift.

- An AVAC is to be completed when adjustments need to be made to a rostered shift e.g., Shift swaps.
 - Please ensure all fields are completed and information is correct.
 - For assistance in completing an AVAC please consult the Switchboard Manager.

- **Sick / Emergent Leave:**

To ensure shift coverage can be found it is requested as much notice as possible be given if calling in for sick or emergent leave, particularly for night shifts and early morning shifts, however a minimum of two hours' notice is required unless extraordinary circumstances prevent appropriate notice from being given.

During Business Hours M-F, Switchboard staff are to call through to the Switchboard and ask to be referred/transferred through to the Switchboard Manager, who will take the call and provide coverage for the shift.

After-hours, Switchboard staff are to call through to the Switchboard Oncall for any sick/emergent leave. The Switchboard staff who take the call are then responsible for organising coverage and need to follow the below process. Sick/emergent leave will first be offered to any permanent/temp part-time staff members. If the shift needing to be filled is not congruent with other part time shifts, then the shift will be offered to casual staff. If in the event that a casual is also unavailable, available full-time staff will then be offered overtime, approval from the Switchboard Oncall must be sought prior to a full-time staff member being offered overtime. Once the shift has been covered, a detailed email from the Logan Switch account will need to send to the Switchboard Manager/Oncall outlining who has been called, at what time the call was made who declined/accepted the shift.

- On the days where there is an additional 06:00-14:00 or 07:00-15:00 shift that requires covering of emergent leave, coverage of these shifts is at the discretion of the Switchboard Manager based on operational need. Therefore, contact must first be made with the Switchboard Manager to see if coverage for the shift is necessary.
- When there is a duplicate 06:00-14:00 shift (apricot in colour on the roster) depending on staffing levels in Switchboard and call volume at the time, this shift can be used for switchboard operation and/or for switchboard administration and to up-skill staff, training and provide professional development for switchboard staff.

Below process is to be followed by Switchboard afterhours for sick/emergent leave:

To be offered following below order

- Permanent part-time staff – always least amount of shifts gets asked first (up to 9 shifts)
- Temporary part-time staff – always least amount of shifts gets asked first (up to 9 shifts)
- Casual staff – always least amount of shifts gets asked first (up to 9 shifts)

Permanent and Temporary Full-time staff – last option and needs prior approval from Switchboard Manager.

- If there is no one available to work the full shift and need to split shift this also needs to be prior approved by the Switchboard Manager/Oncall
- All emergent leave and sick leave, please email Switchboard Manager/Oncall with shift changes and who has been called, what time the call was made, who declined/accepted shifts. Email needs to be sent from Logan switch account.
- Afterhours coverage needs to be prior approved by Switchboard Manager/Oncall if resulting in overtime. Email follows with shift changes and who has been called, what time the call was made, who declined/accepted shifts. Email needs to be sent from Logan switch account.

Meal Breaks

To ensure Switchboard is manned to an optimal level, it is important to adhere to the scheduled break times as listed below. If for any reason staff are unable to observe these times during a shift, please consult Switchboard Manager, noting the requirement that meal breaks are taken between the third and sixth hours of duty in accordance with section 16 of the Award and rest breaks are to be taken during the first and second half of the working day in accordance with section 17 of the Award. Should staff be requested to work additional hours they are advised to check with Switchboard Manager if they are eligible for a supplementary break. This is dependent on the number of additional hours worked. Noting the relevant section of the Award is section 13.7(c) of the Award as this details when an additional break becomes an entitlement and that is every four hours of continuous overtime.

Shift Start	1 st Break	2 nd Break	3 rd Break
06:00-14:00 A	0800 10 mins	10:30 30 mins	13:00 10 mins
06:00-14:00 B	08:10 10 mins	11:00 30 mins	13:10 10 mins
07:00-15:00 A	9:00 10 mins	11:30 30 mins	14:00 10 mins
07:00-15:00 B	9:10 10 mins	12:00 30 mins	14:10 10 mins
12:00-20:00	14:00 10 mins	17:00 30 mins	19:00 10 mins
14:00-22:00	16:00 10 mins	18:30 30 mins	21:00 10 mins
15:00-23:00	17:00 10 mins	19:30 30 mins	22:00 10 mins
22:00-06:00	12:00 10 mins	02:30 30 mins	05:00 10 mins
23:00-07:00	01:00 10 mins	03:30 30 mins	06:00 10 mins

ROSTER INFORMATION

- Penalties have been allocated based on FTE to ensure fair and equitable distribution of penalty shifts across the whole roster.
- In this roster nights are allocated based on proportion of FTE so to be fair and equitable to all Switchboard staff as far as possible and to maximise health safety and wellbeing, work-life balance and to minimise staff fatigue. Noting there is one exception to this rule and there are two new roster lines for a 0.7 FTE and 0.5 FTE which do have extra night shifts rostered (to reduce the number of nights our full-time and part-time staff need to work, providing a more equitable balance)
- This roster is a rotational and cyclic roster. Thus, staff work a line relevant to their FTE, e.g., weeks 1-4 and then they drop down to the line below and work their way through the various lines and back to the top again and repeat. For our part-time lines where there is only one line staff rotate through weeks 1-4 and then back to beginning of the roster pattern in week 1.

- Allocation of lines: Roster lines will be allocated based on payroll number.
- This roster will be trialled for duration of one whole rotation of the fulltime lines, for 6 months. Where we will then review and seek further feedback from staff. However, should there be any significant issues or faults identified in the roster during this trial these will be reviewed as they occur.
- Once implemented this roster will be projected forward for 12 months for staff advanced knowledge of the shifts.
- This roster has been created using principles from the 'Metro South Best Practice Rostering Guideline', which includes designing to mitigate a roster to try and mitigate fatigue, to promote fairness, equity and work-life balance whilst still meeting organisational staffing and service needs.
- Due to call volumes, there will be no reduction in workforce on Public Holidays.

Consultation outcome

Proposed Rosters for Switchboard Services, Logan Hospital

December 2024



1. About this document

This decision document provides an overview of the proposed roster for Switchboard Services, Logan Hospital, Corporate Services, Logan and Beaudesert Health Service (**LBHS**), Metro South Health (**MSH**).

This document summarises the consultation process undertaken with the affected employees, relevant unions and other key stakeholders. It outlines the decisions that have been made in accordance with government policy and relevant industrial obligations and reflects upon the feedback received and the influence this had on the final decision.

2. Consultation process

On the **14 November 2024** a consultation document and proposed roster for Switchboard Services, Logan Hospital was released for consultation. Affected staff, unions and stakeholders were invited to provide feedback by **8 December 2024**.

During this consultation process, all feedback was reasonably considered through a structured and transparent approach of collecting feedback through multiple channels, consideration of feedback and communication of the responses.

3. Summary of feedback and responses

A total of **nine (9)** submissions of feedback was received during the consultation period. The feedback themes and responses are provided in **Attachment One**.

4. Ballot results

In accordance with Clause 6.2 of the [*Hospital and Health Service General Employees \(Queensland Health\) Award – State 2015*](#), agreement between MSH and the union, or MSH and the majority of employees affected must occur prior to changes to a shift roster.

Agreement is defined as obtaining consent from the majority (50+1%) of affected employees, with all affected employees invited to cast their vote via an online Ballot Form.

17 affected employees were invited to cast a vote, with a total of **17** votes received. The breakdown of votes are as follows:

14 'Yes' votes received
3 'No' votes received

5. Decision

Following consideration of the feedback and **82%** of affected staff voting '**Yes**', the Director, Corporate Services, LBHS **approved** progressing with the proposed rosters, date effective **20 January 2025**.

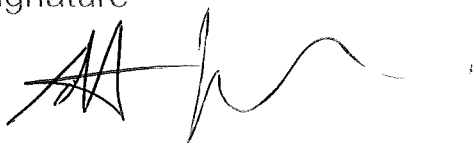
The proposed roster will be implemented on a trial basis for **six months**. On the conclusion of the trial period, affected staff will be invited to provide feedback on the effectiveness of the roster.

6. Supporting employees through change

LBHS recognises organisational change can be difficult for affected staff. Affected employees are offered the following ongoing support:

- Free and confidential support through the Employee Assistance Program. Information on the program and how to access it are available at:
 - [Metro South Employee Assistance Program \(EAP\)](#)
Or by telephoning TELUS Health on 1800 604 640

7. Signature

A handwritten signature in black ink, appearing to be 'S. Hughes', written over a horizontal line.

Mr Scott Hughes

Director

Corporate Services

Logan and Beaudesert Health Service

12/12/24

Feedback received		Response
Happy with this roster - [REDACTED]		Thank you for your feedback.
Hi it's [REDACTED] I am very happy with the new roster		Thank you for your feedback.
I agree to trial this roster [REDACTED]		Thank you for your feedback.
I'm happy to trial proposed roster, the only way is work the roster and shifts.		Thank you for your feedback.
I appreciate your email and for the proposed roster. I am in agreeance with the proposal and happy for this to be allocated. Although I do have concerns that I wish to raise, even though the idea is for this to be a permanent roster. Specifically, I am concerned that staff members that have been allocated to fulfill all their nightshift obligations and subsequently request a reduction or provide a medical certificate for fewer nightshifts, it could disrupt the equilibrium of the roster. This situation may lead to a recurrence of past scenarios, where one half of the staff had their nightshift commitments reduced, while the other half had to bear significantly increased nightshift duties to compensate for the shortage. Such a scenario could pose challenges for those staff members who find themselves with additional nightshifts in place of afternoon or day shifts due to others being unable or unwilling to fulfill their nightshift responsibilities. Is there a plan or pathway in place for this situation if it occurs as it has in the past? Also in accordance to page 1 of the roster proposed, "The proposed roster will be trialled for a period of 6 months. At the conclusion of 6 months a review will be undertaken to evaluate the effectiveness." What parameters would be evaluated in the review to determine the effectiveness of the roster after the 6 months? Thank you for your attention to this matter	- The effectiveness of the roster after the six-month trial period will be based on staff feedback/staff satisfaction survey. - In the event a staff member is unable to work nights due to a medical condition, this will be managed confidentially between the affected employee, Switchboard Manager and in consultation with the LBHS Injury Management and Human Resources teams in accordance with the provisions of the relevant legislation, industrial instruments, and Queensland Health HR Policies. - All shifts that require coverage will be managed by the Switchboard Manager in accordance with MSH Best Practice Rostering Guideline. Team members are able to speak to their colleagues and request a shift swap with sufficient notice to the Switchboard Manager.	

I would like to raise a concern regarding the distribution of night shifts among the .07 lines. I feel that these shifts should be allocated more equitably. Currently, one staff member is assigned as many as 9 night shifts, while others are only allocated 2 or 3. This disparity feels unfair and could be addressed by balancing the shifts more evenly among the team.

Thank you for your feedback.

- The 0.7fte (new) line was implemented as part of this proposed roster and consists mostly of nightshifts. At the time the temporary incumbent was converted from temporary to permanent, there were no vacancies within Switchboard Services. The affected employee was offered this line and accepted the terms and conditions of the role (primarily night shifts).
- This line was created to eliminate the need for the fulltime/part time staff to work more than four nights per month.

After looking at the proposed roster as a full time employee I have noted that the full time members have lost seven weekend penalty shifts over a seven month period as you probably already know historically full time team members have always had three weekends a month and can be shown on all previous rosters. We are being asked to take a significant pay cut and I would like to know if this is even a legal request. As full time employees we are the last team members to be asked to do any overtime therefore it would be unlikely we would be able to recoup our loss of earnings and I'm also wondering if the people who have worked on this roster would be prepared to take a pay cut with their own employment ? I have also noted that we are being asked to now do four nights a month and as you can see from previous rosters we have been doing only three. A pay cut and an extra night it hardly seems fair or equitable or not I. The best interest of us as QLD Health employees, i have also noted that there are numerous weeks that full time staff only have one day of in that particular week. I don't feel that this roster has been done with the best interest of full time staff maintaining fatigue management. Kind regards [REDACTED]

Thank you for your feedback.

- This feedback was considered but unfortunately, we are unable to offer full-time staff three weekends per month due to the requirement to roster penalty shifts fairly and equitably across all FTE lines.
- We also have an obligation to consider wellbeing and fatigue, and to avoid a situation where we are asking full-time employees to work proportionally more weekends than other employees.
- Penalties have been allocated proportionate to FTE. Some minor modifications have been made to the full-time line in terms of weekend shifts. This is a rotational, cyclic roster; whilst one fortnight may have less penalties it is evened out over the duration of the four week roster cycle.

<i>I do not agree to having to take a pay cut, with this roster we are losing 7 penalties. We have lost 4 Saturday shifts and 3 Sunday shifts. With this roster you are forcing a pay cut. Is that legal? The current rostering system we got 3 weekends a month and 3 night shifts. The current rostering system works if done fairly. Which one of you would take a pay cut? We have been doing 3 nights a month for months now this roster goes to 4 nights and less money. Is the union involved with this roster? Isn't this for permanent staff casual staff don't get to agree to this. Why can't we submit an email where we can read what we have written? It seems that we have a lot of late starts there was a survey done where all staff preferred the same start time which was at the early times. 6am 1400 pm 22.00 pm. Don't say we can swap shifts that is not as easily done. There are weeks where we have only one day off how is that fatigue management? How is my pay going to be boosted when we are the last ones to get any overtime? Please don't lie to us if you force us to trial this wage theft you have no intentions of changing it or stopping it. Can we speak to the union representatives about the loss of income for starters. The current rostering system could work if favouritism wasn't a factor and used to bully people.</i>	Thank you for your feedback. - Together Queensland were invited to provide feedback as part of this consultation and ballot process. - The proposed roster has been developed after many years of consultation and feedback from previous rejected rosters and in accordance with the provisions of Hospital and Health Service General Employees (Queensland Health) Award – State 2015 and the MSH Best Practice Rostering Guideline. - Start times and shift cross overs are in place to support effective shift handover and to mitigate against inadequate coverage, should a staff member be delayed in commencing their rostered shift due to unforeseen circumstances (traffic etc.). This also aligns with other MSH Switchboard Services who roster a variety of shifts to cover all hours - Clause 15.1(j) provides the basis on which rostered days off have been considered for this proposed roster: (j) <i>Rostered days off - where work is performed on other than a Monday to Friday basis</i> (i) <i>Unless prescribed elsewhere in this Award, all employees shall be entitled to not less than two whole consecutive rostered days off duty each week.</i> (ii) <i>In lieu of two whole rostered days off in each week, an employee may be allowed in each fortnightly period either one rostered day off in one week and three consecutive rostered days off in the other week, four consecutive rostered days off or two groups of two consecutive rostered days off.</i> (k) <i>Two consecutive rostered days off, one at the end of one week and one at the beginning of the following week may be counted as meeting the requirements of clause 15.1(j)(i).</i>
<i>Loss of wages with this roster so unfair. You all take a pay cut as well.</i>	Thank you for your feedback. - The proposed roster has been developed to support fair and equitable rostering and remuneration across all affected staff, proportionate to FTE.

-
- Further, we also have an obligation to consider wellbeing and fatigue, and to avoid a situation where we are asking full-time employees to work proportionally more weekends than other employees.
-

Switchboard Roster Feedback - For Delegates

From: Heath Moran heath.moran@together.org.au

To: Carolyn Jeffrey

Carolyn.Jeffrey@health.qld.gov.au, Patricia

Conaghan Patricia.Conaghan@health.qld.gov.au,

Cory Shepherd Cory.Shepherd@health.qld.gov.au

Cc: Billy Colless billy.colless@together.org.au

Sent: Monday 3 November 2025 at 10:57

**This email originated from outside Queensland Health.
DO NOT click on any links or open attachments unless
you recognise the sender and know the content is safe.**

Good morning DELEGATES!

Firstly, congratulations to you all on being endorsed to become workplace delegates!

You'll be receiving some invites to attend our delegate catchups as well as our monthly Local Consultative Forum.

In relation to the switchboard roster feedback, I've attached Chloe's feedback in a PDF attached in this email. Please note that while we're in negotiations with Chloe, this is not for wider distribution.

I have also attached the Consultation paper – which Chloe advised on Thursday last week has the amount of shifts each day that Switch can currently allocate.

I'd be eager to hear your feedback, and I'd like to come together this week to discuss your thoughts. I'll be in the hospital on Thursday around midday for a staff orientation so if you'd like to meet up on Thursday to discuss, please let me know what works for you. Once we've discussed it further amongst ourselves, I'll book in a time to meet with Chloe – likely next week (noting I'll be in Melbourne for work, Billy will be able to attend in my absence).

I look forward to hearing back from you.

Yours in Union,

Heath Moran

Organiser – South Team



0414 225 851

[heath.moran@toget](mailto:heath.moran@together.org.au)

her.org.au



Looking for information? Check the [Member Portal!](#)

View our [Privacy Policy](#) here.

Together recognises and respects the Aboriginal and Torres Strait Islander peoples of the lands upon which we undertake our work to promote and defend the interests and values of members. We acknowledge the Traditional Owners and custodians who have cared for this country for thousands of years.

Reasons for decision

24 October 2024

Our reference: 69983

Insurer reference: S23LW142013

Direct telephone: 07 3842 2787

Dear Mr Shepherd

RE: Application for review - Mr Cory Shepherd

I refer to the application for review received by the Workers' Compensation Regulator on 16 September 2024 in relation to the decision by WorkCover Queensland (WorkCover) to reject your application for compensation (the application).

Review decision of 22 October 2024

I have confirmed the decision of WorkCover to reject your application for compensation in accordance with section 32 of the *Workers' Compensation and Rehabilitation Act 2003* (the Act).

Background

You submitted an application for compensation to WorkCover on 1 July 2024 for a psychological injury sustained on 18 June 2024 as a result of workplace stress and bullying. At the relevant time, you were employed on a full-time basis as an Administration Officer by Metro South Hospital and Health Service, undertaking your duties at Logan Hospital.

WorkCover investigated your application and on 13 September 2024 issued written reasons for their decision to reject your application pursuant to section 32 of the Act. In arriving at this decision, WorkCover identified four factors causative of your psychological injury – manager's failure to follow rules, management's failure to take action regarding an ethical complaint made by you, your payment being withheld, and roster guidelines not being followed. WorkCover determined that the first and third factors were unable to be substantiated, while the second and fourth factor amounted to reasonable management action taken in a reasonable way. This meant to the extent that your injury arose out of your employment where employment was a significant contributing factor, it arose out of reasonable management action taken in a reasonable way. Consequently, the

Reasons for decision

exclusionary provisions in section 32(5) of the Act were enlivened and your psychological injury was excluded from the definition of 'injury' according to section 32 of the Act. As a result, your claim was not one for acceptance.

It is this decision you seek to be reviewed.

Grounds for review

On 16 September 2024, you lodged an application for review with the Workers' Compensation Regulator, requesting a review on the following grounds:

- there was significant oversight by WorkCover in the handling of your claim
- you have additional documentation to provide
- WorkCover failed to consider the entirety of the evidence
- you have several ongoing workplace concerns.

In addition to the application for review, you also submitted:

- copy of email of Ms Taegan Griffiths, employee at Together ASU, dated 24 July 2023
- copy of your email to Ms Griffiths dated 24 July 2023
- copy of your email to Together ASU dated 18 May 2024
- copy of email of Ms Heather Hayes, Organising Director at Together ASU, dated 20 May 2024
- annotated email of Ms Hayes dated 20 May 2024
- your email to Ms Zaida Variava, employee at Together ASU, dated 16 June 2023
- copy of your email to WorkCover dated 26 August 2024 regarding a union delegate
- copy of the first page of WorkCover's reasons for decision dated 13 September 2024
- annotated response of employer dated 15 August 2024
- copy of various text messages between you and Ms Chloe Taylor, your Line Manager
- annotated extracts from undated documents.

Review process

The review is an independent administrative process or a 'review on the papers'. This means that as the decision-maker, I have considered the material on WorkCover's claim file (S23LW142013) and any new information provided by you or another party during the review process.

All of the evidence including submissions, the application for review and WorkCover's file has been considered, although I may not specifically refer to all of the documents in my review decision.

Reasons for decision

Evidence considered

In undertaking this review, I have considered all of the evidence on WorkCover's file as well as the evidence provided to the Workers' Compensation Regulator, and in particular the following documents:

- application for compensation, lodged 1 July 2024
- reasons for decision of WorkCover dated 13 September 2024
- application for review, lodged 16 September 2024
- WorkCover's communications report (communications report), as at 18 September 2024
- hospital and health service general employees (Queensland health) award – State 2015
- recruitment and selection policy dated June 2020
- fatigue leave following weekends and/or rostered days off policy dated June 2020
- fatigue risk management policy dated February 2021
- union encouragement policy dated August 2021
- corrupt conduct complaint form 13 May 2024
- attendance variation and allowance claim signed by Ms Taylor dated 28 May 2024
- validation of claims older than 3 months form signed by Ms Taylor dated 28 May 2024
- statement provided by you on 12 July 2024
- two statements of Ms Carolyn Jeffrey, Switchboard Operator, dated 18 Jul 2024 and 1 August 2024
- two responses provided by you dated 9 August 2024
- two responses provided by you dated 19 August 2024
- various emails sent by you to WorkCover
- various emails between you and other employees
- various switchboard services rosters
- various annotated pay advices
- various text messages between you and Ms Taylor
- annotated employer's response dated 15 August 2024 submitted by you at review
- two responses of the employer dated 15 August 2024 and 6 September 2024
- three work capacity certificates of Dr Peter Hawes, General Practitioner, dated 1 July 2024, 11 August 2024, and 8 September 2024
- work capacity certificate of Dr Ki Pang, General Practitioner, dated 7 August 2024.

Issues for determination

Section 108 of the Act states that compensation is payable for an 'injury' sustained by a 'worker'.

Reasons for decision

It is not disputed that you are a 'worker' within the meaning of the Act. This is not an issue for review.

The issue to be decided at review is whether you sustained an 'injury' within the meaning of section 32 of the Act. If the provisions of section 32(5) of the Act are satisfied, they exclude a psychiatric or psychological condition from the definition of 'injury' within section 32(1) of the Act.

Therefore, I must determine whether it is more probable than not that:

- you sustained a personal injury
- the personal injury arose out of or in the course of your employment
- the employment is a significant contributing factor to the injury
- the injury arose out of or in the course of:
 - reasonable management action taken in a reasonable way by the employer in connection with your employment
 - your expectation or perception of reasonable management action being taken against your
 - action taken by WorkCover in connection with your application for compensation.

I will now consider each element in turn.

Did you sustain a personal injury?

The work capacity certificate of Dr Hawes dated 1 July 2024 stated you sustained anxiety and stress. This was maintained in the work capacity certificate of Dr Pang dated 7 August 2024 and two work capacity certificates of Dr Hawes dated 11 August 2024 and 8 September 2024.

Having regard to the medical evidence, and in the absence of evidence to the contrary, I am satisfied you sustained a personal injury of a psychological nature.

Did the personal injury arise out of or in the course of your employment and was the employment a significant contributing factor to the injury?

The communications report details a conversation you had with WorkCover on 4 July 2024, whereby you reported your psychological injury was caused by workplace bullying, predominantly with the manager and director.

In the statement provided by you on 12 July 2024 you highlighted various issues which you believed were causative of your psychological injury. You first indicated the employer

Reasons for decision

communicated they would provide accredited payment for a recognised Australian education, and other additional payments, but you never received either despite providing to the employer your accredited diploma of fitness. You noted you made multiple requests for these payments, which were treated with hostility and declined. You then expressed Ms Taylor was determined to make you feel undervalued and undermined by her actions, which included spreading rumours about you. You also noted that while you were employed as a casual, you signed an agreement allowing for an 8-hour break between your shifts, even though the standard provided for a minimum break of 10 hours or an entitlement to fatigue pay.

Another concern which you expressed was the lack of advertising for the position for which Ms Cherie Abbott, employee in Switchboard Services team, was hired, which prevented other employees from applying. A major issue for you was the rostering. You indicated there was a reduction in penalty shifts, an increase in fatigue and your experience was undervalued in favour of less qualified individuals due to bias and friendships. Your requests for days off were ignored while other employees were granted shifts as per their requests. The roster was not fairly allocated, and employees favoured by Ms Taylor received shifts according to their requests while you were subjected to arbitrary scheduling.

Following a complaint made by you, you noticed you were treated differently, and your questioning of policies and procedures increased tension. You also disclosed a specific incident which you believed contributed to your injury. You corrected a mistake and noted the importance of verifying numbers in the communication book, but the note was torn out the following day and you were accused by Ms Taylor and her friend of having malicious intent in writing the note. You expressed it seemed that all actions of the director and the manager were inconsistent with legislation, policy and interest, and any concerns raised by you were met with grief. You then provided another example of an employee being hired for a newly created position, when there was no consultation, advice, transparency or equal opportunity with respect to the creation and hiring of this position. You then continued to express concerns regarding further payment issues. During your employment you also raised the absence of a union delegate and expressed your interest in obtaining training to become one. After some time, Ms Taylor told you no one replied to her advertisement of the role, even though you noted you expressed your interest in the position via email to Ms Taylor and you were never directly asked about you taking the role. You stated Ms Taylor was avoiding appointing you as the delegate on purpose.

Reasons for decision

While I acknowledge the causative factors identified by you, I am also mindful of the finding in *Rowe v Q-COMP*,¹ that an injured worker is not always best placed to identify the cause of a psychological injury and that factors identified by an injured worker may not be found upon investigation by health professionals to be real or actual factors contributing to the injury. Accordingly, I must consider the medical evidence concerning whether these factors were causative of your injury.

The work capacity certificate of Dr Hawes dated 1 July 2024 attributed your psychological injury to harassment at work, specifically the ongoing violation of workplace rules and victimisation of you by bosses. This was maintained in all three later work capacity certificates.

According to the communications report, WorkCover spoke with Dr Hawes on 2 September 2024, whereby Dr Hawes opined your psychological injury was caused by management failing to follow rules, management's lack of action in response to your complaint, the withholding of your payment and non-adherence to and manipulation of the roster.

Upon review of the available factual and medical evidence, I find the following factors have been nominated by you and identified by your doctor as medically causative of your psychological injury:

Factor 1: Non-adherence to rules by management

Factor 2: Management action taken in response to your complaint

Factor 3: Withholding of pay

Factor 4: Non-adherence to and manipulation of the roster guidelines

I must now determine whether these factors are able to be substantiated.

Factor 1: Non-adherence to rules by management

In the overview of events provided by you to WorkCover on 12 July 2024, outlined management did not adhere to rules in the following aspects:

- hired employees without advertising the position and providing other employees with an opportunity to apply
- failed to obtain a union delegate, which was a departmental requirement
- did not keep employees accountable, including managers, who failed to follow rules

¹ [2009] QIRC 9; 190; QGIG 93.

Reasons for decision

- Ms Taylor and Ms Stibbard allocated themselves as the on-call contacts after hours without departmental input
- issued directives without consultation or input.

I note the supplied Recruitment and Selection policy dated June 2020 records, allows for direct appointments to be made at the discretion of the delegate.

The supplied Union Encouragement policy dated August 2021 states the employer recognised the constructive role of democratically elected union delegates and that role was to be formally recognised, accepted and supported on conditions that unions notify the employer of such delegates.

You supplied several emails in support of your application which I have summarised below:

On 22 July 2021, Ms Tammy Reese, Director, emailed staff members regarding the Business Case for Change, which she attached to the email. She invited feedback from employees and relevant unions regarding the proposed changes and noted the consultation period was open from 22 July 2021 until 5 August 2021, following which the feedback would be reviewed and considered prior to the implementation plan being drafted.

In an email dated 8 December 2021, Ms Reese informed staff members that Ms Sandra Johnstone, Switchboard Line Manager at the time, was extending her leave and then going on secondment, which meant Ms Reese was looking to backfill her position temporarily for three months as soon as possible. Ms Reese stated all staff were welcome to apply and in the meantime, she would continue to act as line manager.

In an email dated 27 October 2022, Ms Reese invited employees to attend a consultation meeting regarding the proposed organisational change.

The email of Ms Reese dated 2 November 2022 attached another Business Case for Change and encouraged staff members to review it and provide feedback regarding it. She confirmed the consultation period was open and would close on 16 November 2022.

An email of Ms Taylor dated 16 February 2023, sent at 6:03 am, informed employees of her changed hours in the office that day.

Reasons for decision

In emails dated 8 May 2023 and 9 May 2023, Ms Taylor informed the team she was not attending the office on those days and to contact her by telephone for anything urgent.

On 12 May 2023, Ms Taylor emailed the team and confirmed she would be in the office the following Monday.

Ms Ellen Stibbard, Switchboard Telecommunications Coordinator, emailed the staff members on 18 July 2023 to provide an update with respect to her role. She explained her role was a project role, which was a temporary position that focused on fixing the database used by the switchboard. She informed that while she fixed it, she would be removing everyone's access to it, and asked for anyone who needed to amend, add or remove entries, to directly contact her or Ms Taylor.

In an email dated 23 August 2023, Ms Taylor stated she was a full-time employee and worked flex hours, and acknowledged she recently had been starting and finishing at different times. She stated she was trying to set a routine around her children. She noted her start time would range between 6:00 to 9:00 am, and her finish time would range between 2:00 to 5:00 pm, and when there were any changes to her times or presence, she would always inform Ms Reese and the team regarding it.

Ms Taylor emailed the team on 1 September 2023 and informed she would be on leave from 13 September 2023 until 18 September 2023. During this time, she directed employees to contact Ms Stibbard, and Ms Taylor if they cannot reach Ms Stibbard.

On 12 September 2023, Ms Taylor emailed the team and reminded she commenced her leave on 13 September 2023 until 17 September 2023.

The email of Ms Taylor dated 14 November 2023 stated she would be attending the office at around 10:30 am that day.

On 16 November 2023, Ms Taylor informed staff members, via email, that she had a planned day off that day.

The email of Ms Taylor dated 30 November 2023 informed she would be away the following day as it was her planned leave day.

Reasons for decision

On 15 April 2024 Ms Taylor emailed the team attaching the updated rosters. She wrote herself and Ms Stibbard added after-hours on call and the roster showed who was on call after hours.

Ms Taylor explained employees were able to contact Ms Taylor or Ms Stibbard after hours for any queries.

On 19 April 2024, Ms Taylor emailed the team and expressed that due to errors being made with respect to data entry, a new process was to be followed which included more checks to ensure accuracy of data entry.

Ms Taylor emailed the team on 17 May 2024 and clarified when she would be in the office, when she would be working from home and when she was contactable.

On 18 May 2024, you emailed Together ASU and expressed the switchboard department did not have a designated union delegate, and in these circumstances, you volunteered to become the delegate. To do so, you required training, and thus requested access to paid union training from Together ASU.

You received a response from Together ASU via email on 20 May 2024. It was stated the team would contact you regarding the training and becoming a delegate.

Ms Reese emailed employees on 30 May 2024 to provide an update regarding organisational change. She stated the feedback with respect to the Business Case for Change had been invaluable and the Implementation Plan was at the time with Human Resources for final review as per standard process and would proceed to the delegate for consideration. Further, Ms Reese informed Ms Taylor, who had been backfilling the Switchboard Manager role, had been permanently appointed through a higher classification review process following her temporary appointment from an expression of interest in March 2022. The employer planned to advertise the vacant AO3 Administration Officer position in the coming weeks.

The email of Ms Taylor dated 18 June 2024 informed staff members of her work arrangements.

You supplied to WorkCover a statement of Ms Jeffrey dated 18 July 2024 which submitted Ms Taylor was handed her position and was raised from an AO4 to an AO5 with no formal process, and no one was able to apply for this position. Ms Jeffrey wrote that since Ms Taylor commenced the manager role, her actual start and finish times were never transparent. She came when she liked and left in less than 8.5 hours.

Reasons for decision

In the statement dated 1 August 2024, Ms Jeffrey reiterated Ms Taylor came to the office late and left early. Ms Jeffrey asserted she was committing fraud, but it was ignored, and concluded management did not follow the code of conduct or support their staff.

In your procedural fairness response dated 9 August 2024, provided to WorkCover, you stated the following issues in the workplace required addressing: hiring of staff members, lack of union delegation, bullying, addressing changes in workplace cooperatively and overall adherence to policies. You specified the employer did not follow many principles in the HHS Act and EB11 agreement, particularly section 1.9 of the agreement. Further, Ms Taylor consistently removed the first aid kit in the department and placed it in non-allocated spots.

In your further response dated 19 August 2024, you wrote Ms Taylor paid herself for her 30-minute meal break which was supposed to be unpaid. Further, you noted Ms Taylor was very selective about who she updated regarding her office hours.

In a separate response also dated 19 August 2024, you expressed the proposal for a business case for change was not compliant with evidence-based practice or the FRMS. You stated the changes made were questionable and showed unfair bias and corruption, and the change in the business structure occurred without consultation. You reaffirmed Ms Stibbard was hired without consultation and without a formal recruitment phase prior to her commencement, which removed the opportunity for others to apply and created an unfair workplace. You asserted the first time the role was advertised, Ms Stibbard was confident she would be assigned the newly created role as she obtained it before its existence, and it was not advertised to the public or staff beforehand. You then referred to Ms Taylor and Ms Stibbard initiating themselves to be on call after hours. You felt this was a plot to obtain additional income, which was a misuse of government funds.

You emailed WorkCover on 26 August 2024 and expressed that under the EB11 agreement, a union representative was to be provided by the employer for the employees to raise their concerns, but when you informed Ms Taylor and Ms Reese of your situation, they could not identify the current delegate. Even though you offered to complete the necessary training to become the delegate, and a group email was sent to allow everyone the opportunity to participate, Ms Taylor later confirmed no one had responded. You concluded the department took no further action regarding you becoming the delegate or to provide you with a union delegate.

Reasons for decision

In an email to WorkCover dated 29 August 2024, you referred to Ms Taylor's email dated 15 April 2024 regarding the after hours on call, and stated Ms Taylor and Ms Stibbard decided to implement this without consultation.

You emailed WorkCover on 30 August 2024 and in referring to the email of Ms Taylor dated 19 April 2024 regarding the new process for data entry, commented she failed to consult regarding this new process.

During the review, you submitted an annotated response of the employer, whereby you wrote, in relation to the union delegate issue, that Together ASU forwarded the employer the forms for you to become the delegate, but the employer discriminated against you and did not action this.

The response of the employer dated 15 August 2024 referred to various issues raised by you. The employer first explained the union delegate nomination and election process was managed by the respective union who provided coverage, which meant your concerns with respect to it were not the responsibility of the employer. The employer referred to the Union Encouragement policy in support of this. It was then submitted the employer conducted all appointments in accordance with their Recruitment and Selection policy. The selection process for Ms Danielle Cook, previous Switchboard Manager, and Ms Taylor were advertised to the entire Switchboard team through an Expression of Interest process and an interview process was undertaken, at the end of which it was decided Ms Cook and Ms Taylor were suitable for the position and were given the opportunity to job share. Ms Taylor was subsequently appointed to the Switchboard Manager role according to the Public Sector Commission Directive 03/23 Review of acting or secondment at higher classification level. With respect to your concern regarding Ms Taylor not working her required hours, the employer wrote this was incorrect and Ms Taylor consistently fulfilled her contracted hours each week and adhered to the employer's flexible working arrangement policy. It was noted that when Ms Taylor would start work late or finish early, she communicated this to her team, and there were no concerns regarding her punctuality or attendance.

I appreciate that you perceived various management actions to be taken in violation of rules. However, I find there is insufficient evidence to substantiate this. With respect to the hire of employees, I find the employer did not breach the rules and adhered to their policies. The employer also was not responsible for obtaining a union delegate, but rather was responsible for merely allowing one to be present, and thus also did not breach rules by the lack of a delegate due to no fault on their part. No evidence has been provided in support of your assertion that management did not keep employees accountable for their rule-breaking activities and

Reasons for decision

consequently I find this also was not substantiated. Finally, you did not provide evidence in support of your submission that changes made to processes and structures were in violation of rules, and the employer confirmed the changes were valid. Therefore, I find this factor is unable to be substantiated.

Factor 2: Management action taken in response to your complaint

You filled out a corrupt conduct complaint form on 13 May 2024, whereby you stated it appeared Ms Taylor was engaging in fraudulent activities by claiming unnecessary on-call hours in the department and logging on from home when she was not present. You further alleged Ms Taylor displayed inappropriate behaviour, such as harassment, bullying, dishonesty, and an autocratic management style.

On 15 May 2024, Ms Janelle Loader, Director of the Ethical Standards Unit, emailed you to acknowledge receipt of your complaint form. She noted that since you had further information to provide, they would wait for further details from you to progress the assessment.

Ms Murray Brooks, Principal Investigator, emailed you on 20 May 2024 to follow up further information regarding your complaint. She advised that to progress the assessment of the broad concerns raised by you, they required particulars for each matter.

On 27 May 2024, Ms Brooks emailed you regarding your complaint. She stated that in the absence of any particulars relevant to your allegations, they finalised an assessment of the matter and determined it did not raise reasonable suspicion of corrupt conduct and would refer your concerns and complaints to Human Resources. However, they invited you to provide further information if you wished to in the future.

In an undated email, Ms Reese scheduled a meeting with you at 10:00am on 12 July 2024 to discuss the outcome of your complaint.

In an email dated 12 July 2024, Ms Reese stated she rescheduled the meeting to 16 July 2024 since you were away on that day.

The overview provided by you on 12 July 2024 stated that following the complaint, you noticed a change in your treatment, and you believe your questioning of policies and procedures was perceived as a challenge to your manager and director, leading to further tension.

Reasons for decision

In your response dated 19 August 2024, you asserted you raised concerns regarding corruption and actions of Ms Taylor. It appeared the claims were taken seriously due to the response you received on 20 May 2024 and the subsequent closing of the case on 27 May 2024. However, you found the short period within which the case was closed concerning. Due to the great importance of the matter, and the Unit's reliance on the information presented by you, you assumed there was no formal investigation. You noted that due to the short timeframe and your work commitments, you were unable to gather all the necessary evidence to support your claims.

In their response dated 15 August 2024, the employer stated you submitted a complaint on 16 May 2024 to the Ethical Standards Unit broadly outlining potential inappropriate behaviours of Ms Taylor, but you provided no additional information when it was requested. The Unit subsequently determined the information reported did not provide any reasonable avenue of further enquiry and the allegations as initially provided did not raise a reasonable suspicion of corrupt conduct. Ms Reese attempted to contact you to discuss your allegations and concerns, but you did not respond to her several attempts to meet. The employer concluded that due to your refusal to participate in the process, they were unable to take any action to resolve your concerns.

In considering the evidence, I find you submitted a complaint regarding Ms Taylor and the employer took steps in response to receiving your complaint. However, the evidence does not support that no action was taken by management in response to your complaint. Therefore, I find this factor is able to be substantiated to the extent that you lodged a complaint, and some action was taken by management to review it, but not to the extent that there was no response to it.

Factor 3: Withholding of pay

In an email to Ms Taylor dated 8 April 2024, you requested a formal review of your payment for the period between 8 March 2024 and 31 March 2024. You expressed you did not receive payment for the Saturday public holiday on which you were not required to work and were not compensated with the correct penalty rates for your Monday shift, 6:00 am to 2:00 pm, following the Sunday shift, 2:00 pm to 10:00 pm.

Ms Elaine Grant, Client Service Officer in payroll, emailed Ms Taylor on 3 May 2024 explaining the issues with your pay.

On 10 May 2024 you emailed payroll and expressed your concerns regarding the challenges faced by Ms Taylor in relation to payment processing.

Reasons for decision

Payroll emailed you on 13 May 2024, and confirmed they could not see any issues had been corrected. They instructed you to speak with your manager to correct the issues through the submission of an AVA.

In an email to you dated 21 May 2024, Ms Taylor stated she was awaiting payroll confirmation about a few issues and as soon as she received it, she would submit an AVAC for the next pay run.

In the overview provided by you on 12 July 2024, you outlined various issues regarding your pay. These included the following:

- despite providing an accredited diploma of fitness, you did not receive the payment for recognised Australian education, as was communicated you would, and also did not receive additional payments
- you did not receive income for time off work due to Covid even though you provided the necessary paperwork for this entitlement
- Ms Taylor cancelled your shift without pay when you informed her in advance that you would be delayed in commencing work by approximately 1 hour
- you were denied fatigue pay due to an agreement you signed while you were employed on a casual basis, and which was inconsistent with your later signing of the full-time contract.

In an email to WorkCover dated 18 July 2024, you wrote Ms Taylor had been withholding payments for overtime you completed.

The employer, in their response dated 6 September 2024, submitted there was no evidence to suggest pay was withheld from you in accordance with your entitlements. They confirmed corrective action was taken by Ms Taylor as per internal processes to correct the payroll and entitlement errors you experienced.

Given the evidence, I am satisfied there were issues with respect to your payment and various actions were taken in response to them. I therefore find this factor is able to be substantiated.

Factor 4: Non-adherence to and manipulation of the roster

The policy regarding fatigue leave following weekends and/or rostered days off dated June 2020 stated the worker is entitled to either fatigue leave or overtime pay if the worker does not have at least 10 consecutive hours off duty between shifts.

Reasons for decision

The switchboard services roster for the period between 19 September 2022 and 2 October 2022 showed what shifts you were rostered to complete. I note all shifts had at least 10 hours between the finish time of the previous shift and the start time of the next shift.

On 7 August 2023, you emailed Ms Taylor and management and wrote you did not agree to be rostered as you were. You noted the union passed an agreement for rest days after night shifts and you did not agree to be rostered differently to this. For this reason, you refused to work on Monday morning with less than a 24-hour break following a night shift.

The switchboard services roster for the period between 4 March 2024 and 17 March 2024 depicted the shifts you were allocated. I note all shifts had a minimum of 10 hours between the finish time of the previous shift and the start time of the next shift.

The shifts allocated to you for the period from 18 March 2024 to 31 March 2024, according to the switchboard services roster, had a minimum of 10 hours in between them, however, this was not so for your shift on 17 March 2024 and 18 March 2024, which only had 7 hours in between.

The shifts allocated to you in the switchboard services roster for the period between 15 April 2024 and 28 April 2024 had at least 10 hours off duty between each shift. The same is seen in the switchboard services roster for the period from 29 April 2024 to 12 May 2024.

On 12 July 2024 you provided an overview of the events to WorkCover whereby you raised an issue with the rostering practices. You referred to signing an agreement while you were working on a casual basis which allowed you to only have 8 hours in between shifts, even though the standard break was 10 hours or fatigue pay was provided. However, later in the overview you submitted this agreement ceased operating once you commenced full-time employment, and you would never agree to an 8-hour break without compensation while working five shifts per week. You also asserted there was a reduction in penalty shifts, but an increase in fatigue due to the poor roster development. You believed your experience was undervalued in favour of less qualified individuals due to bias and friendships. Further, while other employees were granted shifts as per their requests, your requests for days off were ignored or taken as leave not scheduled. Additionally, the shifts were not fairly allocated, and employees favoured by Ms Taylor received shifts according to their requests while you were subjected to arbitrary scheduling.

In the response dated 9 August 2024, you submitted you were the only employee with such a short break time between shifts, and this was not fair or appropriate. Specifically in relation to the shifts

Reasons for decision

scheduled on 17 March 2024 and 18 March 2024, there was only 7 hours of break in between, and in considering travel time, it left you with less than 5 hours of break time. As a result of the rostering practices, you felt fatigued. You wrote Ms Taylor had a malicious intent in attempting to apply the agreement you signed at the commencement of your employment to change the minimum break time. You submitted that since your commencement, you have signed new terms and working arrangements as a full-time employee, and applying such a condition in these circumstances was legally and ethically malicious. You asserted the agreement to allow an 8 hour break instead of 10 hours does not require signing to relinquish it, was not legally binding and was no longer fit for purpose since your transition from casual employment to full-time. You concluded Ms Taylor exploited her role to control employees' income through malicious rostering structures and specifically in your case, also the use of the agreement made by you as a casual employee.

The response of the employer dated 6 September 2024 submitted that in pay period 20, commencing 4 March 2024, you were scheduled for 3 night shifts with appropriate rest days allocated. These shifts included two Sundays and one on Saturday. On 11 March 2024 you left work one hour early and arrived 15 minutes late on each Sunday. During pay period 21, you were not scheduled for any night shifts. On 19 March 2024 you took sick leave, and on 24 March 2024 arrived 27 minutes late. On 25 March 2024 and 26 March 2024 you took annual leave. You were also scheduled to work one full weekend and were rostered on for Good Friday on 29 March 2024 and Easter Sunday on 31 March 2024. The employer also confirmed you did not formally rescind the 8-hour agreement signed by you on 17 June 2020 and the change to your employment contract and adjustments in your working hours did not automatically invalidate the agreement. That meant it remained current.

In considering the information available, I find the employer only failed to maintain a 10-hour break in between your shifts, as required by policy, on one occasion, namely between the shifts on 17 March 2024 and 18 March 2024. No evidence has been provided in support of the remainder of your assertions made in relation to the rostering. I therefore find this factor is able to be substantiated only to the extent of the one occasion of non-adherence to policy.

After considering the evidence available at review, I am satisfied that factors 2, 3 and 4 have been established to an extent. Therefore, to the extent that your psychological injury has arisen out of these factors, I am satisfied that the injury has arisen out of your employment.

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The work capacity certificate of Dr Hawes dated 1 July 2024 indicated your mechanism of injury was consistent with his clinical findings and there was no pre-existing factor or condition. This was maintained in all later work capacity certificates.

According to the communications report, in a conversation with WorkCover on 2 September 2024, Dr Hawes stated that work events were the sole cause of your psychological injury.

Having regard to the medical evidence, I am satisfied your employment was a significant contributing factor to the psychological injury.

It is now necessary for me to determine whether your injury has arisen out of or in the course of management action, and if so, whether management action is found to be reasonable and taken in a reasonable way.

Did the injury arise out of or in the course of management action?

The term 'management action' is not defined in the Act. It has been interpreted by the Courts broadly and will vary depending on the circumstances of the case. The action must relate to action taken by or on behalf of the employer in connection with the worker's employment.

In *Avis v WorkCover Queensland*², the Industrial Court held that the phrase 'arising out of' involves a causal or consequential relationship between the management action and the injury, but does not require a direct or proximate relationship.

Factor 2: Management action taken in response to your complaint

Submitting complaints regarding aspects of your employment is connected to your employment. The complaint was reviewed by the Ethical Standards Unit, and then by Human Resources. These departments have the power to take action in response to your complaint on behalf of the employer. I find the steps taken by them were taken on behalf of the employer in connection with your employment and therefore amounted to management action.

Factor 3: Withholding of pay

The pay issues arose as a result of Ms Taylor refusing your claims for payment in specific situations. Ms Taylor was your direct manager and thus in a superior position, which provided her with the power to manage your employment. Receiving payment for your work is directly

² [2000] QIC 67

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connected to your employment. As such, I find Ms Taylor's decisions with respect to your remuneration amounted to management action.

Factor 4: Non-adherence to and manipulation of the roster

The roster scheduled shifts of employees and indicated when employees were to undertake their employment. The roster was developed by Ms Taylor, who was in a position to manage your employment. The development of a roster and consequent allocation of shifts to you is directly related to your employment since you are not able to undertake it without the roster. Based on this, I am satisfied that in developing a roster which was directly related to your employment, Ms Taylor was undertaking management action since she was able to manage your employment in this way. Therefore, the factor constitutes management action.

Was the management action reasonable and taken in a reasonable way?

In the matter of *Prizeman v Q-COMP*,³ President Hall stated that the onus is on the worker seeking compensation to establish that management action was unreasonable or undertaken in an unreasonable way, and that in the absence of evidence to support that assertion, management action must be considered reasonable.

I have also had regard to the case of *Bowers v WorkCover Queensland*,⁴ in which the Industrial Court determined that 'management action need not be perfect or above criticism and reasonable management action permits some failings, deficiencies and flaws'. It was also considered that blemishes in respect to performance by management does not make the management action unreasonable.

Factor 2: Management action taken in response to your complaint

I refer to the previous evidence outlined earlier in the decision with respect to this factor. I acknowledge that you felt no formal investigation was undertaken since the Ethical Standards Unit relied solely on the information provided by you to assess the allegations. The employer conceded this was true, but submitted this was because they required further information from you and despite requesting it, you refused to participate in the process. For this reason, they did not have sufficient information to undertake a thorough assessment of the allegations. Since the Unit acknowledged receipt of your complaint in a timely manner, emailed you on various occasions to request additional information, and ultimately delivered a timely decision, I find their actions were reasonable and taken in a reasonable way. It was reasonable for them to proceed without

³ [2005] QIC 53; (2005) 180 QGIG 481.

⁴ [2002] QIC 18.

Reasons for decision

undertaking a thorough investigation given you, as the complainant, were not willing to participate and provide further information. I note you submitted you did not have enough time to provide further information, however, due to your lack of communication of this information to the Unit, they could not have known this was the reason for your lack of participation. Likewise, I find Ms Reese contacted you twice in an attempt to schedule a meeting with you to discuss your concerns and allegations, and without your participation in this process, there was no other actions available for Ms Reese to take. Therefore, I am satisfied this factor amounted to reasonable management action taken in a reasonable way.

Factor 3: Withholding of pay

Section 23 of the Hospital and Health Service General Employees (Queensland Health) Award provides for payment entitlements with respect to public holidays. It indicates that an employee shall be paid at the rate of double time and one-half with a minimum of 4 hours for work done on Easter Saturday and Easter Sunday. All employees are also entitled to be paid a full day's wage for Labour Day irrespective of whether no work may be performed on such day, and if an employee does work on Labour Day, they are entitled to the full day's wage in addition to a payment for the time actually worked at one and one-half times the ordinary time rate of pay with a minimum payment as for 4 hours' work. Further, where an employee is in receipt of the additional week's leave as prescribed in clause 19.2(a) and is rostered off on Easter Saturday, Easter Sunday, Show Day or Labour Day, such employee shall be paid an additional day's wage or be granted a day's holiday in lieu at a time to be mutually arranged, but this is not applicable where an employee is not ordinarily required to work on a Saturday or Sunday.

The policy regarding fatigue leave following weekends and/or rostered days off dated June 2020 stated an employee was to be granted fatigue leave if the employee worked more than 2 hours overtime between 10:15 pm and the commencement of work on their next ordinary rostered working day and did not have at least 10 consecutive hours off duty during the 15 hours immediately preceding the commencement of work on the next ordinary rostered working day. This leave is to be granted until the employee had 10 consecutive hours off duty and no pay is to be deducted during this period. If an employee continues work without having had 10 consecutive hours off duty, payment is made at double rates until the employee is released from duty for such period and the employee is entitled to be absent until completing the required 10 consecutive hours off duty without loss of pay. The policy notes that many problems can be overcome regarding fatigue leave by either not rostering the employee who is on call for the first shift of the following day and not rostering on call an officer who is rostered for the first shift on the following day.

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The annotated pay advice dated 28 April 2021 indicates you received no payment for the public holiday you had off on 5 April 2021.

The annotated pay advice dated 26 May 2021 states you did not receive the public holiday payment due to you being rostered on the night shift and still required the payment.

The annotated pay advice dated 27 April 2022 submitted your shifts fell on days without public holiday rates and consequently you required payment for two public holidays.

On 4 April 2023 you messaged Ms Taylor with an image of the upcoming roster and stated 'Definitely needs amendment prior to the 4 public holidays'. In response she asked whether she gave you a weekend on that roster. You replied you received 1 of 5 public holidays and that public holiday was the only Sunday shift. She then wrote, 'I meant did I roster you the next weekend'. You responded you were okay with it, but her roster actually reduced your pay by \$1,500 for the fortnight. She replied that she would look at it tomorrow.

In the annotated pay advice dated 26 April 2023, you indicated you were allocated a night shift and again were not paid for the Easter Monday public holiday as per section 23.

On 20 February 2024, you messaged Ms Taylor, stating you had tested positive for Covid and asked what the current policy was. She stated you were eligible for up to 5 days paid leave with the Special Pandemic Leave request and you would have to provide photo evidence of your Covid test against your identification. She requested that you send it to her so she could submit it in MyHR. On 21 February 2024 Ms Taylor then messaged you and informed she had to decline your request in MyHR because the minimum period for Covid leave was 5 days for a positive staff member. She noted the statutory declaration you provided was sufficient evidence. She requested for you to resubmit your leave days. After you asked for some clarification and she provided it, you messaged you were confused about why your amended leave request was declined. You noted you did everything as required. Ms Taylor replied that the dates were fine, but it was necessary for you to attach the statutory declaration to the request, and once it was attached she could approve it. She stated she was then in her office if you wanted further assistance with this. You then sent her screenshots and stated you were not sure what was occurring, and you never deleted the declaration on your end.

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You emailed Ms Taylor on 8 April 2024 and requested a review of your payment for the pay period between 8 March 2024 and 31 March 2024. You specified you did not receive payment for the Saturday public holiday on which you were not required to work and had not been compensated with the correct penalty rates for your Monday shift following a Sunday shift. You noted the Hospital and Health Service General Employees (Queensland Health) Award stipulated in part 5 section 15 that unless there was a mutual agreement of regular rosters, employees were entitled to a minimum 10-hour break.

In response to your email dated 8 April 2024, Ms Taylor sent you an email on 9 April 2024. She stated she submitted an AVAC for public holiday not required on Saturday 30 March 2024 and this would be processed in the next pay run. With respect to the other dates you outlined, she asked for you to submit an enquiry through MyHR payroll enquiries so they can investigate your entitlements for public holiday not required payments, since they are older than 3 months. Ms Taylor noted that if payroll decided you were entitled to those payments, she would happily submit a validation of claims older than 3 months form. She added she also escalated your enquiry regarding fatigue leave to Human Resources to confirm policies around this. She then proceeded to acknowledge the importance of communication and requested that you raise your concerns with her as soon as they arise so she could action them sooner for you.

On 24 April 2024 you emailed Ms Taylor and informed you had been in touch with the payroll department regarding discrepancies in your payments, who advised you to notify Ms Taylor that payments for public holidays not worked would need to be processed as overtime in the fortnightly pay cycle. You asked for Ms Taylor to please take the necessary action and provide a PRN for confirmation. You then expressed your dissatisfaction with the process and that it had been more than 2 weeks without any response or overtime payment.

Ms Taylor responded to your email on 1 May 2024 via email and stated that after a consultation with payroll and Human Resources, your request for fatigue payment for 18 March 2024 would not be processed due to the existing 8-hour agreement signed by you on 17 June 2020. However, she noted you are able to terminate this agreement going forward. In relation to your query about the top-up/reduction of hours on your payslips, Ms Taylor indicated an investigation was currently underway by payroll. She explained the payroll system automatically generated adjustments for top-up/reduction hours which she was unable to view directly and that if you notice any discrepancies on your future payslips, to inform her immediately so she could facilitate a prompt resolution. Likewise, payroll was also at the time still reviewing your entitlements regarding public holidays not required arising while you were a part-time employee, and also your overtime

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concerns. She confirmed that since your commencement of full-time employment, all payments for public holidays not required had been processed.

Ms Grant emailed Ms Taylor on 3 May 2024 and outlined you had wages top up on fortnight commencing 5 February 2024 for 0.95 minutes as you were under contracted hours, and it was topping up the missing wages from your RDO balance. This was due to you having a 7-hour shift on 9 February 2024 instead of an 8-hour shift. Ms Grant stated that if a shift was shortened when RDO was accruing, the remaining shift not worked needs to be booked as leave without pay to stop the top up. The same occurred for the following fortnight commencing on 19 February 2024. The opposite occurred for the fortnight commencing on 18 March 2024. There were too many ordinary shifts, which resulted in a wage reduction of 7.55 hours because one of the shifts had to be overtime. Likewise, during the fortnight commencing 1 April 2024, you worked overtime. Ms Grant requested Ms Taylor to submit an AVAC to correct the shifts for each fortnight so you were paid correctly and your RDO balance could be amended.

On 10 May 2024, you emailed payroll and expressed your concern regarding Ms Taylor's challenges with payment processing.

In response to your email of 10 May 2024, payroll responded via email dated 13 May 2024 and informed they could not see that any issues had been corrected. They instructed for you to speak to Ms Taylor to have them corrected with a submission of an AVAC.

On 21 May 2024, Ms Taylor emailed you and informed she was awaiting payroll confirmation regarding some payroll issues and as soon as she received it, she would submit an AVAC for the next pay run.

The Attendance Variation and Allowance Claim signed by Ms Taylor dated 28 May 2024 indicated you finished an hour early on 9 February 2024 and on 28 February 2024, had the public holiday off on 30 March 2024 and finished an hour late on 1 April 2024.

The Validation of Claims Older than 3 months Form signed by Ms Taylor on 28 May 2024 stated adjustments were to be made to your shifts due to incorrect entries, and once these adjustments were processed, your RDO balance would be correct.

The overview of events provided by you on 12 July 2024 generally stated it was unreasonable for Ms Taylor to decline various payments claimed by you given you had an entitlement to them. You

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submitted you were entitled to the following payments: accredited payments for recognised Australian education, income while you were on sick leave due to Covid, and fatigue pay.

In an email to WorkCover dated 18 July 2024, you submitted Ms Taylor was withholding payment for your overtime hours. You explained that initially Ms Taylor advised you to contact payroll to resolve this. At that point, you had already reached out to them but were of the understanding that the proper process to address pay discrepancies was to involve management, which is why you initiated the contact with Ms Taylor. Following an email from payroll, Ms Taylor informed she was awaiting their response. You noted you found it appropriate that payroll included Ms Taylor and yourself in the emails. After waiting for a week, you followed up with payroll to inquire about Ms Taylor's response and were informed she had not yet replied, however eventually Ms Taylor again told you she was waiting for a response from payroll. You described the situation as frustrating and exhausting. You felt this was an act of control and abuse, forcing you to fight for your rightful earnings. Further, you submitted it was unreasonable for Ms Taylor to avoid resolving this issue during work hours.

In a response dated 9 August 2024, you stated you were entitled to fatigue pay due to only having a 7-hour break between the end of your shift on 17 March 2024 and beginning of the shift on 18 March 2024. Due to the fatigue from this, you took leave on 19 March 2024, but the payment was taken from your sick leave, and you did not receive fatigue pay. You also noted Ms Taylor attempted to apply the agreement you signed as a casual employee allowing only an 8, instead of a 10, hour break in between your shifts. You highlighted this agreement was no longer in force, since you signed a permanent full-time contract, which superseded it.

In another response also dated 9 August 2024, you stated that an examination of the dates requiring payment adjustments, compared to the dates when your payment was actioned and processed, is concerning. You further submitted Ms Taylor preferentially rostered employees, which led to a lack of shifts for you on public holidays and consequently a reduction in payment. You then proceeded to submit you were not paid as required for the most recent public holiday, and when you contacted Ms Taylor regarding it, she was slow to action it and falsely misrepresented that it was payroll's fault. You noted she eventually processed the pay correction on 28 May 2024, which was not timely given that the pay error dated back to February 2024. With respect to the income you were entitled while on sick leave due to Covid, you wrote Ms Taylor abused her authority and bullied you by delaying and not approving your leave. You enquired from Ms Taylor whether she preferred you to send her the information directly or through MyHR. As per her request, you uploaded the statutory document to MyHR on 20 February 2024. Ms Taylor then

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sent you a message on 21 February 2024 informing she declined the leave because the dates were incorrect. You submitted that at a later time, Ms Taylor then informed you the dates were correct, after you sent her the guidelines, and claimed you had not uploaded the statutory declaration. She again declined your leave. On 29 February 2024, you sent Ms Taylor a text message asserting the documents were there and showed her on your phone that the declaration was still uploaded, whereby the upload time was stated as 20 February 2024. Ms Taylor contended it was not there when she declined the leave, and she finally approved the entitlement after the third attempt on 29 February 2024. You noted you were eventually paid, in both instances, but Ms Taylor used her authority to withhold your payments maliciously.

The response of the employer dated 6 September 2024 submitted they had no evidence to suggest your pay had been withheld. Corrective action was taken by Ms Taylor as per the employer's internal process to correct payroll and entitlement errors.

I acknowledge there were various issues regarding your pay. Throughout your submissions you have expressed your dissatisfaction with the process of the employer in reviewing your payments and highlighted it was unreasonable for you to be required to fight for your entitlements. I find that the extent of pay issues experienced by you amounted to blemished management action. However, I note the pay issues ultimately were all resolved. Ms Taylor, payroll and Human Resources were always encouraging of you expressing your issues as soon as possible, were attentive to the issues you expressed and undertook necessary reviews and corrective actions to resolve them. As such, while the occurrence of the issues was a blemish, the resolution of them amounted to reasonable management action taken in a reasonable manner. Therefore, I find this factor constitutes reasonable management action taken in a reasonable manner.

Factor 4: Non-adherence to and manipulation of the roster

I note previously in this decision I found this factor was substantiated only to the extent that the employer failed to provide you with a minimum 10-hour break between shifts on 17 March 2024 and 18 March 2024. As a result, my decision regarding whether this factor amounts to reasonable management action taken in a reasonable way will be focused only on this substantiated non-adherence.

The policy regarding fatigue leave following weekends and/or rostered days off dated June 2020 advocates for a minimum 10 hours of break between the finish of one shift and the commencement of the next shift by an employee.

Reasons for decision

The fatigue risk management policy dated February 2021 requires line managers to ensure fatigue risk is assessed and recorded, and where level of risk is high, to manage, monitor, review and communicate the risk.

The switchboard services roster for the period between 4 March 2024 and 17 March 2024 shows you were scheduled to work until 11:00 pm on 17 March 2024.

The switchboard services roster for the period between 18 March 2024 and 31 March 2024 demonstrated you were scheduled to commence working on 18 March 2024 at 6:00 am.

On 8 April 2024, you emailed Ms Taylor and requested a review of your pay for the period between 8 March 2024 and 31 March 2024. You particularly requested that you be compensated with the correct penalty rates for your Monday shift from 6:00 am to 2:00 pm, following a Sunday shift rostered from 2:00 pm until 10:00 pm on the previous fortnight.

In response to your email dated 8 April 2024, Ms Taylor sent you an email on 9 April 2024. She stated that, as discussed previously with you, if you have any concerns regarding your rosters, to please see her as soon as possible.

On 1 May 2024, Ms Taylor informed you via email that following a consultation with payroll and Human Resources, your request for fatigue pay for 18 March 2024 would not be processed because of the existing 8-hour agreement. She mentioned you had the option to terminate the agreement and she could provide you with the information regarding how to do so.

In the overview provided by you on 12 July 2024 you explained that upon commencing your casual employment, when you were working 2-4 days per week, you signed an agreement allowing the employer to roster you on shifts which had a minimum 8-hour break, instead of the standard 10-hour break required. Eventually, after you had transitioned to full-time employment, you raised your concerns about fatigue pay. Ms Taylor responded to this that she had to consult Human Resources about your underpayment, and after a lengthy process, she rejected your claim and cited an agreement for an 8-hour break between your shifts, which you noted contradicted your full-time contract. You pointed out she was wrongfully applying the agreement made during your casual employment, but received no further correspondence from her and no payment was made. You submitted it was unreasonable for an employer to intentionally schedule you for shifts with less than a 10-hour break in between and to apply the contract which you signed when you were only working twice a week.

Reasons for decision

In your response dated 9 August 2024, you stated you were expected to finish on 17 March 2024 at 11:00 pm and start again on 18 March 2024 at 6:00 am. This meant you received only a 7-hour break between the shifts, which did not include travel time. If you factored in travel time, your break between shifts would have been less than 5 hours. You reiterated it was unreasonable for Ms Taylor to apply the 8-hour agreement you signed earlier in this situation.

The response of the employer dated 6 September 2024 submitted that during pay period 20, commencing on 4 March 2024, you were scheduled for 3 night shifts with appropriate rest days allocated. You arrived 15 minutes late on 17 March 2024. The employer wrote that records indicate Ms Taylor took appropriate action by shortening your shift, but this resulted in you being remunerated for the 30 minutes you did not work. The employer did not mention your shift on 18 March 2024. They also confirmed you had not formally rescinded your agreement for only an 8-hour break, which you signed on 17 June 2020, and the change of your contracts and adjustments in working hours did not invalidate this agreement. This meant the agreement remained current. In their response, the employer included an extract of the Hospital and Health Services General Employees (Queensland Health) Award, which stated that employees must be provided with a break of not less than 10 hours between the termination of one shift and the commencement of another shift, and 8 hours applied instead of 10 only in specific circumstances.

In considering the evidence, I find there was uncertainty between you and the employer regarding whether the 8-hour agreement continued to apply. However, it is not necessary for me to consider whether the employer's stance on its application was reasonable. The break between the shift on 17 March 2024 and 18 March 2024 equated to 7 hours. Even if you were allowed to leave early on 17 March 2024 as suggested by the employer, you left a maximum of 30 minutes early, which meant you still did not receive a minimum 8-hour break. Based on this, I find the rostering of these two shifts amounted to unreasonable management action given that it was in direct contradiction to the award and the 8-hour agreement.

Conclusion

After considering the evidence available at review, in relation to the factors for determination, I have determined:

- you sustained a personal injury of a psychological nature
- your injury arose out of employment, to the extent that it arose out of factors 2, 3 and 4, where employment was a significant contributing factor
- factors 2 and 3 amounted to reasonable management action taken in a reasonable way

Reasons for decision

- factor 4 amounted to unreasonable management action.

In the decision of *Delaney v Q-COMP*⁵ the Industrial Court held that in determining the issue of whether management action was reasonable and taken in a reasonable way, a global evaluation of management actions must be made. In *Delaney*, it was held that when management actions are joined by subject matter, time and personality, the actions are entitled to be evaluated on a global basis.

I note that since two out of the three causative factors amounted to reasonable management action taken in a reasonable way, this means your psychological injury mainly arose out of such management action. Further, factor 4 was initially only substantiated to the extent of one incident and it was only the one incident which amounted to unreasonable management action. Therefore, I find overall your psychological injury arose out of reasonable management action taken in a reasonable way and section 32(5) is consequently enlivened.

Summary

I therefore conclude that the provisions of section 32(5) of the Act exclude your psychological condition from the definition of 'injury' within section 32(1) of the Act.

Therefore, I have determined that you did not sustain an 'injury' in accordance with section 32 of the Act and you do not have an entitlement to compensation.

Appeal rights

If either party disagrees with this decision then either party may appeal to the Queensland Industrial Relations Commission in Brisbane. Either party has 20 business days from the date of receipt of this decision in which to lodge an appeal. A copy of the notice lodged with the Commission must also be served on the Workers' Compensation Regulator within 10 business days.

For information about the appeal process please contact me or refer to:

<https://www.worksafe.qld.gov.au/claims-and-insurance/compensation-claims/appealing-a-review-decision/guide-to-appealing-a-review-decision>.

⁵ [2005] QIC 11; 178 QGIG 197

Reasons for decision

Yours sincerely



Victoria Squires

Senior Reviewing Officer

Review Unit

Workers' Compensation Regulatory Services

cc Metro South Hospital And Health Service

cc WorkCover Queensland

Form 24 – Notice to admit facts

Industrial Relations Act 2016, section 989
Industrial Relations (Tribunals) Rules 2011, rules 41, 49, 108 and 113

seal

Applicant/Appellant:	Cory Lea Shepherd
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V

Respondent:	
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PLEASE NOTE:
If there are more than two parties to this application, please complete a **Form 1 – Parties list** and file it with this form.

To:	
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Take notice that the ○ applicant ○ appellant ● respondent in this proceeding proposes to prove the facts specified below, and if you do not within 14 days serve a notice on the ○ applicant ○ appellant ● respondent disputing the facts you are taken to admit, for this proceeding only, the facts specified in this notice.

No.	Fact to be Admitted (with Exhibit Reference)	Admit / Deny
1	On 17–18 March 2024 , the Appellant was rostered to finish work at 23:00 and commence his next shift at 06:00, resulting in a break of only 7 hours (Exhibit C2) .	
2	In the Review Decision dated 24 October 2024 (Exhibit B1 , p. 27), the Review Officer found that rostering the Appellant with a 7-hour break " <i>amounted to unreasonable management action</i> ".	
3	The Employer's "Fatigue Risk Management Policy" (Exhibit F1) and the relevant Award require a minimum break of 10 hours between shifts, or 8 hours by written agreement.	
4	No written agreement existed for the Appellant to work an "Eight-hour break" shift pattern in March 2024.	

No.	Fact to be Admitted (with Exhibit Reference)	Admit / Deny
5	Ms. Tammy Reese admitted in an email dated 7 August 2023 : <i>"I do realise in this instance there was a rostering error that was accidentally made by Chloe with regards to night shifts"</i> (Exhibit E1 , p. 20).	
6	On 6 June 2023 , Line Manager Ms. Chloe Taylor removed a page containing the Appellant's handwritten entry from the workplace Communication Book (Exhibit I2).	
7	The entry removed by Ms. Taylor contained operational information regarding the necessity of verifying and updating doctors' contact details.	
8	Maintaining accurate contact details for medical staff is a critical function of the Switchboard to ensure effective clinical handover and patient safety.	
9	By removing this entry without consultation, Ms. Taylor prevented the communication of safety-critical information to other staff members.	
10	In an email to Ms. Reese dated 6 June 2023 at 4:05 PM (Exhibit E1 , p. 10), Ms. Taylor referred to the Communication Book—a government record—as a <i>"burn book"</i> .	
11	The term "Burn Book" is a specific pop-culture reference to the film <i>Mean Girls</i> , describing a book used to spread rumours, insults, and social gossip.	
12	Ms. Taylor's characterisation of a safety-related record as a "burn book" indicates she viewed the workplace through a social clique lens rather than an objective professional assessment.	
13	Ms. Carolyn Jeffrey (witness) provided a statement (Exhibit I2) corroborating that Ms. Taylor <i>"rips the page out"</i> and confirmed the action was <i>"humiliating for Cory"</i> .	
14	In the email to Ms. Reese dated 6 June 2023 (Exhibit E1 , p. 10), Ms. Taylor explicitly admitted: <i>"I did raise my voice and asked him to please stop talking over the top of me."</i>	
15	Ms. Jeffrey's statement (Exhibit I2) corroborates this event, stating Ms. Taylor <i>"yelled at Cory over something that did not warrant her reaction"</i> .	
16	Raising one's voice at a subordinate in an open office environment constitutes unprofessional conduct and is not "reasonable management action".	

No.	Fact to be Admitted (with Exhibit Reference)	Admit / Deny
17	The "Union Encouragement Policy" (QH-POL-248) (Exhibit F2) requires managers to take a "positive, supportive role" to facilitate union membership and delegate elections.	
18	The Appellant notified management of his intent to become a Union Delegate on 11 August 2023 (Exhibit F3).	
19	Management failed to facilitate a vote or provide necessary delegate information to the Appellant for a period exceeding 9 months after his initial request.	
20	On 24 December 2024 , the Ethical Standards Unit (ESU) determined that the Appellant's complaint regarding Ms. Taylor and Ms. Reese constituted a Public Interest Disclosure (PID) (Exhibit E3).	
21	On 15 May 2024 (48 hours after the Appellant lodged the PID complaint), Director Tammy Reese sent an email directing the Appellant to <i>"retract"</i> an email he had sent inquiring about office hours (Exhibit E5).	
22	On 9 May 2024 , Ms. Taylor sent a substantively similar email regarding another staff member's hours, for which she was not directed to issue a retraction (Exhibit E5).	
23	The Respondent obtained the Appellant's full medical history (Exhibit A5) directly from "Our Medical Ashmore" on or before 8 July 2025 .	
24	The copy of Exhibit A5 filed by the Respondent bears the system-generated footer: <i>"Printed on 8th July 2025"</i> .	
25	The Respondent did not serve a Form 29 Notice of Non-Party Disclosure on the Appellant prior to obtaining these records (breach of Rule 64E).	
26	Exhibit A5 contains medical entries irrelevant to the appeal, specifically: Treatment for diarrhoea ("Lomotil", p. 48).	
27	Exhibit A5 contains medical entries irrelevant to the appeal, specifically: Treatment for haemorrhoids ("Proctosedyl", p. 53).	
28	Exhibit A5 contains medical entries irrelevant to the appeal, specifically: Pathology requests for "Urine Chlamydia & Gono PCR" (p. 50).	
29	Exhibit A5 contains medical entries irrelevant to the appeal, specifically: Prescriptions for "Viagra" and "Cialis" (p. 48, 53).	
30	The Respondent served these unredacted records containing the particulars listed in Items 26–29 directly on the Appellant's former solicitors on 22 July 2025 (Exhibit H1).	
31	The disclosure of these private medical particulars caused the Appellant distress and contributed to the breakdown of his legal representation.	

No.	Fact to be Admitted (with Exhibit Reference)	Admit / Deny
32	On 14 July 2025 , the Respondent's representative (Renee Matheson) emailed the Appellant stating she had " <i>received last week</i> " documents but did not disclose possession of Exhibit A5 (Exhibit H1) .	
33	Exhibit A5 (p. 52) contains a consultation record dated 16 November 2023 authored by Dr. Priyal De Silva Nanayakkara.	
34	The entry dated 16 November 2023 explicitly states: " <i>No psychological illness such as depression/ psychosis.</i> "	
35	The Respondent is not in possession of any contemporaneous medical evidence prior to 2024 that contradicts the finding in Item 34.	
36	The Work Capacity Certificate issued by Dr. Peter Hawes dated 1 July 2024 (Exhibit A1) identifies the mechanism of injury as " <i>ongoing breaking of workplace rules by bosses, victimizing him</i> ".	
37	In the Review Decision (Exhibit B1 , p. 16), the Regulator stated: " <i>I am satisfied your employment was a significant contributing factor to the psychological injury</i> ".	
38	Dr. Ravikumar Krishnaiah (Psychiatrist) diagnosed the Appellant with Major Depressive Disorder on 13 February 2025 (Exhibit A4) .	

No.	Fact to be Admitted (with Exhibit Reference)	Admit / Deny
39	Dr. Krishnaiah opined that premature exposure to the workplace is likely to result in <i>"significant deterioration in my mental health"</i> (Exhibit A4 , p. 5).	
40	On 3 May 2024 , Payroll Officer Elaine Grant sent an email to Ms. Taylor instructing her to <i>"submit an AVAC to correct these shifts"</i> for the Appellant (Exhibit E2).	
41	Ms. Taylor did not submit the required AVAC until 28 May 2024 (Exhibit D2), a delay of 25 days despite the explicit written instruction from Payroll.	
42	Exhibit D1 (Employee Payslip Comparison) demonstrates a total gross pay for the comparator employee of \$18,169.14 over the sample period.	
43	Exhibit D1 demonstrates a total gross pay for the Appellant of \$12,757.68 over the same period.	
44	This represents a gross pay disparity of \$5,411.46 (approximately 42%) to the Appellant's detriment.	
45	This wage gap was caused primarily by the differential allocation of penalty-rate shifts (weekends and public holidays) to the comparator employee rather than the Appellant.	
46	On 21 May 2024 at 12:33 PM , Ms. Taylor emailed the Appellant stating: <i>"I am waiting payroll confirmation... as soon as I do get that confirmation, I will submit an AVAC"</i> (Exhibit E2 , p. 236).	
47	At the time of sending the email described in Item 46, Ms. Taylor had already been in possession of written instructions from Payroll Officer Elaine Grant since 3 May 2024 (Exhibit E2 , p. 237).	
48	Ms. Taylor's statement on 21 May 2024 that she was "waiting payroll confirmation" was false or misleading, as the instruction had been provided to her 18 days earlier.	
49	This conduct demonstrates a lack of diligence and transparency in managing the Appellant's financial entitlements.	
50	In the Review Decision (Exhibit B1), the Regulator attributed the payroll errors to "human error," but failed to address the misleading statement made by Ms. Taylor on 21 May 2024.	

Signature:	
Print name:	
Title of office held:	

Date:

____ / ____ / _____

Respondent's response to the appellant's notice to admit facts

Industrial Relations Act 2016, section 989
Industrial Relations (Tribunals) Rules 2011, rules 41 and 49

seal

Appellant:	Cory Shepherd
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v

Respondent:	Workers' Compensation Regulator
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PLEASE NOTE: If there are more than two parties to this application, please complete a **Form 1 – Parties list** and file it with this form.

To:	Cory Shepherd
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Take notice that the Respondent in this proceeding responds to the Appellant's notice to admit facts as follows:

	Respondent's Response
1.	With respect to paragraph 1, admits the facts contained therein.
2.	With respect to paragraph 2, does not admit the fact contained therein because the appeal is a hearing de novo and it is for the Commission to determine if this fact amounted to unreasonable management action.
3.	With respect to paragraph 3, admits the facts contained therein.
4.	With respect to paragraph 4, denies the fact because in June 2020, the Appellant signed an agreement allowing an 8 hour break between shifts. This fact also appears at paragraph 34 of the Appellant's statement of facts and contentions, which have been filed in the Commission.
5.	With respect to paragraph 5, admits the facts contained therein.
6.	With respect to paragraph 6, admits that Ms Taylor removed a page from the workplace communications book but does not admit that the page contained the appellant's handwriting because the respondent does not have a copy of the page.
7.	With respect to paragraph 7, does not admit the facts contained therein because the Respondent does not have a copy of the document referred to and is unaware of its contents.
8.	With respect to paragraph 8, admits the facts contained therein and says that there was a procedure in place for this to occur.
9.	With respect to paragraph 9, denies as untrue the fact, because there was already a procedure in place.
10.	With respect to paragraph 10, does not admit the fact that the email dated 6 June 2023 at 4.05 pm Ms Taylor refers to the Communication book as a "burn book" because the email states that Ms Taylor

	Respondent's Response
	felt like the Communications Book was being used as a “burn book” and not being used a professional tool. Further the Respondent says the email as a whole, needs to be read to determine the meaning of the phrase and context. Further, the Respondent denies as untrue that the Communications book is a government record
11.	With respect to paragraph 11, does not admit the fact, because the Respondent it unsure of the relationship to the film Mean Girls.
12.	With respect to paragraph 12, denies as untrue because Ms Taylor identified the Communications Book as “a professional tool” that switchboard can use to communicate.
13.	With respect to paragraph 13, the respondent has viewed an email from Ms Jeffrey dated 18 July 2024 where she states the words “rips the page out” and “humiliating for Cory”
14.	With respect to paragraph 14, admits the facts contained therein.
15.	With respect to paragraph 15 the Respondent does not admit this fact because the Respondent has viewed an email from Ms Jeffrey dated 18 July 2024 where she states the words “ <i>yelled at cory over something that did not warrant her reaction</i> ” but confirms that she did not witness the event and was told about it by the Appellant.
16.	With respect to paragraph 16, denies as a fact, and says that the characterisation of any management action is a matter for determination by the Commission.
17.	With respect to paragraph 17, admits the Policy says words to that effect.
18.	With respect to paragraph 18, admits that the Appellant sent a text to Ms Taylor at 5.03pm that he was “just putting his hand up” but says that this was not a formal notification.
19.	With respect to paragraph 19, denies as untrue the facts, because: • the Appellant did not request assistance; • Ms Taylor did provide advice and support to the Appellant; • the Appellant was provided with delegate information.
20.	With respect to paragraph 20, admits the facts contained therein.
21.	With respect to paragraph 21, admits the facts contained therein but says there was no correlation between the two events. The Respondent says further that Ms Reese also emailed the Appellant about retracting the email on 21 May 2024 and all of the emails need to be read as a whole to determine the meaning and context
22.	With respect to paragraph 22, denies as untrue the facts, because the circumstances were different and did not require Ms Taylor to retract her email.
23.	With respect to paragraph 23, denies as untrue the fact that the Respondent obtained the Appellant’s full medical history and says that the Respondent obtained records in a date range in line with Form 29 - Notice of non-party disclosure, signed by a Senior Registry Officer on 4 July 2025, and the Respondent received the medical records some time before 16 July 2025.
24.	With respect to Paragraph 24 does not admit the facts, as there is no footer on the medical records but does admit the records state on page one that the records were printed on 8 July 2025. The Respondent is unable to verify the source of the date but admits that the Form 29 - Notice of non-party disclosure, was signed by a Senior Registry Officer on 4 July 2025 and that the medical records were received by the Respondent at some time before 16 July 2025.
25.	With respect to paragraph 25, admit the fact and says that the Respondent did not serve a Form 29 – Notice of Non-Party Disclosure on the appellant prior to obtaining the records and says there was no requirement to do so pursuant to section 64D <i>Industrial Relations (Tribunals) Rules</i> 2011. The Respondent denies as untrue that the Respondent breached Rule 64E.

	Respondent's Response
26.	With respect to paragraphs 26 to 29 denies the fact that details contained in the records are not relevant because, read as a whole, the medical records are relevant in that they tend to prove or disprove an allegation in the proceeding.
27.	With respect to paragraph 30, admits the fact that the Respondent served the medical records on the Appellant's solicitors on 22 July 2025 and says further the Respondent was not notified that the Appellant's solicitors no longer acted for the Appellant until 29 July 2025.
28.	With respect to paragraph 31, does not admit the facts because these are matters within the Appellant's knowledge.
29.	With respect to paragraph 32, admits that the Respondent sent an email directly to the Appellant on 14 July 2025 and says that the email addressed to the Appellant on that date was in error and that the email should have been sent to the Appellant's solicitors, because he was legally represented. The Respondent states that the medical records were not in the possession of the Regulator at the time of sending the email on 14 July 2025.
30.	With respect to paragraph 33, admits the fact that there is an entry from Dr Nanayakkara in the records but does not admit the accuracy of the record.
31.	With respect to paragraph 34, admits the entry is listed in the record, but does not admit the accuracy of the entry.
32.	With respect to paragraph 35, denies untrue the fact because the records show a past medical history of Anxiety and ADHD from 26 October 2022 which is missing from Exhibit A5.
33.	With respect to paragraph 36, admits the fact Dr Peter Hawes provided a medical certificate stating that the mechanism was "ongoing breaking of workplace rules by bosses, victimizing him" but denies that as a fact and further denies the date of the certificate is 1 July 2024 because the certificate is signed 8 September 2024.
34.	With respect to paragraph 37, admits the contents of the Respondent's review unit decision but says that this fact is not relevant because the appeal is a hearing de novo and it is for the Commission to determine. action
35.	With respect to paragraph 38, admits that the report does say that the Appellant was suffering from Major Depressive Disorder, but does not admit the accuracy of the report.
36.	With respect to paragraph 39, admits that the report does say: <i>"Given the complex workplace issues, ongoing WorkCover claim and disputed unfair dismissal proceedings and impaired decision making abilities it is unclear at this stage the work goal and return to work. I will further update on this after next consultation. Please note that premature exposure to the workplace is more likely result in significant deterioration in his mental health..."</i> , but does not admit the accuracy of the report.
37.	With respect to paragraph 40, admits the facts contained therein.
38.	With respect to paragraph 41, admits the fact that Ms Taylor submitted the AVAC on 28 May 2024 but also says that on 21 May 2024 Ms Taylor emailed the appellant stating she was waiting for payroll confirmation on a few issues and would submit an AVAC for the next pay run as soon as she received it.
39.	With respect to paragraphs 42 to 45, denies untrue the fact, because the comparator used does not represent a true comparator in the circumstances and because the roster was equitable.
40.	With respect to paragraph 46, admits the facts contained therein.

	Respondent's Response
41.	With respect to paragraphs 47, denies untrue the fact, because Ms Taylor needed confirmation from payroll and then she needed to be satisfied that the AVAC was accurate.
42.	With respect to paragraph 48, denies as untrue the fact, because the statement was not false or misleading.
43.	With respect to paragraph 49, denies as untrue the fact, because Ms Taylor's conduct does not lead to a conclusion that there was a lack of due diligence or transparency in managing the Appellant's financial entitlements.
44.	With respect to paragraph 50, admits the contents of the Respondent's Review Unit decision and says that this fact is not relevant because the appeal is a hearing de novo and it is for the Commission to determine. Does not admit that Ms Taylor made a misleading statement

Name:	Workers' Compensation Regulator		
Name of contact person:	Ms Renee Matheson		
Postal address:	150 Mary Street		
	Brisbane		4001
Phone number:	07 3842 2792	Fax number:	07 3811 6443
Mobile number:			
Email address:	Renee.Matheson@oir.qld.gov.au		
Signature:			
Date:	18/02/2026		



Leave Request ▾

Estelle Bain



EB

Person ID: 388372

Process Reference: 000015480560

Position: AO Switchboard (32002276)

Organisational Unit: MSH CI LOGH Switchboard Srvs (72010875)



Request



Attachments



Comments



Process Flow



History

Employee - Cory Shepherd : created draft.

Cory Shepherd · 20.02.2024, 11:24:48

Employee - Cory Shepherd : submitted request.

Cory Shepherd · 20.02.2024, 11:41:27

Reviewer - Chloe Donovan-Taylor : declined request.

Chloe Donovan-Taylor · 21.02.2024, 12:23:43

Employee - Cory Shepherd : submitted request.

Cory Shepherd · 28.02.2024, 06:07:32

Reviewer - Chloe Donovan-Taylor : declined request.

Chloe Donovan-Taylor · 29.02.2024, 09:06:07

Employee - Cory Shepherd : submitted request.

Cory Shepherd · 29.02.2024, 11:07:28

Reviewer - Chloe Donovan-Taylor : approved request

Chloe Donovan-Taylor · 29.02.2024, 11:21:03

Manager - Tammy Reese : approved request

Tammy Reese · 01.03.2024, 16:05:08

Print

Instrument of Human Resource Sub-Delegation

COVID-19 Pandemic Event - Paid Special Pandemic Leave

The Director-General, Department of Health has administratively applied by way of Memoranda paid Special Pandemic Leave under the *Minister for Industrial Relations and Commission Chief Executive Directive 01/20: Employment Arrangements in the Event of a Health Pandemic* and paid discretionary Special Leave under *Minister for Employment and Industrial Relations Directive 05/17: Special Leave* to health service employees employed under the *Hospital and Health Boards Act 2011* during the declared pandemic event. Further the Director-General has authorised Health Service Chief Executives to sub-delegate the authority to approve or not approve these leave types. These Directives provide leave entitlements beyond those provided in relevant industrial Awards and Certified Agreements.

Therefore, I, Noelle Cridland, Health Service Chief Executive (HSCE), Metro South Hospital and Health Service, in pursuance of Section 34, Division 3, Part 2 and Section 46*, Division 1, Part 3 of the *Hospital and Health Boards Act 2011*, effective as at **5 December 2022**:

- (a) sub-delegate the power to approve/not approve **paid Special Pandemic Leave** in accordance with Directives 05/17 and 01/20 respectively, for this pandemic event (COVID-19) only, to a Band 9 delegate.

This Instrument does not permit the further sub-delegation of the powers outlined in (a) above.

Approved by:



Noelle Cridland
A/Health Service Chief Executive
Metro South Hospital and Health Service

23/11/22